
Dr. Pooja's Physio — Complete Manual E2E Test Plan & Feature Guide

Feature guide and click-by-click regression suite

Application

Dr. Pooja's Physio — Next.js 16 · React 19 · Supabase · Razorpay

Document version

1.0

Audience

A tester who has never used this application before

Environment

Throwaway Supabase project · Razorpay test mode · npm run dev

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Section 6 — STEP 0, Reset the test environment

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Dr. Pooja's Physio — Complete Manual E2E Test Plan & Feature Guide

Document type	QA / UAT manual — feature guide plus click-by-click regression suite
Application	Dr. Pooja's Physio (Next.js 16 App Router, React 19, Supabase, Razorpay, Google Calendar/Meet)
Audience	A tester who has never used this application before
Version	1.0
Status	Pre-launch. The application has no real patients. The Debug Bar is deliberately visible in every environment.

1. Document purpose

This document does two jobs at once.

- Feature guide.** Before every group of test cases there is a plain-English explanation of what the feature is, why it exists, who uses it, which admin configuration controls it, what it interacts with, and what can go wrong. You should be able to read a section and understand the feature without opening the code.
- Executable test plan.** Every test case names the exact screen, the exact control, the exact value to type, and the exact result to expect. Nothing says "select a slot" or "verify the dashboard".

How to read a test case

Every case uses this fixed structure:

Field	Meaning
Test ID	Stable identifier. Quote it when reporting a defect.
Feature	The feature under test.
Role	The account you must be signed in as.
Priority	P0 critical / P1 high / P2 medium / P3 low.
Purpose	Exactly what is being verified.
Preconditions	Everything that must already exist. Usually names an earlier Test ID.
Test Data	The exact values to enter.
Steps	Numbered clicks/taps. Each step names one control and one value.
Expected Result	UI state, message text, navigation, data state, and cross-role visibility.
Cleanup	What to restore, or "leave in place for <Test ID>".

Conventions used in the steps

- **Tap** means click on desktop and tap on mobile. They are the same instruction.
- Control names are written exactly as they appear on screen, in **bold**: *Tap Sign In***.
- Values to type are written in `code`: *Enter ``qa.patient.a@example.test`` in the Email Address field.*
- Where a control's name is dynamic (a patient's name, a session's time), the step tells you how to identify it — for example, "the row whose **Patient** column reads `QA Patient A`".
- Money is shown to the user in rupees (₹) and stored in the database in **paise** (₹1 = 100 paise). Where a test names a stored figure, it names paise.
- All times in the application's own copy are Asia/Kolkata (IST) unless a screen says otherwise.

What "verify in the database" means

Some expected results reference database state. You do not need SQL for the ordinary path — the same fact is visible on an admin screen, and the test names that screen. Where a case genuinely needs the table (ledger append-only checks, RLS checks), it says so and gives the query, and it is marked as requiring Supabase SQL editor access.

2. Application overview

Dr. Pooja's Physio is a production web application for a physiotherapy practice. It has five surfaces:

1. **A public marketing site** — eight pages that explain the service and start a booking.
2. **A patient portal** — book, pay, attend, manage a health profile, answer therapist recommendations.
3. **A therapist portal** — availability, sessions, clinical records, recommendations, earnings.
4. **A hospital/partner portal** — refer patients, track referrals, see partner earnings.
5. **An admin back office** — six sections that run the clinic: Today, Sessions, People, Money, Catalog, Settings.

The business model in one paragraph

A patient's **first purchase is always one session** — a video consultation, or (if the clinic's home-visit switch is on and their pincode is serviceable) a single home visit. Multi-session programmes cannot be bought from a price list at all: a therapist must run a session, then write a **care plan** recommending an admin-configured package, and an admin must approve it before the patient sees anything. The patient then accepts and pays from their own dashboard. That purchase creates **session credits**, which are spent one at a time as sessions are booked and delivered. Money is split between the therapist (a revenue share, earned only on delivered sessions), the referring hospital (a commission on net revenue), and the clinic.

Integrations

Integration	What it does	Failure behaviour
Supabase	Postgres database, authentication, private file storage, realtime updates	Hard dependency. Nothing works without it.
Razorpay (test mode)	Payment orders, checkout, signature verification, webhooks, refunds	A failed payment leaves an unpaid booking the patient can retry.

Integration	What it does	Failure behaviour
Google Calendar / Meet	Creates the calendar event and video link for a confirmed session	Never blocks a booking. Failures are recorded on the appointment and retried.

Architectural facts a tester needs

- **There is no cron job and no background worker.** Anything that must happen "when time passes" — a package expiring, a failed Meet sync being retried, risk detection — runs as a bounded sweep at the top of a relevant page render. **Consequence for testing: a time-based state change may not appear until you load the page that owns the sweep.** Each affected test names that page.
- **Email confirmation is deliberately OFF.** A sign-up returns a session immediately. If any sign-up path ever shows you a "check your email" step, that is a defect (see `SEC-AUTH-004`).
- **The public pages are ISR-cached for 300 seconds.** Against a production server (`next start`), a catalog row you just created may not appear on a public page for up to five minutes. Run the tests against `next dev`, where nothing is cached. This is not a bug in the application; it is a property of the server you point at.
- **The Debug Bar is on in every environment on purpose** and is deleted (not switched off) before real launch.

3. Application route map

Every route below is covered by at least one test. The rightmost column names the first test that opens it.

3.1 Public marketing pages

Route	What it is	Auth	Covered by
<code>/</code>	Home. Hero, care-area showcase, walkthrough, programmes, testimonials, mission band, connector grid	Public	<code>PUB-HOME-001</code>
<code>/conditions</code>	What we treat, plus the programme cards per condition	Public	<code>PUB-COND-001</code>
<code>/how-it-works</code>	Booking to recovery in four steps	Public	<code>PUB-NAV-001</code>
<code>/home-visit</code>	Home-visit landing page. 404s while the admin master switch is off	Public	<code>PUB-HV-001</code>
<code>/team</code>	Therapist profiles; "Book with" carries the specialist into the wizard	Public	<code>PUB-TEAM-001</code>
<code>/mission</code>	Mission, vision, four promises, testimonials	Public	<code>PUB-NAV-001</code>
<code>/faq</code>	Admin-managed FAQ accordion	Public	<code>PUB-FAQ-001</code>
<code>/hospitals</code>	Partner pitch plus the hospital enquiry form	Public	<code>HOS-LEAD-001</code>

Route	What it is	Auth	Covered by
<code>/get-started</code>	Role hub — where a signed-in user of the wrong role is sent	Public	SEC-ROUTE-003

3.2 Booking

Route	What it is	Auth	Covered by
<code>/book</code>	The 3-step video-consultation booking wizard. Accepts <code>?category=</code> , <code>?therapist=</code> , and answers a stale <code>?package=</code>	Public (guest can sign up inside it)	PAT-BOOK-001
<code>/book-home-visit</code>	The 4-step home-visit wizard (pincode → when → who → pay)	Public	PAT-HV-001

3.3 Patient portal

Route	What it is	Covered by
<code>/patient/login</code>	Sign In / Register Account tabs plus Forgot password	PAT-AUTH-001
<code>/patient/register</code>	Standalone registration (always waits for admin approval)	PAT-AUTH-004
<code>/patient/dashboard</code>	Overview: four figures, activity feed, quick actions	PAT-DASH-001
<code>/patient/dashboard/booking</code>	Booking hub inside the portal	PAT-DASH-002
<code>/patient/dashboard/suggested</code>	Suggested Sessions — therapist recommendations and proposed times. Only in the sidebar when something is waiting	PAT-SUGG-001
<code>/patient/dashboard/sessions</code>	All sessions, List/Calendar toggle, Upcoming/Past/Cancelled + Video/Home visit filters	PAT-SESS-001
<code>/patient/dashboard/packages</code>	Owned programmes and their remaining credits	PAT-PKG-001
<code>/patient/dashboard/payments</code>	Receipts	PAT-PAY-010
<code>/patient/dashboard/health-profile</code>	Health Profile: intake answers, snapshot, documents, care-plan history	PAT-HP-001
<code>/patient/dashboard/profile</code>	Edit Profile: photo, personal, contact, addresses, account security	PAT-PROF-001

3.4 Therapist portal

Route	What it is	Covered by
/therapist/login	Sign In / Apply to Join	THR-AUTH-001
/therapist/dashboard	Overview	THR-DASH-001
/therapist/dashboard/availability	Weekly schedule, exceptions, leave	THR-AVAIL-001
/therapist/dashboard/sessions	Sessions, join, complete, notes, recommend	THR-SESS-001
/therapist/dashboard/earnings	Earnings, payout requests, receipts	THR-EARN-001
/therapist/dashboard/health-profile	My Patients (Patients / Programmes toggle)	THR-PAT-001
/therapist/dashboard/health-profile/[patientId]	One patient's chart: intake, Pain Map, care plans, notes	THR-HP-001
/therapist/dashboard/profile	Edit Profile	THR-PROF-001

3.5 Hospital portal

Route	What it is	Covered by
/hospital/login	Sign In	HOS-AUTH-001
/hospital/dashboard	Overview	HOS-DASH-001
/hospital/dashboard/refer	Refer a Patient form	HOS-REF-001
/hospital/dashboard/referrals	Your Referrals with status	HOS-REF-004
/hospital/dashboard/revenue	Earnings (partner share)	HOS-MONEY-001
/hospital/dashboard/profile	Edit Profile	HOS-PROF-001

3.6 Admin back office

/admin/login and /admin/dashboard . The dashboard is one page; the screen is chosen by ?section=&tab= . All 28 screens:

Section	Tab key	Screen	Covered by
Today	overview	Today	ADM-TODAY-001
Today	approvals	Approvals	ADM-APPR-001
Today	risk	Risk	ADM-RISK-001
Sessions	schedule	Schedule (calendar)	ADM-SCHED-001
Sessions	all	All Sessions	ADM-SESS-001

Section	Tab key	Screen	Covered by
Sessions	roster	Roster	ADM-ROST-001
Sessions	delivery	Delivery (operational rates)	ADM-DELIV-001
Sessions	recommendations	Recommendations — the clinic's review queue, plus every plan	ADM-CARE-001 , ADM-CARE-004
Sessions	new	New Booking	ADM-NEWB-001
People	patients	Patients (+ condition requests)	ADM-PEOP-001
People	therapists	Therapists	ADM-PEOP-005
People	partners	Partners	ADM-PEOP-008
Money	summary	Summary	FIN-SUM-001
Money	transactions	Transactions	FIN-TXN-001
Money	payouts	Payouts + payout requests + Cash Ledger	FIN-PAY-001
Money	costs	Costs	FIN-COST-001
Money	breakdown	Breakdown	FIN-BRK-001
Catalog	conditions	Conditions	ADM-CAT-001
Catalog	packages	Packages	ADM-CAT-005
Catalog	areas	Service Areas + waitlist	ADM-CAT-010
Catalog	purchases	Purchases	ADM-CAT-014
Settings	brand	Brand & Contact	ADM-SET-001
Settings	public	Public Site	ADM-SET-004
Settings	booking	Booking Rules	ADM-SET-010
Settings	clinical	Clinical Questions	ADM-SET-020
Settings	team	Team & Access	ADM-SET-025
Settings	health	System Health	ADM-SET-030
Settings	activity	Activity Log	ADM-SET-033
Settings	security	Account Security	ADM-SET-035

Detail routes (open as an overlay from the dashboard, and as a full page on direct navigation):

Route	Covered by
/admin/dashboard/patients/[id]	ADM-PEOP-003
/admin/dashboard/therapists/[id]	ADM-PEOP-006
/admin/dashboard/conditions/[id]	ADM-PEOP-004

3.7 System routes

Route	What it is	Covered by
/dashboard	Server-side role router. Sends each role to its own dashboard. The admin path never reaches a public bundle.	SEC-ROUTE-006
/pending-approval	Where an unapproved patient/therapist lands	PAT-AUTH-003
/account-suspended	Where a suspended account lands	SEC-AUTH-006
/reset-password	Password reset landing	PAT-AUTH-006

3.8 API routes

The application exposes 150+ POST route handlers under `/api`, grouped by audience: `admin/`, `appointments/`, `patient/`, `therapist/`, `hospital/`, `packages/`, `home-visit/`, `care-plan/`, `razorpay/`, and `medical-documents/`. Individual routes are named inside the tests that exercise them. The security section (`SEC-API-*`) tests them directly with `curl`.

4. Roles, gates and scopes

4.1 The four roles

`profiles.role` is a single column with one of four values. **One account carries exactly one role.** A therapist account can never also be a patient — the booking wizards refuse it, and so do the purchase routes.

Role	How an account is created	Gate before it can be used
patient	Self-registers (at <code>/patient/register</code> , or inside either booking wizard)	<code>approved</code> + <code>active</code> . Exception: a genuine payment attempt auto-approves a patient (see below).
therapist	Self-applies at <code>/therapist/login</code> → Apply to Join	Admin approval, then <code>active</code>
hospital	Provisioned by an admin (People → Partners → onboard)	<code>active</code>
admin	Promoted by hand in Supabase, or minted by a Master Admin	<code>active</code> only — <code>approved</code> is deliberately not checked for admins

4.2 The two flags

`profiles.approved` and `profiles.active` are enforced in **two** places, and both matter:

1. ``src/proxy.ts`` — blocks dashboard *navigation*.
2. ``requireActiveProfile`` inside self-service API routes — blocks a valid session cookie calling the API around the UI.

A test that only proves the UI hides something has not proved the rule. Every authorization test in this plan has an API-level twin.

4.3 The payment-attempt approval rule (important, and easy to mis-report)

For a **single online session**, `/api/razorpay/create-order` flips the paying patient's `approved` to `true` the moment they *genuinely attempt* checkout — on the attempt, not on a completed payment. This is deliberate: a patient whose card fails three times still lands in their dashboard with a pending appointment rather than being bounced to `/pending-approval`.

It does **not** apply to:

- home-visit purchases (`/api/home-visit/create-order` — a *completed* payment vets you), or
- standalone registration at `/patient/register` (always waits for a human admin).

4.4 Admin scopes

`profiles.admin_scope` is one of four values. It decides which **sections** an admin may open. **Every** admin route guards on scope — 92 of the 95 with `requireAdminScope(section)`, and three (`set-admin-scope`, `debug-reset`, `create-account`) with an explicit **full-only** check instead, because a section check would be too weak: a `finance` admin passing a section gate could otherwise widen its own access or mint a full admin. The sidebar hiding a section is presentation only.

Scope	Sections it can open	Cannot
<code>full</code>	Today, Sessions, People, Money, Catalog, Settings	—
<code>operations</code>	Today, Sessions, People, Catalog	Money, Settings
<code>finance</code>	Today, People, Money	Sessions, Catalog, Settings
<code>clinical</code>	Today, Sessions, People	Money, Catalog, Settings

Rules that must hold (tested in `ADM-SET-025` – `ADM-SET-029`):

- Only a `full` admin can change scopes or create another admin.
 - **Nobody can change their own scope.**
 - **The last `full` admin cannot be narrowed.**
 - An unknown/null scope reads as `full` (so a migration can never lock everyone out).
 - The **Risk** queue is `full`-only — a scoped admin's page does not even fetch it.
-

5. Test environment requirements

5.1 What you need before you start

Requirement	Value / note
A throwaway Supabase project	Never a project holding real data. The reset in Step 0 truncates every table.
<code>supabase/schema.sql</code> applied to it	<code>node scripts/run-schema.mjs</code> , or push to <code>main</code> and let <code>.github/workflows/schema-apply.yml</code> run it. Re-apply twice to confirm it is re-runnable.
Supabase Auth → Confirm email = OFF	The application assumes this. With it on, every sign-up path fails.
Razorpay test-mode keys	<code>RAZORPAY_KEY_ID</code> / <code>RAZORPAY_KEY_SECRET</code> from the Razorpay dashboard in Test Mode . Never live keys.
<code>RAZORPAY_WEBHOOK_SECRET</code> set	Without it <code>/api/razorpay/webhook</code> answers <code>503 {"error": "Webhook not configured"}</code> and a patient who pays and closes the tab leaves a paid order against an unpaid booking. Webhook tests need it.
<code>ALLOW_DEBUG_DATA_RESET=true</code>	Server-side only. Arms the Reset data button. Never set on a live site.
<code>NEXT_PUBLIC_SHOW_DEBUG_NAV</code>	Leave unset. The Debug Bar must be visible.
Google Calendar credentials	Optional. Without them Meet sync fails and is recorded — which is itself a test (<code>ADM-SET-031</code>).
The app running with <code>`npm run dev`</code>	Not <code>next start</code> . The public pages are ISR-cached for 300s, so a production build serves HTML that predates your fixtures.
At least one Master Admin (<code>admin_scope = 'full'</code>) in <code>profiles</code>	The reset keeps admin logins and refuses to run if it would leave none.
Browsers	Chrome/Edge desktop at 1440×900, plus a mobile viewport at 390 × 844 .
Supabase SQL editor access	Only needed for the handful of tests marked [SQL] .

5.2 Environments

Environment	May be reset?	Notes
Local <code>npm run dev</code> against a throwaway Supabase project	Yes	The default target for this plan.
Staging/preview deployment against a throwaway project	Yes , if <code>ALLOW_DEBUG_DATA_RESET=true</code> is set there	Public pages are cached; expect ISR delays.
Production / any project with real patients	NEVER	<code>ALLOW_DEBUG_DATA_RESET</code> must be unset there, which makes the reset route answer <code>404</code> .

6. STEP 0 — Reset the test environment

Run this first, before anything else in this plan. Every numbered section after this assumes a clean database plus the setup in Section 12 (Execution Order).

6.1 Why the database must be reset

Testing this application means filling it with throwaway patients, bookings, purchases and payouts. Several rules in the product are **one-per-thing** rules — one open care plan per patient (published or awaiting the clinic), one pending suggestion per purchase, one open risk signal per rule+subject, a unique Razorpay order id — so leftover rows from a previous run make a correct application look broken. Two known examples:

- A booking test that books a fixed slot leaves the appointment behind; the next run's identical booking is refused as a clash. That refusal is **correct behaviour**, not a defect.
- An audit-log count assertion picks up the previous run's writes.

If a test fails on a second consecutive run but passes on a fresh one, suspect leftover state before suspecting the application.

6.2 What the reset removes and keeps

The Reset data button calls `/api/admin/debug-reset`, which calls the database function `debug_reset_all_data()`. It is **one atomic `TRUNCATE`**, not a list of deletes.

Removed: every appointment, package purchase, home-visit purchase, payment, payment webhook event, payment failure log, entitlement and credit ledger row, session note and revision, pain assessment, condition profile, condition change request and access grant, patient address, medical-document metadata row, admin notes, profile change request, availability template and override, referral, B2B lead, home-visit waitlist, service area, both package catalogs, treatment categories, testimonials, FAQs, intake question templates, payout requests and batches, business expenses, session suggestions, care plans, their versions and the clinic's reviews of them, risk signals and reviews, communication flags, contact reveal log, and the admin activity log. `site_settings` is put back to its defaults. **Every non-admin account is deleted.**

Kept:

- **Admin logins** — the function refuses to run if it would leave no admin behind.
- **Detector thresholds** (`risk_rules`) — configuration, like `site_settings`, so it is **reset to its seeded defaults** rather than emptied. Emptying it would silently disable every detector instead of restoring it.
- **Objects in the private `medical-reports` Storage bucket.** The metadata rows go; the files do not. Storage is not reachable from SQL. Clear that bucket from the Supabase dashboard if you need the space back.

Regression worth knowing about. An earlier version of this function predated the care-plan, evidence and risk tables and cleared none of them; three were saved by CASCADE, but `communication_flags`, `risk_signals` and `risk_reviews` survived a "delete everything". The one that actually bit a tester was `risk_signals`: it carries a partial unique index allowing at most one **open or reviewing** signal per `(rule, subject)`, so a leftover open signal held the slot and the same rule firing again wrote nothing — an empty Risk queue on a supposedly clean database, which reads exactly like a broken detector. All three are now named in the TRUNCATE list. `SETUP-RESET-001` asserts it.

6.3 The four gates (all must pass)

1. `ALLOW_DEBUG_DATA_RESET=true` in the **server** environment. Unset, the route answers ``404``, not ``403`` — deliberately, so the route's existence is not confirmed.
2. A signed-in admin.
3. That admin's scope must be `full`.
4. The exact phrase `RESET ALL DATA`, typed by hand.

TEST: `SETUP-RESET-001` — Reset the test environment

Feature	Pre-launch data reset
Role	Admin (full scope)
Priority	P0

Purpose. Bring the database to a known-empty state, and prove all four gates behave.

Preconditions. `ALLOW_DEBUG_DATA_RESET=true` in the server environment. A Master Admin account exists.

Test Data. Admin: `qa.admin@example.test` / `QaTest!2024pass`. Confirmation phrase: `RESET ALL DATA`.

Steps

1. Open `http://localhost:3000/admin/login`.
2. Tap the **Email Address** field. Enter `qa.admin@example.test`.
3. Tap the **Password** field. Enter `QaTest!2024pass`.
4. Tap **Sign In**.
5. Confirm the black **Debug** bar is pinned across the top of the page.
6. In the Debug bar, tap **Reset data**.
7. Read the red warning that appears: *"Deletes everything — people, sessions, purchases, catalog, settings. Admin logins survive. No undo."*
8. Tap the confirmation text field (its placeholder reads `RESET ALL DATA`). Enter `reset all data` (lower case, deliberately wrong).
9. Observe the **Reset** button.
10. Clear the field. Enter `RESET ALL DATA` exactly.
11. Tap **Reset**.
12. Wait for the button to stop reading **Resetting...**

Expected Result

- Step 7: the warning text is present and names admin survival explicitly.
- Step 9: the **Reset** button is **disabled** (visibly faded) while the typed phrase does not match exactly. A wrong-case phrase never arms the button.
- Step 12: the button returns to normal and a teal confirmation message appears in the bar. No error message.
- The page refreshes. The admin remains signed in — the session is not destroyed.
- Navigating to **People → Patients** shows an empty-state message, not a table of rows.
- Navigating to **Catalog → Conditions** shows no treatment categories.
- Navigating to **Sessions → All Sessions** shows no sessions.

- Navigating to **Settings → Activity Log** shows an empty log (the reset itself truncates it).
- Navigating to **Settings → Team & Access** still lists at least one admin, and your own row is there. **If this list is empty, stop immediately and restore from backup — the reset must never leave the clinic without an admin.**
- Navigating to **Today → Risk** shows an **empty** queue. **[SQL]** confirm with `select count(*) from communication_flags;` and `select count(*) from risk_signals;` — both must return `0`. A non-zero count here is the regression described above, and it will silently suppress the detector tests later in this plan.
- **[SQL]** `select rule_key, enabled from risk_rules;` still returns the eight rules, with `plan_conversion_low` and `post_consultation_dropout` back to **disabled** — thresholds are restored to their seeded defaults, not wiped.

Cleanup. None. This is the starting state for the whole plan.

TEST: `SETUP-RESET-002` — The reset is refused for a non-full admin

Feature	Pre-launch data reset — gate 3
Role	Admin (operations scope)
Priority	P1

Purpose. Prove that scope, not merely being an admin, gates the wipe.

Preconditions. `ADM-SET-026` has created `qa.admin.ops@example.test` with scope **Operations**.

Steps

1. Sign out of the full admin account.
2. Sign in at `/admin/login` as `qa.admin.ops@example.test` / `QaTest!2024pass`.
3. In the Debug bar, tap **Reset data**.
4. Enter `RESET ALL DATA` in the confirmation field.
5. Tap **Reset**.

Expected Result

- A red error appears in the bar reading exactly: `Only a Master Admin can reset data.`
- No data is deleted. **People → Patients** still lists the same rows as before step 5.
- No `admin_activity_log` row is written for a reset.

Cleanup. Sign back in as the full admin.

TEST: `SETUP-RESET-003` — The reset route is invisible when disarmed [server config]

Feature	Pre-launch data reset — gate 1
Role	Admin (full scope)
Priority	P1

Purpose. Prove the route answers `404` (not `403`) when `ALLOW_DEBUG_DATA_RESET` is unset, so its existence is not confirmed to a caller.

Steps

1. Stop the dev server. Remove `ALLOW_DEBUG_DATA_RESET` from the environment. Start the dev server again.
2. Sign in at `/admin/login` as the full admin.
3. In the Debug bar, tap **Reset data**, enter `RESET ALL DATA`, tap **Reset**.

Expected Result

- The bar shows an error. The browser **Network** tab shows `POST /api/admin/debug-reset` returning **HTTP 404** with body `{"error": "Not found"}`.
- **It must not return 403.** A 403 would confirm the route exists.
- No data is deleted.

Cleanup. Restore `ALLOW_DEBUG_DATA_RESET=true` and restart the dev server before continuing.

7. The Debug Bar and time simulation

7.1 What the Debug Bar is

A black bar pinned to the top of **every** page, in **every** environment. It exists because this application is pre-launch. It carries three tools:

Control	What it does
Jump to page dropdown	Navigates to any of the app's main routes, including the four protected dashboards. Entries 1-8 are the public pages; 9-15b are Get Started, both booking wizards, and each role's login and dashboard.
Simulate now (<code>datetime-local</code> input) + Set	Sets a simulated clock.
Reset to Real Time	Appears only while a simulation is active. Clears it.
Reset data	The wipe covered in Step 0.

The right-hand label reads *"Remove this bar before real launch"*. That is the intended end state: the bar is **deleted**, not switched off, because `NEXT_PUBLIC_SHOW_DEBUG_NAV` is a public flag and the bar names `/admin/login` and `/admin/dashboard`.

7.2 How the simulated clock actually works — read this before writing a defect

This is the single most misunderstood part of the application, and mis-reading it produces false defects.

The simulated clock is stored as an OFFSET, not as a fixed target. When you set "12 September 2026, 18:00", the app stores `target - Date.now()` in `localStorage` under `debugNowOffsetMs`. The simulated clock then keeps **ticking forward at normal speed**. It never freezes.

It is client-side only. It affects only client-rendered advisory gates that call `debugNow()` instead of `Date.now()`. It is **deliberately never wired into any server-side or API-route time check.**

Affected by the simulated clock	NOT affected (uses the server's real clock)
The <code>/book</code> wizard's date calendar and hour list (which dates/times are offered)	<code>/api/appointments/create</code> 's lead-time validation
The <code>/book-home-visit</code> wizard's date/time picker	<code>/api/home-visit/*</code> lead-time validation
Tap to Join / Session Completed button states on every dashboard	<code>/api/appointments/complete-session</code> 's join-window gate
Session card greying and Upcoming/Past bucketing in the browser	Refund-eligibility maths in <code>/api/appointments/cancel</code>
The therapist's own suggestion picker	<code>/api/therapist/suggest-session</code> 's lead-time check
	Payout maths, <code>completed_at</code> stamping, audit timestamps, <code>paid_at</code>

The practical consequence, stated plainly: you can use the simulated clock to make the *UI offer* a slot or a button. You cannot use it to make the *server accept* a time-gated write. If you simulate a date far in the future and then try to complete a session, the client will show you the **Tap to Join** control and the server will still answer `409` with *"You can mark this done once the session's join window has opened."* **That is correct behaviour, not a defect.** Tests that need a server-side time gate to pass say so explicitly and tell you to use a real near-future slot instead.

Because the storage key is `localStorage`, the simulation is **per browser profile**, and it survives navigation and reload until you reset it. Applying it triggers a **full page reload** — soft re-renders would not pick it up, because every consumer reads the clock once in a lazy initializer.

7.3 How to use it

To set a simulated time

1. Tap the **Simulate now** field in the Debug bar.
2. Type or pick the date and time **in your browser's local timezone** (the field is `datetime-local`, so it is not UTC).
3. Tap **Set**.
4. The page reloads. A **Reset to Real Time** button appears beside the field.

To verify the app is using simulated time (do this before every time-dependent test)

1. After Step 4 above, navigate to `/book`.
2. Look at the Step 1 calendar. The month shown and the highlighted "today" cell must match your simulated date, not the real date.
3. Alternatively, open the browser console and run `localStorage.getItem('debugNowOffsetMs')`. A non-zero number means a simulation is active. `null` means real time.

To reset

1. Tap **Reset to Real Time**. The page reloads and the button disappears.
2. Confirm `localStorage.getItem('debugNowOffsetMs')` returns `null`.

Always reset to real time at the end of a time-simulation test. A left-over offset silently changes every later test's calendar and every join-button state, and the failures it produces look like product bugs.

7.4 Standard simulated-time scenarios

These are referenced by ID throughout the plan.

Scenario	Simulate	What it is for	Related tests
TIME -A	2026-09-10 10:00	Baseline booking. Online lead time is 12h, so the earliest offered slot is 10 September 22:00.	PAT-B00K-002 , PAT-B00K-003
TIME -B	2026-09-12 18:00	Same rules, different day-of-week and a later hour, so the boundary lands on the <i>next</i> day. Proves the boundary is computed, not hardcoded.	PAT-B00K-004
TIME -C	2026-09-10 23:30	Late-night boundary: no slot remains today, so the calendar's earliest bookable date must roll to 11 September.	PAT-B00K-005
TIME -D	Real clock + 24h ahead of a home-visit slot	Home-visit lead time defaults to 24h — longer than online. Proves the two lead times are separate settings.	PAT-HV-003
TIME -E	10 minutes before a confirmed session's slot	The join window opens <code>join_window_minutes</code> (default 15) before the slot. Tap to Join must be live.	THR-SESS-004 , PAT-SESS-005
TIME -F	90 minutes after a confirmed session's slot	Past <code>session_completed_after_minutes</code> (default 60). Every join control on every surface must read Session Completed .	XR-CUTOFF-001
TIME -G	30 hours before a paid session's slot	Outside the 24h cancellation window → full refund path.	PAT-CANCEL-001
TIME -H	2 hours before a paid session's slot	Inside the 24h window → no refund, and the confirm dialog must say so.	PAT-CANCEL-002

TEST: `DBG-TIME-001` — The simulated clock changes what the booking picker offers

Feature	Debug Bar — time simulation
Role	Public (no sign-in needed)
Priority	P1

Purpose. Prove the simulation is applied, is an offset (not a freeze), and is reversible.

Steps

1. Open `/book` with no simulation active. Note the earliest date highlighted in the Step 1 calendar and the first hour offered. Write both down.
2. In the Debug bar, set **Simulate now** to `2026-09-10T10:00` (scenario TIME-A). Tap **Set**.
3. After the reload, open `/book`.

- Note the calendar month, the "today" cell, and the first hour offered.
- Open the browser console and run `localStorage.getItem('debugNowOffsetMs')`.
- Wait 60 seconds. Reload `/book`. Note the first offered hour again.
- Tap **Reset to Real Time**.
- Open `/book` again.

Expected Result

- Step 4: the calendar shows **September 2026**, the "today" cell is **10**, dates before 10 September are not selectable, and the earliest selectable slot is **10 September 2026 at 22:00** — twelve hours after the simulated now, matching the 12-hour online booking lead time.
- Step 5: returns a non-zero numeric string.
- Step 6: the offering is **not frozen**. The clock advanced by roughly a minute, so the picker's boundary has moved forward too (at a minute's granularity you will usually see the same 22:00 hour offered, but the *underlying* now has advanced — confirm by re-reading the "today" cell and by the fact that at a 23:00+ simulated time the day rolls over, per TIME-C).
- Step 8: the calendar returns to the real current month and the values you wrote down in step 1.
- At no point does any figure change on an **admin** money screen or an appointment's stored `slot_time` — the simulation is display/advisory only.

Cleanup. Confirm `localStorage.getItem('debugNowOffsetMs')` is `null`.

TEST: `DBG-NAV-001` — Jump to page reaches every listed route

Feature	Debug Bar — route jump
Role	Any
Priority	P2

Purpose. Smoke-test that every route in the dropdown renders without a crash, and that the protected ones enforce their gate.

Steps

- Sign out completely (clear cookies or use a private window).
- Open `/`.
- Tap the **Jump to page** dropdown. Select each entry in turn, from `1. Home` to `15b. Partner Dashboard (protected)`.
- After each, note the resulting URL and whether the page renders content or an error screen.

Expected Result

- Entries 1-8 (the eight public marketing pages) render normally — **except** `4. Home visit`, which shows a **404** page while the admin master switch `home_visit_enabled` is off. That 404 is correct and is the point of including the entry.
- `9. Get Started Hub`, `10. Booking Enquiry`, `10b. Home Visit Booking`, `11/12/14/15` login pages all render.
- Every entry ending in `b` (the four protected dashboards) redirects a signed-out visitor to that role's own login page: `/patient/login`, `/therapist/login`, `/admin/login`, `/hospital/login`.
- No entry produces a blank page, an unhandled error screen, or a stack trace.

Cleanup. None.

8. Test Data Library

Use these exact values everywhere. Every test in this plan refers to them by label (for example "Patient A"). All emails use the reserved `.test` TLD so nothing can be delivered to a real inbox.

8.1 Standard password

All test accounts use the same password: `QaTest!2024pass`

Where a test needs a *second, different* password (a change-password test), use `QaTest!2024new`.

8.2 Admin accounts

Label	Email	Scope	Purpose
Admin Full	<code>qa.admin@example.test</code>	<code>full</code>	The main admin. Survives the reset.
Admin Ops	<code>qa.admin.ops@example.test</code>	<code>operations</code>	Proves Money and Settings are blocked.
Admin Finance	<code>qa.admin.finance@example.test</code>	<code>finance</code>	Proves Sessions and Catalog are blocked.
Admin Clinical	<code>qa.admin.clinical@example.test</code>	<code>clinical</code>	Proves Money, Catalog and Settings are blocked.

Admin Full is created by hand in Supabase before Step 0 (set `role='admin'`, `active=true`, `admin_scope='full'`). The other three are created from **Settings → Team & Access** in `ADM-SET-026`.

8.3 Patients

Field	Patient A (main journey)	Patient B (isolation/negative)	Patient C (hospital-referred)
Full name	<code>QA Patient A</code>	<code>QA Patient B</code>	<code>QA Referred Patient C</code>
Email	<code>qa.patient.a@example.test</code>	<code>qa.patient.b@example.test</code>	<code>qa.patient.c@example.test</code>
Password	<code>QaTest!2024pass</code>	<code>QaTest!2024pass</code>	<code>QaTest!2024pass</code>
Phone	<code>+91 98765 43210</code>	<code>+91 98765 43211</code>	<code>+91 98765 43212</code>
Date of birth	<code>1990-04-12</code>	<code>1985-11-30</code>	<code>1978-02-05</code>
Gender	<code>Female</code>	<code>Male</code>	<code>Female</code>
Address line 1	<code>12, 3rd Cross, Indiranagar</code>	<code>44 Residency Road</code>	<code>8, 100 Feet Road, Indiranagar</code>
Address line 2	<code>Near Metro Station</code>	<code>(leave blank)</code>	<code>Above the pharmacy</code>

Field	Patient A (main journey)	Patient B (isolation/negative)	Patient C (hospital-referred)
City	Bengaluru	Bengaluru	Bengaluru
State	Karnataka	Karnataka	Karnataka
PIN code	560038 (serviceable)	560025 (not a service area)	560038
Emergency contact	QA Contact A , +91 98765 43299	(leave blank)	QA Contact C , +91 98765 43298
Referral code	(blank)	(blank)	The code from Hospital A (HOS-AUTH-002)
Concern	Lower back pain	Knee pain after running	Post-stroke weakness, right side
Booking notes	Desk job, pain worse after sitting all day. Goal: sit through a full workday.	Pain on stairs for six weeks.	Discharged last week, needs home programme.

Negative-test values (used to prove validation, never to create an account):

Purpose	Value
Invalid email	not-an-email
Too-short password	abc12 (5 characters; minimum is 6)
Mismatched confirm password	Password QaTest!2024pass , confirm QaTest!2024pas
Invalid phone	12345
Invalid PIN code	0560038 (leading zero — the pattern requires [1-9] first) and 56003 (5 digits)
Unknown referral code	ZZZZZZ
Contact-leak block text	Pay me on UPI 9876543210@okhdfc instead
Contact-leak flag text	Call me on 9876543210
Clinical text that must not flag	Grade III PA mobilisation x3 sets, 30s hold. Repeat 10 reps, twice daily.

8.4 Clinical answers — Orthopaedic (Patient A)

The orthopaedic intake is **seven questions**. Question keys are fixed and globally unique.

Question (as shown)	Key	Answer to enter
What's the main issue you'd like help with?	chief_complaint	Lower back pain that spreads into my right hip
How long has this been going on?	since_when	About four months
Overall severity right now (0-10)	severity	6
Where does it hurt? (tap each area, rate 0-10)	area_pain	Tap Lower back → rate 7 ; tap Right hip → rate 5

Question (as shown)	Key	Answer to enter
What makes it worse?	worsens	Sitting for more than an hour, and bending to pick things up
What helps or relieves it?	helps	Walking, and lying flat for ten minutes
Anything else the therapist should know?	notes	I work at a desk nine hours a day. No previous surgery.

8.5 Clinical answers — Neurological (Patient C)

All neurological keys are prefixed `neuro_`.

Question	Key	Answer
What is the neurological condition or event, and when did it start?	neuro_diagnosis	Ischaemic stroke, six weeks ago
Which part of the body is affected?	neuro_affected_side	Right side
How do you move around indoors right now?	neuro_mobility	With a walking stick and someone nearby
Day-to-day independence right now (0-10)	neuro_independence	4
Which of these are present?	neuro_symptoms	Tick Weakness on one side and Difficulty with balance or walking
Falls in the last three months?	neuro_falls	One
What would you most like to be able to do again?	neuro_goal	Walk to the end of my street without help

8.6 Clinical answers — Paediatric (Patient D, if used)

Paediatric keys are prefixed `peds_`. **The two caregiver fields are a pre-step, not part of the seven-question count** — the answered counter must never include them.

Question	Key	Answer
Your name	peds_caregiver_name	QA Caregiver D
How are you related to the child?	peds_caregiver_relationship	Mother
What is the main concern about your child?	peds_concern	He is not walking on his own yet at 20 months
How was your child born?	peds_birth_history	Born at 34 weeks, two weeks in special care
Which of these can your child do on their own today?	peds_milestones	Tick Sits without support and Pulls to stand
Has a doctor given a diagnosis, or ordered any tests?	peds_diagnosis	No diagnosis yet, an MRI is booked

Question	Key	Answer
Does your child use a brace, splint, walker, wheelchair or special footwear?	peds_equipment	Ankle splints on both feet
What is hardest for your child in a normal day?	peds_daily_difficulty	Standing long enough to play at the table
What would you most like your child to be able to do in the next few months?	peds_goal	Take a few steps holding my hand

8.7 Triage answers (therapist asks these at first contact)

Triage is four questions. Its answers are stored separately from the patient's own record and are never shown to the patient.

Question	For an ortho outcome (Patient A)	For a neuro outcome (Patient C)
How old is the patient?	18 to 64	65 or older
What brought them in?	Injury, strain or overuse	After a stroke, brain or spinal injury
Any of these present?	None of these	Weakness on one side + Difficulty with balance or walking
Any concern about milestones...?	(not shown — only appears when age is `Under 18`)	(not shown)

Expected suggestion: **Orthopaedic** for Patient A, **Neurological** for Patient C. The suggestion is shown with its reason and is **never auto-accepted** — the therapist confirms.

8.8 Therapists

Field	Therapist A (main)	Therapist B (isolation tests)	Therapist C (leave / spare)
Full name	QA Therapist A	QA Therapist B	QA Therapist C
Email	qa.therapist.a@example.test	qa.therapist.b@example.test	qa.therapist.c@example.test
Password	QaTest!2024pass	QaTest!2024pass	QaTest!2024pass
Phone	+91 90000 10001	+91 90000 10002	+91 90000 10003
Qualifications & License / Council Reg No.	MPT (Ortho), KSCP Reg 44821	MPT (Neuro), KSCP Reg 44822	BPT, KSCP Reg 44823
Specialty / display note	Spine and lower-limb rehabilitation	Stroke and neurological rehabilitation	Paediatric physiotherapy
Experience	9 years	12 years	5 years
Bio	Works with desk-based patients on posture-driven back pain.	Post-stroke gait and balance retraining.	Early-intervention paediatric care.
Revenue share % (set by admin)	60	55	50

Field	Therapist A (main)	Therapist B (isolation tests)	Therapist C (leave / spare)
Home-visit revenue share %	65	(leave unset — must fall back to 60/55)	(unset)
Weekly schedule	Mon-Fri 09:00–13:00 and 14:00–18:00	Mon-Fri 10:00–16:00	Tue/Thu 09:00–12:00
Date exception	2026-09-15 : 14:00–18:00 only, reason Clinic audit in the morning	—	—
Leave dates	—	—	2026-09-14 to 2026-09-18 , reason Annual leave
Timezone	Whatever the browser reports; the roster header states it	same	same

8.9 Hospitals / partners

Field	Hospital A (main)	Hospital B (isolation)
Organisation name	QA Sunrise Hospital	QA Lakeside Clinic
Contact person	QA Hospital Admin A	QA Hospital Admin B
Email	qa.hospital@example.test	qa.hospital.b@example.test
Phone	+91 80400 10001	+91 80400 10002
Address	18 Airport Road	5 Lake View Street
City / State / PIN	Bengaluru / Karnataka / 560017	Bengaluru / Karnataka / 560034
Revenue share %	10	12
Referral code	Generated at onboarding — write it down , Patient C needs it	Generated at onboarding

Referral payload (Hospital A → Patient C)

Field	Value
Patient Full Name	QA Referred Patient C
Session Type	Online (and Home visit for the second referral)
Address	8, 100 Feet Road, Indiranagar, Bengaluru
Preferred Language	English
Pincode	560038 (required only for a home-visit referral)
Medical Issue	Right-sided weakness following a stroke six weeks ago
Treatment Needed	Gait and balance retraining, twice weekly

8.10 Treatment categories (conditions)

Create these in **Catalog** → **Conditions**.

Category Name	Price (₹)	Session Length (min)	Order	Button Text
QA Back & Spine Care	1999	60	1	Book Assessment
QA Knee & Joint Care	1799	45	2	Book Assessment
QA Neuro Rehabilitation	2499	60	3	Book Assessment

Negative values for validation tests: Price 0 , Price -100 , Price abc , Session Length 0 , Order xyz .

8.11 Session packages (programmes)

Create in **Catalog** → **Packages**. Every one of these has session_count ≥ 2 , so **none of them is directly purchasable** — they can only reach a patient through a care plan. That is the rule under test, not a limitation of the fixtures.

Field	Package P1	Package P2	Package P3 (consultation)
Category	QA Back & Spine Care	QA Neuro Rehabilitation	QA Back & Spine Care
Package Name	QA Spine Recovery 6 Sessions	QA Neuro Rehab 8 Sessions	QA Single Session
Subtitle	Six weeks, one therapist, measured progress	Eight sessions of gait and balance work	One assessment
Description	A structured six-session block for persistent lower-back pain.	An eight-session neurological rehabilitation block.	A single 60-minute session.
What We Promise (one per line)	The same therapist every session / A written home programme / Progress measured, not guessed	The same therapist every session / Gait retraining / Family guidance	A full assessment
Sessions Included	6	8	1
Bundle Price (₹)	9999	17999	1999
Compare-at Price (₹)	(blank — auto-computes from the category price)	(blank)	(blank)
Therapist Pay Basis	Discounted package price	Category list price	Discounted package price
Validity (days)	90	120	30
Session Duration (min)	(blank — inherits 60)	(blank)	(blank)
Minimum gap between sessions (hours)	24	48	(blank)

Field	Package P1	Package P2	Package P3 (consultation)
Maximum sessions per week	3	2	(blank)
Maximum purchases per patient	2	1	(blank)
Display Order	1	2	3
Active	ticked	ticked	ticked

Package validation negatives (expected error text in brackets):

Input	Expected message
Package Name blank	Package Name is required.
Sessions Included 1	Sessions Included must be a whole number of 2 or more.
Bundle Price 0	Bundle Price must be a positive number.
Compare-at 5000 with Bundle 9999	Compare-at Price can't be lower than the Bundle Price.
Order abc	Order must be a number.

8.12 Home-visit packages

Create in **Catalog → Packages** (home-visit section).

Field	HV1 (consultation)	HV2 (programme)
Package Name	QA Home Visit – Single	QA Home Visit Recovery – 4 Visits
Subtitle	One visit at your door	Four visits over a month
Description	A single home assessment.	A four-visit home rehabilitation block.
Benefits (one per line)	A physiotherapist at your door / Full assessment	The same therapist each visit / Family training
Visits Included	1	4
Package Price (₹)	2499	8999
Visit Duration (minutes)	60	60
Validity (days)	30	90
Minimum gap between visits (hours)	(blank)	48
Maximum visits per week	(blank)	2
Travel fee included in price	unticked	unticked
Lock to one therapist	ticked	ticked

Field	HV1 (consultation)	HV2 (programme)
Active	ticked	ticked

HV1 is the only home-visit row that may be bought directly (one visit = a consultation). HV2 (`visit_count > 1`) must be refused by both `/api/home-visit/create-order` and `/api/home-visit/book-cash` .

8.13 Service areas (home visit)

Create in **Catalog** → **Service Areas**.

Field	Area 1	Area 2
City	Bengaluru	Bengaluru
Area name	Indiranagar	Koramangala
Travel fee (₹ per visit)	150	200
Pincodes	560038	560095
Notes	Core service area	Second phase

Not serviceable (use for the negative path): `560025` . **Invalid formats:** `0560038` , `56003` , `abcdef` .

8.14 Documents (patient uploads)

Create small dummy files locally. Content does not matter; the filename and type do.

Filename	Type to pick in the uploader	Purpose
Spine_Report_E2E.pdf	Scan or X-ray	Happy path
Posture_Assessment_E2E.pdf	Lab report	Second upload
Discharge_Note_E2E.pdf	Hospital summary	Third upload
Referral_Letter_E2E.pdf	Referral letter	Fourth
Knee_Xray_E2E.jpg	Scan or X-ray	Image path (browser re-compresses images before upload)
Oversize_Report_E2E.pdf	Scan or X-ray	Must be larger than 10 MB. Create with: <code>head -c 11000000 /dev/urandom > Oversize_Report_E2E.pdf</code>
Not_Allowed_E2E.txt	(the picker will not accept it)	Wrong MIME type

Caps: 10 MB per file, **20 files per patient**. Both are enforced in the upload route, which is the only writer.

8.15 Business expenses (Money → Costs)

Description	Category	Amount (₹)	Incurred on
QA Clinic rent September	(pick the rent/premises category)	25000	2026-09-01
QA Software subscriptions	(pick the software/tools category)	4000	2026-09-03
QA Marketing test spend	(pick the marketing category)	6000	2026-08-28 (deliberately outside a September range, to test date filtering)

8.16 Free-text fixtures for the contact-leak scanner

Text	Expected tier	Where to enter it
Pay me directly on 9876543210@okhdfc, it's cheaper	block — the write is refused	Therapist's care-plan <i>Why this, for this patient</i>
https://rzp.io/l/abcd1234 pay here	block	Therapist's suggestion note
Call me on 9876543210 before the session	flag — delivered, and recorded	Therapist's suggestion note
Email me at therapist@example.test	flag	Care-plan <i>Anything they should do or know</i>
Grade III PA mobilisation ×3 sets, 30s hold. 10 reps, 2× daily. Order ref 90210.	no hit — clinical text with digits must not fire	Session note
Call me on 9876543210 written by the patient	record only — never blocked	Patient's booking notes on /book

8.17 Admin settings this plan assumes

Unless a test says otherwise, leave every setting at its default. The four that matter most for setup:

Setting	Default	Note
Therapist-Suggested Sessions	on on a fresh database	Needed by THR-SUGG-* and PAT-SUGG-*. On a database that predates the change it stays at its old value — confirm the toggle before running those tests
Assign a Therapist Automatically	off	ADM-SET-021 switches it on; several booking tests assume the queue behaviour while it is off
Session Balances From The Ledger	off	ADM-SET-019
Home Visit	off	ADM-SET-013 switches it on for the home-visit journey

8.18 Setup aliases used in preconditions

Several tests name a setup step by a SETUP-* alias. Each is simply the catalog test that creates that fixture:

Alias	Is	Creates
SETUP-CAT-001	ADM-CAT-001	The three treatment categories (§8.10)
SETUP-PKG-001	ADM-CAT-005	Packages P1, P2, P3 (§8.11)
SETUP-HVPKG-001	ADM-CAT-005 (home-visit section)	HV1 and HV2 (§8.12)
SETUP-AREA-001	ADM-CAT-010	Service Areas 1 and 2 (§8.13)

9. Payment test data and the payment model

9.1 Razorpay test mode — credentials

Use Razorpay Test Mode only. Test cards fail in live mode with an "invalid card issuer" error, which is the intended safety net. Before executing this section, re-confirm the values against Razorpay's own pages, because Razorpay changes them from time to time: *Test cards:*

`<https://razorpay.com/docs/payments/payments/test-card-details/>` Test UPI:

<https://razorpay.com/docs/payments/payments/test-upi-details/> The values below were confirmed against those pages at the time of writing. **Do not substitute invented numbers.** If a value no longer works, take the current one from the pages above and note the change in your defect report.

Test UPI IDs (India / domestic)

Purpose	UPI ID
Payment succeeds	success@razorpay
Payment fails	failure@razorpay

Domestic test card

Field	Value
Card number	4111 1111 1111 1111
Expiry	Any future date, e.g. 12 / 30
CVV	Any 3 digits, e.g. 123
Name on card	QA Patient A
OTP on the simulated 3-D Secure page	Any random 4-10 digit number, e.g. 1111

International / US test cards (only if the account is configured for them)

Network	Number
Visa (International)	4239 5360 0631 5640
Mastercard (International)	5421 1393 0609 0628
Amex (International)	3779 849088 69514
Discover	6011 0833 2733 7267

Network	Number
Visa (US)	4384 7968 2770 3274
Mastercard (US)	5312 6865 5677 9641
Amex (US)	3782 8224 6310 005
Diners (US)	6594 7300 0000 0001

The Razorpay checkout in test mode also shows explicit Success / Failure buttons on the card and netbanking flows. Where a test says "force a failure", using the checkout's own **Failure** button is equivalent to using `failure@razorpay` and is the quickest route.

9.2 How money actually moves in this application

Understanding this model is what makes a payment defect report useful.

One row per Razorpay order. The `payments` table has **unique indexes on `razorpay\_order\_id` and on `razorpay\_payment\_id`**. Nothing else in the database previously stopped one payment id being recorded against two rows. A unique-violation on either is not a bug to work around — it means a duplicate already exists and wants investigating.

A capture is applied in exactly one place. The database function `record_payment_capture` moves the `payments` row and the row it paid for **together**, under a real `select ... for update`. It is called by the three verify routes and by the webhook. It is **idempotent by construction**: the second caller for an order finds it already captured and changes nothing. That single property is what makes all four of these safe without any of them knowing about the others:

1. a duplicate webhook,
2. Razorpay's at-least-once retries,
3. a webhook racing the browser callback,
4. a double-clicked **Pay** button.

Two confirmations, whichever arrives first. A payment is confirmed by *either* the browser callback (`/api/razorpay/verify`) *or* the server webhook (`/api/razorpay/webhook`). Both go through the same capture function.

The webhook checks its signature against the raw body (`await request.text()`), never a re-serialised parse. It inserts its `payment_webhook_events` row **before** doing any work — that insert colliding on `razorpay_event_id` is the deduplication.

Without `RAZORPAY\_WEBHOOK\_SECRET`, the webhook half does not exist. The route answers `503`. A patient who pays and closes the tab before the callback lands leaves a **paid Razorpay order against an unpaid booking**. Test `PAY-WH-004` covers exactly this.

Meet/Calendar creation is deliberately outside the capture function, because it needs an outbound Google call. Confirmation of the appointment and the Meet link stay in the route.

Cash-on-visit is real, and breaks the usual assumption. A cash home-visit purchase sits at `payment_status: 'unpaid'` for its whole life with real confirmed visits hanging off it. **Never read `payment\_status` as "did money change hands" for a home visit — check `payment\_mode` first.**

9.3 The payment surfaces

Flow	Order route	Verify route	What it buys
Single online session	<code>/api/razorpay/create-order</code>	<code>/api/razorpay/verify</code>	One appointment
Home visit (prepaid)	<code>/api/home-visit/create-order</code>	<code>/api/home-visit/verify</code>	A <code>home_visit_package_purchases</code> row
Home visit (cash at the door)	<i>(none)</i>	<code>/api/home-visit/book-cash</code>	Same, <code>payment_mode='cash'</code> , <code>payment_status='unpaid'</code>
Care-plan programme	<code>/api/care-plan/create-order</code>	<code>/api/care-plan/verify</code>	A <code>patient_package_purchases</code> or home-visit purchase, plus session credits
Webhook (all of the above)	—	<code>/api/razorpay/webhook</code>	Confirms whichever the order pointed at

Direct programme checkout no longer exists. `/api/packages/create-order` and `/api/packages/verify` are deleted. `/book?package=<id>` shows a "Programmes come from your therapist now" page instead of quietly selling one session to somebody who came to buy six.

9.4 Refunds

Path	Route	Behaviour
Patient cancels outside the refund window	<code>/api/appointments/cancel</code>	Full refund via Razorpay
Patient cancels inside the window	<code>/api/appointments/cancel</code>	No refund. The confirm dialog says so before you commit.
Admin refunds a package	<code>/api/admin/refund-package</code>	VOIDS available credits only; delivered sessions stay delivered
Admin partial refund on a session	<code>/api/admin/refund-session-partial</code>	Requires an amount > 0 and a mandatory reason
Cash refund with no Razorpay payment behind it	<code>/api/admin/refund-home-visit-package</code>	Becomes <code>refund_status: 'manual_pending'</code> , surfaced on the admin Cash Ledger until an admin confirms the cash was handed back

A refund never touches a therapist's cut — a refunded session was cancelled, so it never earned one. It **does** reverse the hospital's commission, because that is a commission on net revenue.

9.5 What a single payment must never produce

Every one of these is a separate test in Section 15:

Must never happen	Guarded by	Test
Two appointments from one payment	The appointment is created before the order; the order is minted against it	<code>PAY-DUP-001</code>
Two purchases from one payment	Unique index on <code>razorpay_order_id</code>	<code>PAY-DUP-002</code>
Two entitlements from one purchase	<code>ensure_entitlement_for_purchase</code> is idempotent	<code>PAY-DUP-003</code>

Must never happen	Guarded by	Test
Double revenue on the Money screens	One <code>payments</code> row per order	<code>PAY-DUP-004</code>
Double therapist payout	<code>therapist_payout_paid_at</code> CAS on settle	<code>PAY-DUP-005</code>
Double partner commission	Commission derived from net revenue per appointment, not accumulated	<code>PAY-DUP-006</code>
A ledger credit granted twice	Idempotency key derived from the appointment/payment id, never random	<code>PAY-DUP-007</code>

10. Patient Booking Wizard: Complete E2E Flow

This section is the reference for `/book`. Read it before executing any `PAT-B00K-*` case.

10.1 What the booking wizard is, and why it exists

`/book` is the single entry point through which a patient buys their **first** video consultation. It is a three-step wizard that does four jobs in one screen: it picks a time, it creates an account if the visitor does not have one, it records what they need help with, and it takes payment. It exists as one wizard rather than four screens because a visitor who has to register before they can see whether a suitable time exists usually leaves.

It sells **one session**. It cannot sell a programme. A programme comes from a care plan a therapist writes after seeing the patient.

10.2 Who uses it

Anyone. A brand-new visitor (creates an account inside Step 2), or a signed-in patient (Step 2 shows "Booking as ..." instead of the sign-up fields). A signed-in **therapist, hospital or admin** is refused outright and shown a "wrong account" panel — see 10.10.

10.3 What information it collects

Step	Collected	Required?
1	Preferred date	Yes (auto-preselected)
1	Preferred hour	Yes (auto-preselected)
1	Preferred language	Yes (auto-preselected to the first admin-configured language)
2	Full Name, Email, Create Password, Confirm Password, Phone	Yes — guests only
2	Referral Code	Optional
2	"What would you like help with?" (treatment category)	Yes

Step	Collected	Required?
2	"Continue with the same therapist?"	Optional; only shown to a returning patient with previous therapists
2	Requested specialist (from <code>?therapist=</code>)	Optional; carried in, removable
2	"Anything else we should know?" notes	Optional
2	Telehealth consent checkbox	Yes
3	Nothing — Step 3 is review and pay	—

10.4 How the wizard decides which dates are offered

A date is offerable exactly when **at least one of its hours clears the booking lead time**. The lead time is `site_settings.online_booking_lead_time_hours`, default **12**. The calendar is built from `AVAILABILITY_HOURS` (the clinic's standard hour rows) filtered by that rule.

Consequences you will see and must not report as bugs:

- **"Today" usually drops off entirely.** At 10:00, the first bookable slot is 22:00 the same day; at 23:30, no hour remains today at all and the earliest bookable date is tomorrow.
- The boundary day is **partially** available, not all-or-nothing — early hours are greyed, later ones are live.
- Changing the date **re-presents that day's earliest eligible hour**. A previously-picked hour is not carried forward, because it may not clear the lead time on the new date.
- The picker's rule and the server's validator read the **same setting**, so the calendar can never offer a slot the server would reject. If it does, that is a genuine defect.
- The date/hour are interpreted in the **browser's local timezone**, which is the timezone Step 1 displays. The detected timezone is shown on Step 1 and stored on the appointment.

10.5 How therapist availability affects it — the rule that surprises everyone

It does not. The therapist roster (weekly schedule, date exceptions, leave) is the **clinic's planning record** — who can be *offered* a session. It deliberately does **not** filter the patient's `/book` picker, which applies the lead-time rule alone.

This is a deliberate product decision, not an oversight. A patient picks a time they want; an admin then assigns whichever therapist is actually free, and only the admin can see that. Connecting the two is a change with a deploy-sized blast radius.

So: changing a therapist's roster must never change what `/book` offers. That is a regression test (`XCFG-ROSTER-001`), not a bug.

10.6 How service areas affect it

They do not affect `/book` at all. `/book` books `visit_mode: 'online'` only. Service areas gate `/book-home-visit` exclusively.

10.7 How admin configuration reaches the wizard

Setting	Where an admin changes it	Effect on <code>/book</code>
<code>online_booking_lead_time_hours</code>	Settings → Booking Rules → Online Booking Lead Time	Which dates/hours the calendar offers, and the server's own validator
<code>online_cancellation_refund_hours</code>	Settings → Booking Rules → Online Cancellation Refund Window	The sentence on Step 3 ("Free cancellation up to N hours...") and the refund actually paid
Booking Languages	Settings → Booking Rules → Booking Languages	The chips on Step 1. An empty list degrades to <code>English</code> — booking must never present an empty language picker.
Treatment categories	Catalog → Conditions	The "What would you like help with?" dropdown, each entry showing <code>Title – ₹price / duration min</code> . An inactive category disappears, and a booking against it is refused server-side.
Category price and duration	Catalog → Conditions	The header price line, Step 3's Session Fee, the Razorpay amount, and the appointment's duration. All re-derived server-side from the category row, never from the browser — <code>/book</code> is ISR-cached, so the copy the patient filled in can legitimately be older than the one being charged.
Therapist <code>visible_on_team</code> / approval / active	People → Therapists	Whether a <code>?therapist=</code> link resolves at all

10.8 How payment is connected

1. Step 3's primary button reads **Request Booking** the first time.
2. Tapping it creates the account (guest only), runs a client-side self-overlap check, then calls `POST /api/appointments/create`. That route **re-derives** concern, duration, lead time, therapist preference and language server-side, scans the notes for contact leaks (record-only for a patient), and inserts the appointment as `status: 'requested'`, `payment_status: 'unpaid'`, `therapist_id: null`, `visit_mode: 'online'`.
3. The wizard then immediately opens Razorpay checkout via `POST /api/razorpay/create-order`. **That call flips the patient's `approved` flag to true** — on the attempt, not on success.
4. On checkout success, `POST /api/razorpay/verify` checks the signature server-side, marks the appointment paid, auto-confirms it **only if a therapist is already assigned**, creates the Meet event if so, and records the capture.

Appointments are never inserted by the browser. If you ever see a raw Postgres string such as `new` row violates row-level security policy for table "appointments" on screen, that is a P0 defect — the failure mode this route exists to prevent.

10.9 What happens on each payment outcome

Outcome	What the patient sees	What is created
Success	Step 3 is replaced by a Payment Confirmed panel with a Go to Dashboard link	Appointment <code>payment_status='paid'</code> , still <code>requested</code> until an admin assigns; a <code>payments</code> row
Failure (<code>failure@razorpay</code>)	An error message; the primary button now reads Pay ₹... Now (not "Request Booking"); attempt counter increments	The appointment already exists and stays <code>requested</code> + <code>unpaid</code>
Dismissed (patient closes the checkout modal)	<i>"Payment was not completed. You can try again below."</i>	Same as failure
Abandoned (patient closes the tab)	Nothing	Same. The booking sits in their dashboard as an unpaid session with a Pay ₹... Now button. If the webhook secret is set and the order was in fact captured, the webhook confirms it server-side anyway.
Retry	Tapping Pay ... Now re-opens checkout against the same appointment. <code>create-order</code> re-checks the prior order: if Razorpay says it is already paid, the appointment is claimed as paid rather than a second order being minted.	No second appointment, no second order for a paid one
3+ failed attempts	An amber escape hatch appears: <i>"Having trouble paying? Your booking is saved as pending..."</i> with a Go to Dashboard link	Nothing new
Back from Step 3 to Step 2	The draft appointment is deliberately abandoned; the button returns to Request Booking	The unpaid appointment remains in the dashboard, exactly as if the tab had been closed

10.10 Refusals the wizard renders instead of the form

Condition	What renders
Signed in as therapist / hospital / admin	A wrong account panel routing each role to what is theirs (hospitals refer; admins use New Booking; a clinician wanting therapy signs out and uses a separate patient account). Checked before every other branch.
<code>?package=<id></code> in the URL	<i>"Programmes come from your therapist now"</i> plus the explanation and a Book a first session link. Ordered after the wrong-account branch on purpose.
No treatment categories exist	<i>"No condition categories are available right now — please contact us directly to book."</i> and no dropdown

10.11 What happens on Back, Refresh and abandonment

Action	Result
Back on Step 2	Returns to Step 1. All Step 1 values are preserved.

Action	Result
Back on Step 3	Returns to Step 2, clears the draft appointment id and the failed-attempt counter , and the primary button becomes Request Booking again.
Refresh at any step	The wizard restarts at Step 1 with fresh auto-picks. No wizard state is persisted . Any appointment already created stays in the database and appears in the dashboard as unpaid.
Abandon (close tab)	Same as refresh.

10.12 What each role sees afterwards

Role	Where	What
Patient	/patient/dashboard and → Your Sessions	The session under Upcoming , Requested if unassigned, with Pay ₹... Now while unpaid
Admin	Sessions → All Sessions, and Sessions → Schedule	The session in the list and on the calendar. Unassigned sessions raise the All Sessions badge. Opening it gives the assign form, with a requested therapist preselected and marked (requested).
Admin	Money → Transactions	The payment row, once paid
Therapist	/therapist/dashboard/sessions	Nothing until an admin assigns them. A requested unassigned session belongs to no therapist.
Hospital	Your Referrals / Earnings	Only if the patient was referred by them

10.13 Step-by-step screen reference

Step 1 — "When suits you?"

- **Means:** pick a time you would like. It is a *request*, not a locked slot.
- **Expected of the user:** confirm or change three auto-picked values.
- **Controls:** a month calendar (tappable day cells; ineligible days are greyed and not tappable), a row of hour chips, a row of language chips, and **Continue**.
- **On selection:** the day cell highlights; the hour list re-filters to that day's eligible hours; the "we picked this for you" hint disappears from whichever value you changed and never re-fires.
- **Validation on Continue:** date and hour must be set (*"Please select a preferred date and time."*); the slot must clear the lead time (*"Please choose a time at least 12 hours from now."*); a language must be set (*"Please select a preferred language."*).
- **Nothing is created.** No appointment, no account, no charge.

Step 2 — Your details and your concern

- **Means:** who you are and what you need.
- **Guest:** Full Name, Email, Create Password (min 6), Phone, Confirm Password, Referral Code (optional). A signed-in patient sees only a teal *"Booking as Name (email)"* strip.
- **Everyone:** the concern dropdown, optional therapist preference, optional notes, and a **required** telehealth consent checkbox.

- **Referral code** is validated on blur: `Checking code...` → either `Valid – referred by <Hospital>` in teal, or `Code not recognized – double-check it or leave blank` in red. An invalid code **blocks Continue**; a blank one does not.
- **Validation on Review Booking** →, in this order: name/email/password present and password ≥ 6 (*"Please fill in your name, email, and a password (min 6 characters)."*); email shape (*"Please enter a valid email address."*); phone shape (*"Please enter a valid phone number."*); passwords match (*"Passwords do not match. Please re-enter them."*); referral code not invalid; a category is chosen (*"Please select what you'd like help with."*); consent ticked (*"Please agree to the telehealth consent terms to continue."*).
- **Nothing is created.**

Step 3 — Review and pay

- **Means:** confirm the summary, then pay.
- **Shows:** Name, Email, Preferred Time, Language, Concern, and Session Fee, plus two notices — the Razorpay/secure-payment line and the cancellation-window line reading the admin's configured hours.
- **Controls:** **Back** ($\frac{1}{3}$ width) and the primary button ($\frac{2}{3}$ width) reading **Request Booking**, then **Pay ₹...** **Now** after an appointment exists.
- **Records created:** on **Request Booking** — the Supabase auth user (guest only) and the appointment row. On payment success — the `payments` row, the appointment's paid/confirmed state, and (when a therapist is already assigned) the Google Calendar/Meet event.

11. Patient test plan

11.0 Feature guide — the patient's world

A patient's journey is: **register** → **get approved (or auto-approve by paying)** → **book a consultation** → **pay** → **attend** → **receive a recommendation** → **accept and pay for a programme** → **spend its credits session by session**, with a **Health Profile** running alongside that the therapist opens first and the patient then maintains.

Two rules shape almost everything on the patient's screens:

1. **A screen that can only ever be empty is not in the sidebar.** Your Sessions, Your Packages, Payments and Suggested Sessions appear only once the patient actually has one. **Book a Session is the deliberate exception** — always shown, because that is how a patient gets their first of anything.
2. **The patient is read-only on their own Health Profile until a therapist writes the first record.** While locked, the "answer your questions" call to action and the answered counter are **absent, not disabled**; the dashboard banner is dropped rather than recoloured; the overview cell reads `–` on slate rather than `0%` on amber. The **reports uploader stays open**, because it is the one useful thing a patient can do beforehand.

11.1 Registration, login, approval

`PAT-AUTH-001` — Sign in as an existing patient · P0

Feature. Patient authentication. **Role.** Patient. **Purpose.** Confirm the login form authenticates and lands on the dashboard. **Preconditions.** Patient A exists and is approved and active (created by `PAT-B00K-003`)

or `PAT-AUTH-002`). **Test Data.** `qa.patient.a@example.test` / `QaTest!2024pass` .

Steps

1. Open `/patient/login` .
2. Confirm the two tabs **Sign In** and **Register Account** are visible, with **Sign In** active.
3. Tap the **Email Address** field. Enter `qa.patient.a@example.test` .
4. Tap the **Password** field. Enter `QaTest!2024pass` .
5. Tap **Sign In**.

Expected Result. The button reads `Signing in...` while working. The browser navigates to `/patient/dashboard` . The sidebar shows **Overview**, **Book a Session**, **Health Profile** and **Edit Profile** at minimum, with **Back to Home** at the **foot of the nav, directly above Collapse** (above the profile footer in the mobile drawer, which has no Collapse). The public `Navbar` is **not** rendered on the dashboard. No error banner. **Cleanup.** Stay signed in for `PAT-DASH-001` .

`PAT-AUTH-002` — Register a patient account from `/patient/register` • P0

Purpose. Prove standalone registration always waits for a human admin — the payment-attempt auto-approval does **not** apply here. **Test Data.** Patient B from §8.3.

Steps

1. Open `/patient/login` . Tap **Register Account**.
2. Tap **Full Name**. Enter `QA Patient B` .
3. Tap **Email Address**. Enter `qa.patient.b@example.test` .
4. Tap the phone field. Enter `+91 98765 43211` .
5. Tap **Password**. Enter `QaTest!2024pass` .
6. Tap **Confirm Password**. Enter `QaTest!2024pass` .
7. Leave **Referral Code** blank.
8. Tap **Create Account**.

Expected Result. The button reads `Creating account...` , then the browser lands on ``/pending-approval`` — **not** the dashboard, and **not** any "check your email" screen. The page explains the account is waiting for approval. In Supabase, the `profiles` row exists with `role='patient'` , `approved=false` , `active=true` , and a `patient_code` of the form `PT####` . **Cleanup.** Leave. `ADM-APPR-001` approves this account.

`PAT-AUTH-003` — An unapproved patient cannot reach the dashboard • P0

Preconditions. `PAT-AUTH-002` complete; Patient B not yet approved. **Steps**

1. Signed in as Patient B, type `/patient/dashboard` directly in the address bar and press Enter.
2. Then call the API directly. In a terminal, with Patient B's session cookie:

```
curl -i -X POST http://localhost:3000/api/appointments/create -H 'Content-Type: application/json' -b '<patient B cookie>' -d '{"slotTime":"2026-12-01T10:00:00.000Z"}'
```

Expected Result. Step 1 redirects to ``/pending-approval`` . Step 2 returns **HTTP 200** and creates a `requested / unpaid` appointment — this is correct: `/api/appointments/create` gates on `isProfileActive` , **not** approval, because an unapproved self-signup patient must be able to hold the row they are about to pay for. It grants nothing on its own. **What must be refused is a suspended account** — see `SEC-AUTH-006` . **Cleanup.** Delete the stray appointment from Sessions → All Sessions, or leave it as a fixture for `ADM-SESS-002` .

PAT-AUTH-004 — Registration validation (negative) · P1

Steps. On `/patient/login` → **Register Account**, attempt **Create Account** once for each row, resetting the form between attempts:

Atte mpt	Field changed	Value	Expected message
1	Email	<code>not-an-email</code>	An invalid-email message; the account is not created
2	Password	<code>abc12</code>	A minimum-length message; not created
3	Confirm Password	<code>QaTest!2024pas</code>	A passwords-do-not-match message; not created
4	Phone	<code>12345</code>	An invalid-phone message; not created
5	Referral Code	<code>ZZZZZZ</code>	<code>Code not recognized – double-check it or leave blank</code> in red
6	Email	<code>qa.patient.a@example.test</code> (already registered)	A "user already registered" style error from Supabase; no second profile row is created

Expected Result. In every case the form stays on screen with the entered values intact, the message is human-readable, and **no stack trace, database column name or row id appears anywhere on screen.**

PAT-AUTH-005 — Sign out · P2

Steps. From `/patient/dashboard`, tap the sign-out control in the sidebar/header. **Expected Result.** The session ends, the browser lands on a public page, and a farewell banner shows for the admin-configured number of seconds (`farewell_banner_seconds`, default 6; `0` means until dismissed). Navigating back to `/patient/dashboard` now redirects to `/patient/login`.

PAT-AUTH-006 — Forgot password · P2

Steps. On `/patient/login` tap **Forgot password**, enter `qa.patient.a@example.test`, tap **Send Reset Link**. **Expected Result.** The button reads `Sending...`, then a confirmation appears. A ← **Back to Sign In** link returns to the form. (Delivery cannot be checked with a `.test` address — verify the request succeeded and that the reset landing page `/reset-password` renders.)

11.2 The booking wizard — happy paths

PAT-B00K-001 — Reach the wizard from the public site · P1

Steps

1. Open `/`.
2. Tap **Book a session** in the header (or the **Book now** card in the connector grid at the foot of the page).
3. Observe the URL and the wizard header.

Expected Result. The URL is `/book`. The dark header reads **Step 1 of 3** and **Book Virtual Physical Therapy Session**. Because no concern is chosen yet, the subtitle reads *"HD Video Call & Custom Rehab Plan — pricing shown once you pick a concern"*.

PAT-B00K-002 — Step 1 auto-picks and the lead-time boundary (scenario TIME-A) · P0

Preconditions. SETUP-CAT-001 has created the three treatment categories. Online lead time is the default 12 hours. **Test Data.** Simulate 2026-09-10T10:00.

Steps

1. In the Debug bar set **Simulate now** to 2026-09-10T10:00 and tap **Set**.
2. After the reload, open /book.
3. Read the calendar: note the month heading, which day is marked as today, and which day cells are greyed.
4. Read the hour chips: note the first one offered.
5. Read the language chips: note which one is selected.
6. Tap the day cell **11**.
7. Read the hour chips again.
8. Tap the hour chip **09:00**.
9. Tap **Continue**.

Expected Result

- Step 3: heading September 2026; **10** is today; every cell before 10 is greyed and not tappable.
- Step 4: the earliest offered hour on 10 September is **22:00** (10:00 + 12h). Hours before 22:00 are absent or disabled. A "we picked this for you" hint marks the auto-chosen date, hour and language.
- Step 5: the first admin-configured booking language is selected — English by default.
- Step 7: after moving to 11 September, the **full** hour list is offered (every hour on 11 September clears a 12h lead time from 10:00 on the 10th), and the hour has been **re-preselected to that day's earliest**, not carried over from 22:00.
- Step 9: the wizard advances to **Step 2 of 3. No appointment exists yet, no account is created, and nothing is charged.**

Cleanup. Stay in the wizard for PAT-B00K-003.

PAT-B00K-003 — Guest books and pays successfully (the main journey) · P0

Purpose. The single most important path in the application: a brand-new visitor becomes a paying patient with a booked session. **Preconditions.** PAT-B00K-002 left the wizard on Step 2 with 11 September 2026 09:00 and English selected. Razorpay test keys configured. **Test Data.** Patient A from \$8.3. Payment: UPI success@razorpay.

Steps

1. Tap **Full Name**. Enter QA Patient A.
2. Tap **Email**. Enter qa.patient.a@example.test.
3. Tap **Create Password**. Enter QaTest!2024pass.
4. Tap the phone field. Enter +91 98765 43210.
5. Tap **Confirm Password**. Enter QaTest!2024pass.
6. Leave **Referral Code** blank.
7. Tap the **What would you like help with?** dropdown. Select QA Back & Spine Care – ₹1,999 / 60 min.
8. Tap the **Anything else we should know?** box. Enter Desk job, pain worse after sitting all day.
Goal: sit through a full workday.
9. Tap the telehealth consent checkbox.
10. Tap **Review Booking** →.

11. On Step 3, read every summary row.
12. Tap **Request Booking**.
13. When the Razorpay checkout opens, choose **UPI**. Enter `success@razorpay`. Submit and complete the simulated approval.
14. Wait for the checkout to close.

Expected Result

- Step 10: the wizard advances to **Step 3 of 3**. The dark header subtitle now reads `₹1,999 INR • 60-Min HD Video Call & Custom Rehab Plan`.
- Step 11: **Name** `QA Patient A`; **Email** `qa.patient.a@example.test`; **Preferred Time** 11 September 2026, 09:00, in local format; **Language** `English`; **Concern** `QA Back & Spine Care`; **Session Fee** `₹1,999 INR`. Two notices are present: the Razorpay/secure line and *"Free cancellation up to 24 hours before your slot. Cancelling within 24 hours of the slot isn't eligible for a refund."*
- Step 12: the button shows `Please wait...`; the Supabase account is created and signed in **with no email-confirmation step**; the appointment is created; Razorpay checkout opens.
- Step 14: Step 3 is replaced by a green tick, **Payment Confirmed**, the sentence about confirming the slot and sending the link, and a **Go to Dashboard** button.
- **Data state:** exactly **one** appointment for Patient A — `status='requested'`, `payment_status='paid'`, `therapist_id=null`, `visit_mode='online'`, `duration_minutes=60`, `concern='QA Back & Spine Care'`, `preferred_language='English'`, timezone recorded. Exactly **one** payments row for that Razorpay order. Patient A's `approved` is now **true** (flipped by `create-order`).
- **Cross-role:** Admin → Sessions → All Sessions lists it as unassigned and raises the badge; Admin → Money → Transactions lists the ₹1,999 payment; **no therapist sees it yet**.

Cleanup. Leave everything. This is the root fixture for most of the plan.

`PAT-B00K-004` — The lead-time boundary is computed, not hardcoded (scenario TIME-B) · P1

Steps. Simulate `2026-09-12T18:00`, tap **Set**, open `/book`. **Expected Result.** Today is **12 September**. The earliest bookable slot is **13 September at 06:00** — `18:00 + 12h` is `06:00` the next day, and no hour remains on the 12th. The 12th is either fully greyed or offers no hour chips; the 13th is preselected.

Cleanup. Reset to real time.

`PAT-B00K-005` — Late-night rollover (scenario TIME-C) · P1

Steps. Simulate `2026-09-10T23:30`, open `/book`. **Expected Result.** The earliest bookable date is **11 September** (`23:30 + 12h = 11:30` on the 11th, so the 11th's later hours qualify while the 10th has none left). The 10th is greyed. The auto-picked hour is the 11th's earliest hour at or after 12:00. **Cleanup.** Reset to real time.

`PAT-B00K-006` — A signed-in patient books again · P1

Preconditions. Patient A signed in, with at least one completed session so a previous therapist exists.

Steps. Open `/book`, complete Step 1, and read Step 2. **Expected Result.** Step 2 shows a teal strip *"Booking as QA Patient A (qa.patient.a@example.test)"* and **no** name/email/password/phone/referral fields. A **"Continue with the same therapist?"** dropdown appears, defaulting to `No preference – any available specialist` and listing `QA Therapist A`. The consent checkbox and concern dropdown are still required.

`PAT-B00K-007` — Booking a named specialist from `/team` · P1

Steps

1. Open `/team`.
2. Tap the card for `QA Therapist A`.
3. In the popup, tap the **Book with ...** action.
4. Read the URL, then advance to Step 2.

Expected Result. The URL is `/book?therapist=<uuid>`. On Step 2 a teal panel reads **Requested: QA Therapist A**, their credentials beneath, and the sentence *"We'll book you with them if they're free at your chosen time. If not, another specialist takes the session and you'll see who before it starts."* It has a **Remove** button. The "Continue with the same therapist?" dropdown is **hidden** while this panel is shown. It must **never** be worded as a confirmed booking with that therapist.

Environment note. This resolution happens **in the browser** against `public_therapist_profiles`, because `/book` is ISR-cached. In a sandbox whose Chromium has no outbound network, the chip never renders and this test fails on a working feature. Check first: run `fetch(location.origin)` and then a direct fetch of `<SUPABASE_URL>/rest/v1/` from the page console. "Failed to fetch" there, with the same call succeeding from Node, means the environment — not the feature.

PAT-B00K-008 — A hidden or suspended therapist link degrades silently · P2

Preconditions. An admin has switched `QA Therapist C`'s **Show on team** off (or suspended them).
Steps. Open `/book?therapist=<Therapist C uuid>`, advance to Step 2. **Expected Result.** No requested-therapist panel appears; the booking proceeds normally with no preference attached. No error is shown. A hand-typed or stale link must never fail a booking.

11.3 The booking wizard — negative, boundary and duplicate

PAT-B00K-010 — Step 1 validation · P1

Attempt	Action	Expected message
1	Deselect the language (if the UI allows) and tap Continue	Please select a preferred language.
2	Hand-edit the URL/state to an in-the-past slot and tap Continue	Please choose a time at least 12 hours from now.

Expected Result. In both cases the wizard stays on Step 1 and no appointment exists.

PAT-B00K-011 — Step 2 validation, in order · P1

Attempt **Review Booking** → once per row, as a guest:

#	Missing/invalid	Expected message
1	Name blank	Please fill in your name, email, and a password (min 6 characters).
2	Password <code>abc12</code>	same as above
3	Email <code>not-an-email</code>	Please enter a valid email address.
4	Phone <code>12345</code>	Please enter a valid phone number.
5	Confirm password mismatched	Passwords do not match. Please re-enter them.

#	Missing/invalid	Expected message
6	Referral code ZZZZZZ (blurred, shown invalid)	That referral code isn't recognized. Please double-check it or clear the field to continue without one.
7	No concern selected	Please select what you'd like help with.
8	Consent unticked	Please agree to the telehealth consent terms to continue.

Expected Result. Each message appears in the red banner at the top of the card, the entered values survive, and the wizard does not advance. **No account and no appointment is created by any failed attempt.**

PAT-B00K-012 — Overlapping booking is refused (client and server) · P0

Preconditions. Patient A already has a requested or confirmed session at 11 September 2026 09:00, 60 minutes. **Steps.** Signed in as Patient A, book the **same** slot again through /book and tap **Request Booking**. Then repeat the attempt directly against the API with Patient A's cookie and the same slotTime. **Expected Result.** Both refuse with: You already have a session scheduled around this time. Please pick a different slot, or check your dashboard for existing bookings. The API returns **409**. **No second appointment row is created.** (The client check exists for fast feedback; the server check is the one that binds.)

PAT-B00K-013 — An inactive category is refused server-side · P1

Steps

1. Load /book and select QA Knee & Joint Care on Step 2, but do **not** submit.
2. In a second browser as Admin Full, open Catalog → Conditions and set QA Knee & Joint Care to inactive.
3. Return to the wizard and tap **Request Booking**.

Expected Result. The booking is refused with That concern isn't available any more. Please pick another one. (HTTP 409). No appointment is created. This proves the route re-derives the category rather than trusting the ISR-cached page. **Cleanup.** Re-activate the category.

PAT-B00K-014 — A stale ?package= link is answered, not ignored · P0

Steps. Open /book?package=00000000-0000-0000-0000-000000000000. **Expected Result.** Instead of the wizard, a panel headed "Programmes come from your therapist now" with the explanation "A course of treatment is arranged by your therapist after they've seen you... Book a first session and they'll recommend the right programme." and a **Book a first session** link to /book. **The wizard must not silently sell one session to somebody who came to buy six.**

PAT-B00K-015 — Refresh mid-wizard loses no money and creates nothing · P2

Steps. Complete Steps 1 and 2, reach Step 3, then press **F5**. **Expected Result.** The wizard restarts at Step 1 with fresh auto-picks. No appointment exists (none was created — creation happens on **Request Booking**). No charge.

PAT-B00K-016 — Back from Step 3 abandons the draft cleanly · P2

Steps. Reach Step 3, tap **Request Booking**, dismiss the Razorpay modal, then tap **Back**. **Expected Result.** The wizard returns to Step 2. The primary button on Step 3 reads **Request Booking** again (not "Pay ... Now"), and the failed-attempt counter is reset. The unpaid appointment created by the first attempt **remains** in the patient's dashboard with a **Pay ₹1,999 Now** button — it is not deleted, and a second

Request Booking creates a *second* appointment only if the slot does not overlap the first (it will overlap, so it is refused — see `PAT-B00K-012`).

`PAT-B00K-017` — Double-tap on Request Booking · P0

Steps. On Step 3, tap **Request Booking** twice in rapid succession (double-click). **Expected Result.** The button disables on the first tap (`Please wait...`). Exactly **one** appointment row exists afterwards. Exactly **one** Razorpay order is minted.

11.4 Payment outcomes from the patient's side

`PAT-PAY-001` — Payment fails, then retries successfully · P0

Steps

1. Book a fresh consultation as Patient A for 12 September 2026 10:00, `QA Knee & Joint Care` .
2. On Step 3 tap **Request Booking**.
3. In the checkout choose **UPI**, enter `failure@razorpay` , submit.
4. Read the wizard.
5. Tap **Pay ₹1,799 Now**.
6. This time enter `success@razorpay` and complete.

Expected Result

- Step 4: an error banner appears; the primary button now reads **Pay ₹1,799 Now**; the appointment already exists as `requested / unpaid` .
- Step 6: **Payment Confirmed** panel. **Still exactly one appointment.** The `payments` table has one row for the successful order. A failed attempt is logged (`payment_failure_log`) and appears nowhere on a patient screen.
- Money → Transactions shows **₹1,799 once**, not twice.

`PAT-PAY-002` — Dismissing the checkout · P1

Steps. Tap **Request Booking**, then close the Razorpay modal with its ×. **Expected Result.** `Payment was not completed. You can try again below.` The button reads **Pay ... Now**. The unpaid appointment is visible at `/patient/dashboard/sessions` under Upcoming with a **Pay ... Now** button.

`PAT-PAY-003` — Reassurance on the first failure, escape hatch on the third · P1

Steps. Fail or dismiss the payment **once** and read the card. Then fail it twice more. **Expected Result.** After the **first** failure a slate panel reads *"Nothing was lost — your booking is saved and still held as unpaid. You can try again above, or pay later from your dashboard."* A patient whose card was declined has no other way of knowing the booking survived, and the likeliest next action is closing the tab. After the third, that line is replaced by an amber panel: *"Having trouble paying? Your booking is saved as pending — you can come back and pay any time from your dashboard."* with a **Go to Dashboard →** link. Following it lands on `/patient/dashboard` with the session listed as unpaid. **The patient is not bounced to `/pending-approval` —** the first genuine attempt already approved them.

`PAT-PAY-004` — Abandon then pay from the dashboard · P0

Steps. Create an unpaid booking, close the tab, sign in again, go to **Your Sessions**, tap **Pay ₹... Now** on that row, and complete with `success@razorpay`. **Expected Result.** The same appointment becomes paid.

No second appointment. The dashboard row's **Pay** button is replaced by the session's normal state. One `payments` row.

`PAT-PAY-005` — **Retry after the order was already captured** · P1

Purpose. Prove `create-order` re-checks a prior order rather than minting a second one. **Steps.** Pay successfully, then (before the page updates) tap **Pay ... Now** again, or reload and tap it. **Expected Result.** The route finds the prior order already `paid` at Razorpay, claims the appointment as paid, and returns a message saying the booking is already paid rather than opening a second checkout. **No second Razorpay order id is written to the appointment.**

11.5 Home visit (patient)

Preconditions for this whole sub-section: `ADM-SET-013` has switched **Home Visit enabled** on, `SETUP-AREA-001` has created Area 1 (`560038` , ₹150/visit), and `SETUP-HVPKG-001` has created HV1 (1 visit) and HV2 (4 visits).

`PAT-HV-001` — **The public home-visit page appears only when enabled** · P1

Steps. With the master switch **off**, open `/home-visit` . Then switch it on in Settings → Booking Rules → Home Visit and reload. **Expected Result.** Off: a **404** page, and the **Home visit** entry is absent from the header nav, the footer Explore column, the home page connector grid and every "Where to go next" strip. On: the page renders with the admin-configured heading and subheading, and the entry reappears everywhere.

`PAT-HV-002` — **Serviceable pincode → address → book and pay** · P0

Test Data. Pincode `560038` ; address from Patient A §8.3.

Steps

1. Open `/book-home-visit` . Confirm the header reads **Step 1 of 4**.
2. Tap the **Pincode** field. Enter `560038` .
3. Tap **Check**.
4. Read the confirmation line.
5. Fill the address form: Address line 1 `12, 3rd Cross, Indiranagar` ; line 2 `Near Metro Station` ; City `Bengaluru` ; State `Karnataka` ; PIN `560038` .
6. Tap **Continue**.
7. On **When suits you?** pick a date at least 24 hours out and an arrival time.
8. Tap **Continue**.
9. On **About you**, choose the package `QA Home Visit – Single – 1 visit` and complete the identity fields (or confirm the "booking as" strip if signed in).
10. Tap **Continue**.
11. On **Review and pay**, read every line of the price breakdown.
12. Choose **Pay online**, tap the pay button, and complete with `success@razorpay` .

Expected Result

- Step 4: a teal line — *"Yes — we visit Indiranagar, Bengaluru. Travel to this area is ₹150 per visit."*
- Step 7: the copy states home visits need at least **24 hours'** notice (the `home_visit_lead_time_hours` setting, deliberately longer than the online 12).

- Step 11: the breakdown shows **programme price**, **travel**, and **total** as three separate figures — ₹2,499 + ₹150 = ₹2,649 . **The button must charge exactly the total shown.** Quoting one figure and charging another is a P0 defect.
- Step 12: a confirmation screen. A `home_visit_package_purchases` row exists with `payment_mode='online'`, `payment_status='paid'`, `travel_fee_paise=15000`, a `default_address_id`, and a **snapshot** of the package.
- The visit's address is **snapshotted onto the appointment** (`visit_address_*`), not referenced live — editing the saved address later must not rewrite a delivered visit.

PAT-HV-003 — Unserviceable pincode → waitlist · P1

Steps. Open `/book-home-visit`, enter `560025`, tap **Check**. Then enter `QA Patient B` and `+91 98765 43211` and tap **Tell me when you do**. **Expected Result.** An amber panel: *"We don't visit 560025 yet."* plus *"Leave your number and we'll tell you the moment we do. Nothing has been charged."* No address form appears and no package can be chosen. After submitting: *"Thanks — we'll be in touch."* The panel also offers a link to an **online consultation**, which is available anywhere. Admin → Catalog → Service Areas shows the waitlist entry, and its badge count increases.

PAT-HV-004 — Pincode validation · P2

Value	Expected
<code>56003</code>	Enter a valid 6-digit pincode.
<code>0560038</code>	Enter a valid 6-digit pincode. (must not start with 0)
<code>abcdef</code>	Enter a valid 6-digit pincode.
<i>(blank)</i>	The Check button does nothing harmful; a validation message appears

PAT-HV-005 — A multi-visit home package cannot be bought directly · P0

Steps. In the Step 3 package picker, attempt to select `QA Home Visit Recovery – 4 Visits`. If the UI offers it, proceed and attempt payment. Also call the API directly: `POST /api/home-visit/create-order` with HV2's id. **Expected Result.** The multi-visit package is **not offered** for direct purchase, and the API refuses it. Only a **one-visit** home package is directly purchasable — that is the patient's home consultation. HV2 can only reach a patient through a therapist's care plan.

PAT-HV-006 — Cash on visit · P1

Preconditions. **Allow cash on visit** is on. **Steps.** Repeat `PAT-HV-002` but choose **Pay at the visit** on Step 4. **Expected Result.** The booking completes with **no Razorpay checkout**. The purchase row has `payment_mode='cash'` and `payment_status='unpaid'` — **for its whole life**, with a real confirmed visit hanging off it. The patient's Payments screen must not describe this as an outstanding failure. Admin → Money → Payouts → Cash Ledger will show the collection once the therapist records it.

PAT-HV-007 — Cash refused when the switch is off · P2

Steps. Admin turns **Allow cash on visit** off. Patient repeats the flow. **Expected Result.** The cash option is absent. Calling `/api/home-visit/book-cash` directly returns `Paying at the door isn't available right now – please pay online instead.`

11.6 Sessions, joining and cancellation

PAT-SESS-001 — The Sessions screen, its filters and its view switch · P1

Preconditions. Patient A has at least one upcoming, one past and one cancelled session, and at least one home visit. **Steps.** Open `/patient/dashboard/sessions`. Tap **Upcoming**, **Past**, **Cancelled** in turn. Then tap the **Video** and **Home visit** filters. Then switch **List** → **Calendar** and back. **Expected Result.** One list, not two. The Video/Home visit filter appears **only** because this patient has both kinds; a patient with only video sessions must not see it. The Calendar view renders **the same server-rendered cards**, so the two views can never disagree about a session. Filter chips are hidden entirely when only one of them would have rows behind it. The list ends with a pager showing "x-y of n", a per-page control, and Previous/Next that grey out at the ends.

PAT-SESS-002 — An unassigned session reads as Requested · P2

Expected Result. A paid but unassigned session shows a `Requested` status pill and **no therapist name**. It must not claim a therapist it does not have.

PAT-SESS-003 — Tap to Join appears only inside the join window (scenario TIME-E) · P1

Preconditions. A `confirmed` session with an assigned therapist and a Meet link. Join window before/after default to 15 minutes. **Steps.** With the real clock well before the slot, look at the session card. Then simulate a time 10 minutes before the slot and reload. **Expected Result.** Before the window: the control is inert and does not offer a call. Inside the window: **Tap to Join** is live and opens the Meet link. **Reminder:** this control reads the *simulated* clock; the server's own gates do not.

PAT-SESS-004 — After the cutoff every surface says Session Completed (scenario TIME-F) · P1

See `XR-CUTOFF-001` for the cross-role version. Simulate 90 minutes after the slot (past the default 60-minute `session_completed_after_minutes`). **Expected Result.** The control reads **Session Completed** on the patient's card — and on the therapist's and the admin's, identically.

PAT-CANCEL-001 — Cancel outside the refund window → full refund (scenario TIME-G) · P0

Preconditions. A **paid**, `confirmed` session whose slot is more than 24 hours away in **real** time (the refund maths runs server-side, so a simulated clock will not move it). **Steps.** Open the session card, tap **Cancel Session**, read the dialog, optionally enter a reason, confirm. **Expected Result.** The dialog reads *"Cancel this session and refund the payment? You can add a reason (optional):"*. After confirming, the card shows `Cancelling...` then a cancelled state. The appointment becomes `cancelled` with `refund_status='processed'` and the refund amount recorded. Admin → Money → Summary shows the refund reducing **Net Revenue** while **Gross Revenue** is unchanged. **No therapist cut is reversed** — a cancelled session never earned one. If the patient was hospital-referred, the **partner commission is reduced**, because it is taken on net.

PAT-CANCEL-002 — Cancel inside the window → no refund, and the dialog says so (scenario TIME-H) · P0

Preconditions. A paid session whose slot is **less than 24 hours** away in real time. **Steps.** Tap **Cancel Session** and read the dialog **before** confirming. **Expected Result.** The dialog reads *"Cancel this session? It's within 24 hours of your slot, so this won't be refunded. You can add a reason (optional):"*. After confirming, the appointment is `cancelled` with **no refund**. Money → Summary is unchanged for refunds. This forfeited amount stays in the clinic's share — it must not deduct a therapist cut, because the session was never completed.

PAT-CANCEL-003 — Home visits use their own refund window · P1

Repeat PAT-CANCEL-001 / 002 against a home visit. **Expected Result.** The window used is `home_visit_cancellation_refund_hours` (default 24, independently configurable). Changing the online window in Settings must **not** change the home-visit dialog, and vice versa.

PAT-SESS-006 — Rate a completed session · P2

Steps. After a therapist marks a session complete, open the patient's card and submit a rating of `5` with feedback `Very clear explanation, the exercises helped.` **Expected Result.** The rating is stored once. A second attempt returns `You've already rated this session.` A rating of `0` or `6` is refused with `Rating must be between 1 and 5`. A no-show session refuses rating entirely: `This session was marked as a no-show — there's nothing to rate.`

11.7 Health Profile (patient side)

PAT-HP-001 — Before the therapist's first record, the patient is read-only · P0

Preconditions. Patient A exists with **no** condition profile yet. **Steps.** Open `/patient/dashboard/health-profile`. Then return to `/patient/dashboard` and look at the overview. **Expected Result**

- On the Health Profile screen: **no** "answer your questions" call to action and **no** answered counter — they are **absent, not greyed out**.
- The **reports uploader is present and usable** — it is the one useful thing the patient can do beforehand.
- On the dashboard overview: the health-profile cell reads `—` **on a slate background**, never `0%` on amber, and there is **no amber banner**.
- The waiting panel **names the session it is waiting on**, by date — *"Your session on 11/09/2026 is when this gets filled in"* before the session, and *"Expected at your session on 11/09/2026. If it still isn't here in a day or two, tell us and we'll chase it."* after it. A locked screen with no date on it leaves the patient unable to tell whether the wait is normal or whether they have been forgotten. A patient with no session yet simply sees no date line — never a placeholder.
- Attempting the API directly (`POST /api/patient/condition-profile/save-draft` with the patient's cookie) is **refused** — the lock is enforced in `submit`, in `save-draft`, and by an insert policy on `condition_change_requests`. A UI-only lock would be cosmetic.

PAT-HP-002 — After the therapist's first fill, the wizard unlocks · P0

Preconditions. `THR-HP-002` has triaged Patient A as Orthopaedic and written the first record. **Steps.** Open `/patient/dashboard/health-profile`. **Expected Result.** The seven orthopaedic answers are shown **as answers, not as inputs**. The severity gauge, the pain-area chips and the progress line render. The answered counter appears. Attribution is correct: the counter must **not** claim the patient answered questions the clinician wrote — answers written by the therapist are attributed to the therapist.

PAT-HP-003 — The intake wizard is one question at a time · P1

Steps. Tap the call to action to review/update answers. **Expected Result.** A **pop-up wizard showing one question at a time**, never a seven-field form rendered on the dashboard. Each question shows its `helpText` in the patient's own words. Progress through all seven with the §8.4 answers.

PAT-HP-004 — Submitting a change goes to review · P1

Steps. Change `severity` from `6` to `4` and submit. **Expected Result.** A confirmation that the clinic will review it. The word used for the reviewer on a **patient** screen is **"the clinic"** — never "admin", never "us". A second submission while one is pending is refused: `Your last change to your health profile is still being checked by the clinic.`

PAT-HP-005 — The record exports as a PDF, not JSON · P1

Steps. Tap the export/download control on the Health Profile. **Expected Result.** A typeset **PDF** downloads, named `QA Patient A_PT####.pdf` (name + patient code). It contains the intake answers and the Pain Map data. **Session notes are excluded from every format.** `?format=json` still serves raw JSON for portability but is not linked anywhere in the UI. **Boundary.** A patient whose name contains non-Latin characters (set the name to `पूजा शर्मा` temporarily) must still export successfully — the PDF encoder transliterates rather than throwing a 500.

PAT-DOC-001 — Upload a report · P0

Steps. On the Health Profile, open the reports panel. Tap the upload control. Choose `Spine_Report_E2E.pdf`. Pick the type `Scan or X-ray`. Submit. **Expected Result.** The file appears in the list with its type label and upload date. Only **metadata** is stored in the database — the file itself is in the private `medical-reports` bucket. Tapping the file opens it through `/api/medical-documents/view`, which mints a **120-second signed URL**. Copying that URL and opening it after two minutes must **fail** — the link is deliberately short-lived.

PAT-DOC-002 — Upload limits · P1

Attempt	Expected
<code>Oversize_Report_E2E.pdf</code> (>10 MB)	Refused before or during upload with a size message; nothing is stored
<code>Not_Allowed_E2E.txt</code>	The file picker does not accept it (<code>accept=".pdf,.jpg,.jpeg,.png,.webp,.heic,.heif"</code>); a forced API call is refused with <code>Upload a PDF or a photo (JPG, PNG, WEBP or HEIC).</code>
No type selected	<code>Pick what kind of report this is.</code>
No file chosen	<code>Choose a file to upload.</code>
A 21st file	Refused — the cap is 20 files per patient , enforced in the upload route

PAT-DOC-003 — Deleting and re-uploading is the only correction path · P2

Steps. Delete `Posture_Assessment_E2E.pdf`, then upload it again. **Expected Result.** Deletion succeeds and the row disappears. **There is deliberately no edit/update path** — correcting a report means delete + re-upload, so the row and the stored object can never describe different things.

11.8 Suggested Sessions, care plans and credits

PAT-SUGG-001 — The Suggested Sessions entry appears only when something waits · P1

Steps. Before any therapist has suggested anything, check the sidebar. Then have `THR-SUGG-001` send a suggestion, and reload. **Expected Result.** Before: **no** Suggested Sessions entry. After: the entry appears **above** Your Sessions — something waiting on the patient's answer outranks a list of what is settled.

PAT-SUGG-002 — Accept a therapist-proposed time · P0

Preconditions. THR-SUGG-001 has proposed a slot on an active programme with credits remaining. **Steps.** Open `/patient/dashboard/suggested`. Read the card. Tap **Accept this time**. **Expected Result.** The card resolves to a booked state. An appointment is created via the package-session path: **auto-assigned to the locked therapist, auto-confirmed, and given its own Meet link**. `sessions_used` on the purchase increases by exactly **one**. A `reserve / consume` ledger entry is written with an idempotency key derived from the appointment id. The therapist's dashboard shows the new session. **A declined suggestion would have claimed nothing** — a suggestion is its own row, never an appointment in a new status.

PAT-SUGG-003 — Decline · P2

Steps. Tap **Decline** on a pending suggestion. **Expected Result.** The card shows `Declined`. No appointment. No credit moves. The therapist's control shows the declined state.

PAT-SUGG-004 — Button spam and a dropped connection · P0

Steps. Tap **Accept this time** three times rapidly. Then, on a second suggestion, throttle the network to Offline and tap **Accept**. **Expected Result.** Spam: exactly **one** appointment and **one** credit consumed. The control guards with a synchronous ref, so the second tap never reaches the server. Offline: the card **does not clear optimistically** — the patient is left exactly where they were, with an error, and can retry. The partial unique index allows at most one pending suggestion per purchase, so a double tap cannot create two.

PAT-SUGG-005 — A lapsed suggestion cannot be accepted · P1

Preconditions. A pending suggestion whose slot is now inside the booking lead time. **Expected Result.** The card reads **Suggestion lapsed** and offers no Accept. **Nothing writes an "expired" status** — `status` records explicit human actions only; lapse is computed at read time. Attempting acceptance via the API is refused.

PAT-CARE-001 — Read a therapist's recommendation · P0

Preconditions. THR-CARE-001 has written a care plan recommending Package P1, and **ADM-CARE-004` has approved it**. Until the clinic approves, the patient must see nothing — check that first: an unapproved recommendation absent from this screen is the feature working, not a missing fixture. **Steps.** Open `/patient/dashboard/suggested` (and `/patient/dashboard/health-profile`). **Expected Result.** An offer card showing the programme name, **session count**, **price**, validity, the therapist's *"Why this, for this patient"* text written **to** the patient, and their instructions. The same rows render on the therapist's chart and on the patient's Health Profile — one record, two readers. **There is no price field the therapist could have set:** the price comes from the admin's catalog row.

PAT-CARE-002 — Accept and pay for a recommended programme · P0

Steps. Tap the buy/accept action on the offer card. Complete Razorpay with `success@razorpay`. **Expected Result**

- The price charged is **re-derived server-side** from the live catalog row. It must equal the amount shown on the card.
- A `patient_package_purchases` row is created with a **frozen `package\_snapshot`** and `sessions_granted = 6`.
- **Session credits are granted: exactly 6.** Not 5, not 12.
- The care plan's status becomes `accepted`. **The thread is now closed** — a later recommendation opens a *new* plan with `supersedes_id` set.

- The payment does **not** refresh into an empty screen. In place of the offer card the patient reads *"Payment received — N sessions are yours"*, who will run them, and one next step. **If the screen goes blank after paying, that is a P0 defect** — it is the highest-intent moment in the product.
- **Your Programmes** appears in the sidebar (not "Your Packages") and lists the programme with `6 sessions`, `0 used`, `6 remaining`.
- A second attempt to buy the same plan is refused: `You've already bought this plan.`

PAT-CARE-003 — A recommended home-visit programme quotes travel correctly · P0

Preconditions. A care plan recommending HV2 (4 visits), patient address in a ₹150 area. **Expected Result.** The offer card shows **programme + travel + total** as three figures, with travel charged **per visit**: `₹8,999 + (₹150 × 4) = ₹9,599`. A card that printed `₹8,999` on a button that charged `₹9,599` would be a P0 defect — travel is per visit, not per purchase. Additionally: if an admin switches **Home Visit enabled** off, `/api/care-plan/create-order` must refuse the purchase with `Home visits aren't available right now. Please talk to your therapist.` A recommendation written before the service stopped must not stay purchasable.

PAT-SCHED-001 — The scheduler opens answered, not empty · P0

Feature. Everything needed to lay out the run is already known — how many sessions, how often the clinician recommended, the programme's gap and weekly rules, the lead time, the validity. Handing the patient a blank calendar asks them to redo arithmetic somebody has already done, immediately after paying.

Preconditions. A paid 6-session programme recommended at **2 a week**, with `min_gap_hours` 48 and `max_sessions_per_week` 2.

Steps. Tap **Choose my times** (straight after payment) or **Schedule sessions** (from Your Programmes). Read what is already selected. Change one date. Read the panel again.

Expected Result

- Dates are **already chosen** — as many as the bulk limit allows — spaced roughly 3 days apart, none inside the 12-hour lead time, none past the programme's expiry, and no more than 2 in any calendar week.
- Every proposed session is at the **same time of day**. A course at one hour is one thing to remember rather than six.
- The panel says *"We've picked N times for you — spaced 2 a week, the way your therapist recommended."*
- After an edit the heading becomes **Your times** and a **Put the suggested times back** link appears. The suggestion must **not** silently reapply itself — that is the calendar overruling the person using it.
- Nothing is booked until Confirm. The proposal is a suggestion; the server re-checks every rule on submit.

PAT-SCHED-002 — A clashing slot can be fixed without starting over · P1

Steps. Deliberately pick two sessions 24 hours apart on a programme with a 48-hour minimum gap, and confirm. **Expected Result.** *"N of M sessions scheduled"*, with the too-close one marked and the reason naming the gap. An amber line confirms the others **are** booked and nothing was charged twice. A **Pick another time** button returns to the calendar with only what is still unspent — the booked ones are gone from the selection, since re-offering them would invite booking the same slot twice.

PAT-SCHED-003 — Unbooked sessions keep asking · P0

Feature. The one thing a patient can buy and then receive nothing for. Paying is the hard part and it is done; what is left is a calendar step on a screen they have to think to visit. **Steps.** Pay for a programme and tap **I'll do it later**. Open `/patient/dashboard`. **Expected Result.** The activity feed carries a **needsYou** item — *"N sessions still to book"* — naming the programme and linking to Your Programmes. It stays until the balance is spent and disappears when it is. The figure must agree with the Your Programmes card exactly; a feed claiming a balance that screen disagrees with is a P0.

PAT-CARE-004 — A declined or withdrawn recommendation · P2

Steps. Decline a recommendation from the offer card. Separately, have an admin withdraw one (`ADM-CARE-002`). **Expected Result.** Declining closes it for the patient; withdrawing removes it from the patient's offers with the plan marked withdrawn. **A purchased plan can never be withdrawn** — the honest lane is a refund.

PAT-PKG-001 — Spend credits by booking package sessions · P0

Preconditions. `PAT-CARE-002` complete: 6 credits, therapist locked. **Steps.** Open `/patient/dashboard/packages`. Use the bulk scheduler to book **3** sessions, respecting the package's rules (min gap 24h, max 3/week). **Expected Result.** Three appointments are created, each **auto-assigned to the locked therapist, auto-confirmed, each with its own Meet link** — never through the admin's per-session assign flow. The widget reads `6 sessions · 3 used · 3 remaining`. Three `reserve` ledger entries exist, each keyed `reserve:<appointment_id>`. **Boundary.** Attempting a 4th session in the same week is refused by the package's max-per-week rule. Attempting more than `package_bulk_schedule_max` (default 8) slots in one request is refused with `Too many slots in one request.` Two slots less than 24h apart are refused by the minimum-gap rule.

PAT-PKG-002 — Credits cannot be overdrawn · P0 [SQL]

Steps. Book all 6 sessions, then attempt a 7th. **Expected Result.** Refused with `Every session in this programme is already used.` In the database, a ledger row that would overdraw fails the CHECK constraint and takes its transaction with it — **an impossible balance is impossible, not merely unwritten**.

PAT-PKG-003 — A locked therapist's clash never fails the booking · P1

Preconditions. The locked therapist already has a session at the requested time. **Expected Result.** The booking still succeeds, but the session lands ``requested` and unassigned` in the admin queue rather than failing. The patient is not told to pick another time.

PAT-PKG-004 — An expired programme · P2

Preconditions. A purchase whose `expires_at` has passed. **Steps.** Load `/patient/dashboard` (the expiry sweep runs at the top of that render). **Expected Result.** The purchase's status moves `active → expired` on that page load — **there is no cron**; the sweep is lazy and idempotent. The widget stops offering booking. A ledger `expiry` entry records the balance that lapsed.

11.9 Payments screen, profile and addresses

PAT-PAY-010 — Receipts · P2

Steps. Open `/patient/dashboard/payments`. **Expected Result.** Every paid session and purchase is listed once, with amount, date and what it was for. A cash-on-visit home purchase appears with its cash status and is **not** presented as a failed payment. Amounts match Money → Transactions exactly.

PAT-PROF-001 — Edit Profile sections · P2

Steps. Open `/patient/dashboard/profile`. Walk the five sub-sections: **Photo, Personal Details, Contact Details, My Addresses, Account Security**. **Expected Result.** Each is reachable from the sidebar's child list. Instant fields (photo, some details) save immediately; **gated** fields (the ones an admin must approve) submit a profile change request instead of writing directly, and say so. The request appears in Admin → Today → Approvals.

PAT-ADDR-001 — Address book · P2

Steps. In **My Addresses**, add the Patient A address, then add a second, then remove the second. **Expected Result.** Both are saved and selectable at home-visit checkout. Removing one does **not** alter any visit already booked — a visit's address is snapshotted onto the appointment at purchase.

PAT-EMPTY-001 — Empty states across the patient portal · P2

Preconditions. A freshly approved patient with nothing at all. **Expected Result.** The sidebar shows only **Overview, Book a Session, Health Profile, Edit Profile**, plus **Back to Home** at the foot of the nav. Sessions, Packages, Payments and Suggested Sessions are **absent**. The Overview shows a friendly empty feed and quick actions, not a blank panel or a zero-filled table.

12. Therapist test plan

12.0 Feature guide — the therapist's world

A therapist applies, waits for admin approval, sets a roster, is assigned patients, delivers sessions, writes clinical records, recommends treatment, and gets paid a revenue share on what they **delivered**.

Five rules shape almost every screen:

1. **Availability is three separate things and reads as three things.** A **weekly schedule** (what they normally work, expressed as working *periods*, not hourly cells), **exceptions** (one date that differs), and **time off** (`profiles.on_leave`, off the roster entirely). Leave never clears the schedule. An exception never edits the weekly template.
2. **Availability never touches an appointment.** Removing hours a session is booked into names who is affected and says the session stays as booked. Nothing cancels, moves or flags it. The booking wins.
3. **A therapist suggests; the patient books.** A therapist can propose a time on a programme locked to them. No slot is held. Only the patient's acceptance spends a credit.
4. **A therapist picks a package, never a price.** There is no price, session-count or discount field on a care plan version. Those columns do not exist.
5. **A patient's phone is masked, their email is not loaded at all**, and revealing the number is logged. Every cross-role free-text field is scanned.

12.1 Application, approval and access

THR-AUTH-001 — Apply to join · P0

Steps

1. Open `/therapist/login`. Confirm two tabs: **Sign In** and **Apply to Join**.

2. Tap **Apply to Join**.
3. Tap **Full Name**. Enter `QA Therapist A`.
4. Tap **Email Address**. Enter `qa.therapist.a@example.test`.
5. Tap the phone field. Enter `+91 90000 10001`.
6. Tap **Qualifications & License / Council Reg No**. Enter `MPT (Ortho), KSCP Reg 44821`.
7. Tap **Password**. Enter `QaTest!2024pass`.
8. Tap **Confirm Password**. Enter `QaTest!2024pass`.
9. Tap **Submit Application**.

Expected Result. The button reads `Submitting...`, then the form returns to the **Sign In** tab with a confirmation that the application is with the clinic. The `profiles` row exists with `role='therapist'`, `approved=false`, `active=true`. **No "check your email" step appears anywhere.** The application shows in Admin → Today → Approvals and raises that tab's badge. **Cleanup.** Leave for `ADM-APPR-002`.

`THR-AUTH-002` — An unapproved therapist is held at the door · P0

Steps

1. Sign in at `/therapist/login` as `qa.therapist.a@example.test` before approval.
2. Then, with that session cookie, call `POST /api/therapist/save-availability` directly with any valid body.

Expected Result. Step 1 lands on ``/pending-approval``, not the dashboard. Step 2 returns **403** with `Your account is not active – it is either awaiting admin approval or has been suspended.` **The API refusal is the one that matters** — a valid cookie must not be able to call around the UI.

`THR-AUTH-003` — Sign in after approval · P0

Preconditions. `ADM-APPR-002` approved Therapist A. **Expected Result.** Sign-in lands on `/therapist/dashboard`. The sidebar shows **Overview, Availability, Sessions, Earnings, My Patients, Edit Profile**, with **Back to Home** at the foot of the nav directly above Collapse (with children Photo / Public Details / Credentials / Account Security).

12.2 Availability — the roster

Feature guide. The editor is the same component on the therapist's own screen and on the admin's Roster. It edits **periods** ("Monday 9 AM – 1 PM and 2 PM – 6 PM"), and converts them to the hour rows the tables have always stored. Every existing schedule — including a sparse exception written one cell at a time by the old grid — must read back as exactly the same hours. A weekly save is a **compare-and-swap under a real row lock**, versioned by `therapist_schedule_state`.

`THR-AVAIL-001` — Set a weekly schedule with two periods a day · P0

Steps

1. Open `/therapist/dashboard/availability`.
2. Read the header line — it states the schedule's timezone.
3. On **Monday**, tap the "working" toggle so the day is on.
4. Tap the Monday start-time control. Set `09:00`. Tap the end-time control. Set `13:00`.
5. Tap **Add hours** on Monday. On the new period, set `14:00` to `18:00`.
6. Tap the copy control on Monday and copy Monday to Tuesday, Wednesday, Thursday and Friday.

7. Tap **Save**.

Expected Result. After saving, a "Saved" indication appears and the week summary reads Mon–Fri 9:00 AM – 1:00 PM, 2:00 PM – 6:00 PM. Reloading the page shows exactly the same periods — **the stored hour rows round-trip to the same periods**. Saturday and Sunday remain off. **Cross-check:** Admin → Sessions → Roster, opening QA Therapist A, shows the identical schedule in the identical editor.

THR-AVAIL-002 — A day with no hours must be explicit · P1

Steps. Turn Monday's working toggle on, remove all its periods, and tap **Save**. **Expected Result.** Refused with Add at least one set of hours, or mark the day unavailable. The save does not go through with an ambiguous day.

THR-AVAIL-003 — Removing hours a session is booked into · P0

Preconditions. Therapist A has a confirmed session on a Wednesday at 10:00. **Steps.** Edit Wednesday's period to 14:00–18:00 (dropping 10:00) and tap **Save**. **Expected Result.** Before saving, the editor **names the affected sessions** and states that **the session stays as booked**. After saving, the schedule changes and **the appointment is unchanged** — not cancelled, not moved, not flagged. The patient's dashboard and the admin's Schedule still show it at 10:00. **Availability and appointments are separate systems and the booking wins.**

THR-AVAIL-004 — A stale save is refused; a double-clicked Save is a no-op success · P0

Purpose. Prove the compare-and-swap. **Steps**

1. Open the availability screen in two browser tabs (Tab 1 and Tab 2), both showing the same schedule.
2. In Tab 1, change Monday to 10:00–14:00 and tap **Save**. Wait for "Saved".
3. In Tab 2 (still holding the old version), change Monday to 08:00–12:00 and tap **Save**.
4. Separately, in a fresh tab, change Tuesday and **double-click Save**.

Expected Result

- Step 3: refused with **HTTP 409** and a message telling the therapist to reload the latest. Tab 1's save is not overwritten.
- Step 4: **both identical requests succeed** — two identical requests carrying the same stale version are one logical change, so this is a **no-op success**, not a 409. Exactly one change is stored.

THR-AVAIL-005 — A therapist reads their exceptions but cannot write them · P0

Steps. On the availability screen, look for a control that creates a date exception. Then call POST /api/admin/set-availability-exception with the therapist's cookie. **Expected Result.** The therapist's own screen **shows** any exception on their record but offers no control to create one. The admin route returns **403 Forbidden**. Writing a date exception is an admin capability and stays one.

THR-AVAIL-006 — Leave leaves the schedule intact · P0

Steps

1. In the Leave panel, set leave from 2026-09-14 to 2026-09-18 with reason Annual leave. Save.
2. Look at the weekly schedule.
3. Remove the leave.
4. Look at the weekly schedule again.

Expected Result. The weekly schedule is **unchanged** in both step 2 and step 4 — leave never clears it, and there is nothing to restore on the way back because nothing was removed. profiles.on_leave reflects

the state. The admin's Roster shows the therapist as on leave for those dates.

THR-AVAIL-007 — An exception owns only its own date · P1

Preconditions. An admin has set a `2026-09-15` exception of `14:00-18:00` for Therapist A (`ADM-ROST-003`). **Expected Result.** 15 September shows `14:00-18:00` ; every **other** Tuesday still shows the weekly template's hours. The weekly template itself is unchanged.

THR-AVAIL-008 — Roster changes never move a booking or the patient's picker · P0

This is the regression that guards the whole design. See `XCFG-ROSTER-001` .

12.3 Sessions

THR-SESS-001 — Assigned sessions appear; unassigned ones do not · P0

Steps. Sign in as Therapist A and open `/therapist/dashboard/sessions` **before** the admin assigns anything. Then have `ADM-SESS-003` assign Patient A's paid session to Therapist A, and reload. **Expected Result.** Before assignment: the session is **absent**. A `requested` , unassigned session belongs to no therapist. After assignment: it appears under Upcoming with the patient's name, the concern, the slot and its status. If Google credentials are configured, it also carries a Meet link.

THR-SESS-002 — Sessions is one list with filters · P2

Expected Result. Video sessions and home visits are in **one** Sessions screen with Upcoming/Past/Cancelled and a Video/Home-visit filter that appears only for a therapist who has both. There is no separate "Home visits" sidebar entry.

THR-SESS-003 — A patient's phone is masked and their email is absent · P0

Preconditions. `contact_masking_enabled` is on (the default). **Steps.** Open an assigned session card and read the contact area. Then open the browser's **View Source / Network** and search the page HTML for the patient's full phone number and for `qa.patient.a@example.test` . **Expected Result**

- On screen: the phone reads in the masked form `+91 ..... *210` — country prefix and last three digits only. **The email is not shown at all.**
- **In the page source, the plaintext phone number must not appear anywhere**, because masking happens where the rows are loaded, not in the component. The email must not appear either — it is not loaded onto therapist surfaces.
- A patient with no number shows `No number on file` .

THR-SESS-004 — Reveal a contact inside the join window · P0

Preconditions. A confirmed **video** session whose slot is within the join window in **real** time (the route uses the server clock). Reason: this route's window check is server-side. **Steps.** Tap **Show number**. Read the number. Then repeat on a session whose slot is a week away, and on a cancelled session. **Expected Result**

- Inside the window: the full number is revealed, and a `contact_reveal_log` row is written. **The log write is not best-effort** — if it fails, the reveal is refused with `Could not show the number just now. Please try again.` A reveal with no trace is the one outcome this route must not produce.
- Outside the window: refused with the route's own explanation (403).
- Cancelled session: refused.

- A **home visit** is revealable **any time on the visit's own day**, not merely in a join window — verify this separately.
- Admin → Settings → Team & Access shows the reveal log. It is **admin-read-only and append-only by trigger**: attempting to update or delete a row raises, even with the service role.

THR-SESS-005 — Completing a session is gated two ways · P0

Purpose. `status='completed' && payment_status='paid'` is the exact condition that makes a therapist's revenue share payable, so the route refuses two things.

Steps

1. On an **unpaid, unprogrammed, no-cash** confirmed session, tap the complete control.
2. On a **paid** confirmed session whose slot is still in the future, tap the complete control.
3. Simulate a time after the slot and try step 2 again.
4. On a paid confirmed session whose join window has genuinely opened in **real** time, tap the complete control, read the dialog, and confirm.

Expected Result

- Step 1: refused (409). Nothing may be completed with no payment, no programme behind it and no cash recorded. A cash home visit must collect first.
- Step 2: refused (409) — nothing may be completed before the join window in which it could have been started.
- Step 3: **still refused**. The simulated clock is client-side only; this gate is server-side. **This is correct behaviour, not a defect**.
- Step 4: the dialog reads *"Mark this session as done? You'll be asked to rate it next."* After confirming, the session becomes `completed`, `completed_at` is stamped, and the therapist's Earnings figure increases by their share.
- A session that is not `confirmed` is refused with `Only confirmed sessions can be marked completed`.
- A second completion attempt returns `This session was already updated – please refresh and try again.` (409). **No double payout**.

THR-SESS-006 — Mark a no-show · P2

Expected Result. The session is completed with `no_show=true`. It shows as `Marked as no-show` and the patient's rating control is suppressed with `This session was marked as a no-show – there's nothing to rate.`

THR-SESS-007 — Record cash on a home visit · P0

Purpose. The person holding the cash must not also decide how much the clinic knows about. **Steps.** On a cash-on-visit home visit, tap the collect-cash control. Observe what the form asks for. Then inspect the network request body. **Expected Result.** The therapist **asserts that money changed hands**; they do **not** type an amount. The request body carries **an appointment id and nothing else** — the total is reconstructed server-side from the purchase using the same per-visit maths that booked it. If the UI ever offers an editable amount field here, that is a P0 defect. The honest exception (a patient short of cash) belongs to **Admin → correct cash amount**, which requires `money` scope, a mandatory reason, a CAS on the figure being replaced, and writes a `cash.correct_amount` audit row — and refuses a visit whose cash has already been remitted. A second attempt returns `This visit's payment has already been recorded.`

THR-SESS-008 — Session notes · P1

Steps

1. Before a session has taken place, open the note dialog.
2. After completion, open it and fill: **What did you treat today?** L4-L5 segmental mobilisation, glute med activation ; **Techniques and dosage** Grade III PA mobilisation ×3 sets, 30s hold ; **How did the patient respond?** Reported easing within the session, straight-leg raise improved ; **Home exercise prescribed** Cat-camel ×10, twice daily. Walking 15 min. ; **Plan for the next session** Progress to loaded hinge if pain stays under 3/10; reassess SLR ; **Anything to watch** Reports night pain – reassess if it persists . Save.
3. Edit the note within 24 hours and change one field.
4. Sign in as Patient A and search their entire portal and their exported PDF for the note text.

Expected Result

- Step 1: refused — You can write the note once the session has taken place.
- Step 2: saved. It appears on the therapist's chart. **Completion is never blocked on a note** — the nudge is a "Notes to write" figure on Overview and a feed item.
- Step 3: succeeds, and a session_note_revisions row records what it replaced. After 24 hours, editing is refused.
- Step 4: **the note text appears nowhere the patient can see, including the PDF export.** session_notes has no patient select policy and must never get one.
- Submitting with required fields blank: Fill in what you treated, how the patient responded, and the plan for next time.
- An unknown field in the payload: Note contains unknown fields.

12.4 Clinical records

THR-PAT-001 — My Patients, with a view switch not a second entry · P2

Steps. Open /therapist/dashboard/health-profile . Toggle **Patients** ↔ **Programmes**. **Expected Result.** The same patients, arranged two ways — by name, and by package purchase. **Programmes is a toggle, not a sidebar entry**, and it only appears for a therapist who actually has package patients.

THR-HP-001 — Triage a new patient · P0

Purpose. The therapist owns the **first fill**, and it is **not reviewed**. **Preconditions.** Therapist A is assigned to Patient A, who has no condition profile.

Steps

1. Open Patient A's chart at /therapist/dashboard/health-profile/<patientId> .
2. Tap the control that starts triage.
3. Answer **How old is the patient?** → 18 to 64 .
4. Answer **What brought them in?** → Injury, strain or overuse .
5. Answer **Any of these present?** → tick None of these .
6. Observe whether the milestones question appears.
7. Read the suggestion panel.
8. Confirm the suggested condition type.

Expected Result

- Step 2: the dialog shows **all four questions at once, with headings** — the clinician's surfaces deliberately invert the patient's one-question-at-a-time pacing, because a clinician filling this after every

assignment wants to scan it.

- Step 6: the milestones question is **not shown** — it only appears when age is `Under 18`.
- Step 7: **Orthopaedic** is suggested, **with its reason stated**, and is **not auto-accepted**. The therapist confirms or overrides.
- On confirming, an already-`approved` `condition_change_requests` row is written, so the triage appears in the ordinary Review History with no new concept and no queue.
- **Nowhere on a patient-facing screen do the words "triage", "onboarding" or "specialty" appear.** A patient is shown the care ("Orthopaedic care"), never the category word.

THR-HP-002 — Write the patient's first record (live, no review) · P0

Steps. Continue from triage into the seven orthopaedic questions and enter the §8.4 answers. Submit.

Expected Result. The record is written **live** — there is no approval queue in front of it. The patient's own Health Profile **unlocks** immediately (see `PAT-HP-002`). The route needs only that the therapist is **assigned** to the patient. If the therapist is not assigned, it is refused with `You aren't assigned to this patient.`

THR-HP-003 — Editing a live record needs an approved grant · P0

Purpose. The line is **create versus edit**. **Steps.** With the record now live, attempt to edit the patient's answers on their behalf. Then request access, have an admin approve it (`ADM-PEOP-004`), and try again.

Expected Result. Before the grant: refused with `You don't have an approved access grant for this patient's health profile.` The **Request access to edit** card sits **inside the Pain Map card** — beside the thing it gates, not three sections above it — and states what is readable regardless and what needs approval. After the grant: the edit submits and goes to admin review. A second request while one is open: `You already have a pending or approved request for this patient.`

THR-HP-004 — Re-triage merges, it never replaces · P0 [SQL or careful UI check]

Purpose. This is the single most damaging possible regression in the clinical layer. **Steps.** With Patient A holding a full orthopaedic record, re-triage them as **Neurological** and complete the neuro question set with the §8.5 answers. Then inspect the profile. **Expected Result.** The neuro answers are stored **alongside** the orthopaedic ones in the same flat blob. **Every orthopaedic key survives** (hidden on screen, never deleted). The chart now renders the neuro summary card, the neuro snapshot strip and the neuro progress line. The Pain Map is **not** rendered. **If any orthopaedic answer is gone, stop and raise a P0** — the approve path must merge, never write the proposed data outright.

THR-HP-005 — Pain Map is orthopaedic and stays so · P0

Steps

1. On an **orthopaedic** patient, open the body-map surface. Tap **Lower back** on the figure. Record an exam through the dialog.
2. On the **neurological** patient (Patient C), look for the body map. Then call `POST /api/therapist/pain-assessments/submit` for that patient directly.

Expected Result

- Step 1: the region is chosen by **tapping the figure** (or a chip inside the dialog), never a `<select>`, and the chosen region stays in the dialog header while the clinician types. Questions are grouped, not listed flat. The exam posts **live with no review**, and is **append-only** — a re-assessment is a new row, so the UI can show a trend against the previous visit. Recording one requires only that the therapist is **assigned**.
- Step 2: the non-ortho page **does not merely hide the map — it never queries** ``pain_assessments`` **at all**, and the submit route answers **400**.

- Every user-facing exam figure is printed **out of ten**, never as a raw percentage. Last exam found 34% beside How you rate it 6/10 is a defect: both must read on the same 0–10 scale.

THR-HP-006 — Read access needs no request · P2

Expected Result. An **assigned** therapist can read the patient's intake and Pain Map with no grant. Only *editing the patient's own account of their history* needs one.

12.5 Recommendations (care plans) and suggested sessions

THR-CARE-001 — Write a care plan from the session note dialog · P0

Preconditions. A **completed** session that **this therapist ran** for Patient A. Package P1 exists and is recommendable.

Steps

1. Open the completed session's note dialog.
2. In the **Recommend treatment** panel, tap the **Condition** dropdown.
3. Read the list of conditions offered, and how they are grouped.
4. Select the session's own condition, then tap the session-count chip for 6 sessions.
5. Read the four read-only figures shown beneath.
6. Tap **How often, per week** and select 2 a week.
7. Tick **Needs hands-on treatment**.
8. Tap **Why this, for this patient**. Enter Your range has improved but the pain returns after a day at your desk. A structured block will hold the gains.
9. Tap **Anything they should do or know**. Enter Keep up the walking between sessions. Book the first one within a fortnight if you can.
10. Read the line above the submit button.
11. Submit.

Expected Result

- Step 3: **conditions are grouped by condition type** — Orthopaedic, Neurological, Paediatric — from treatment_categories.specialty. A category an admin has not tagged appears under **General** and still works — that is where the clinic's general consultation sits, and "General" is deliberately **not** a fourth condition type: a patient's own health profile is only ever ortho, neuro or paediatric. The panel **never shows a programme by name**: a clinician answers "which condition" and "how many sessions", and those two pick the catalogue row.
- Step 3: **only programmes for this session's own condition are offered**. QA Neuro Rehab 8 Sessions must not be reachable for a QA Back & Spine Care session.
- Step 4: a **Delivered as** toggle (Video sessions / Home visits) appears **only** where the clinic sells both against that condition. A toggle with one option is not a decision the clinician has.
- Step 5: **Sessions `6`, Price `₹9,999`, Valid for `90 days`, Each session `60 min`** — all read-only, all from the admin's catalog row. **There is no price field, no session-count field and no discount field anywhere in this panel**. If one exists, that is a P0 defect: "the therapist set their own price" must be a thing the schema cannot express.
- Step 6: the frequency dropdown is capped by care_plan_max_frequency_per_week (default 5) and offers Leave open.

- Step 10: with `care_plan_requires_approval` **on** (the default), the panel says *"Goes to the clinic first. Your patient sees it once it is approved."* With it off, it says the patient sees it on their dashboard. **The copy must match the setting** — telling a clinician their patient can already see something sitting in a queue is a P0 defect.
- Step 11: the plan lands `status = 'pending_review'`, append-only and attributed. `care_plan_versions.source_appointment_id` is NOT NULL and is **re-derived from the appointment, not trusted from the body**. `care_plan_versions.expires_at` is **null** — the offer window is stamped at approval, not now.
- **The patient sees nothing.** Not a greyed-out card: the recommendation is absent from Suggested Sessions and from their Health Profile until an admin approves it (`ADM-CARE-004`).

THR-CARE-006 — The Overview names who is waiting to hear · P1

Feature. Every programme a patient can buy comes from a recommendation written after a completed session, so that one step is the whole distance between a delivered consultation and a course of treatment. It used to be carried only by an aggregate count, which reads as a score rather than as a list of people.

Steps. Complete a session for Patient A and write no recommendation. Open `/therapist/dashboard`.

Expected Result. The activity feed carries a **named** item — *"QA Patient A is waiting to hear what next"* — pinned by `needsYou` and linking to that patient's chart. It disappears once a recommendation is written, or once the patient's plan is accepted. A patient who **already has** a live or purchased recommendation must **not** appear; a patient whose plan was declined or withdrawn **should**, because that thread is open again. At most four are shown, most recently seen first, alongside the note nudge.

THR-CARE-008 — Writing a second one while the first is still queued · P1

Feature. Submitting again is allowed — it lands as a new version on the same thread, which is right when a clinician has genuinely changed their mind. Doing it *without being told* is how the same plan gets submitted twice by someone who assumed the first had failed.

Steps. With a recommendation already waiting for the clinic, open another completed session's note dialog for the same patient. **Expected Result.** The panel says *"You have already recommended a programme for this patient and the clinic has not decided yet — nothing has gone wrong, and your patient has not been asked for anything. Writing another replaces it."* The button reads **Replace it**, not **Add a recommendation**.

THR-CARE-002 — A recommendation needs a completed session this therapist ran · P0

Steps. Attempt to submit a care plan (a) for a patient this therapist is not assigned to, (b) against a session run by Therapist B, (c) against a session that is not completed. **Expected Result.** (a) `That isn't your patient.` (403). (b) and (c) refused by the route's own re-derivation. This is what makes "recommend to everyone and see who bites" **impossible rather than discouraged**.

THR-CARE-003 — A purchased plan is never re-versioned · P0

Preconditions. Patient A has **bought** the recommendation (`PAT-CARE-002`). **Steps.** Attempt to write a new recommendation for the same patient. **Expected Result.** The purchased thread is **closed**. A new recommendation opens a **new plan** with `supersedes_id` set — it does not add a version to the purchased one. Editing a purchased plan would change the description of something already paid for. `care_plans_one_open_per_patient` means the patient never sees two competing live recommendations — and covers a **queued** plan too, so a submission waiting on the clinic blocks a second one exactly as a published one does.

THR-CARE-004 — Versions are append-only by trigger · P1 [SQL]

Steps. In the Supabase SQL editor, attempt `update care_plan_versions set clinical_rationale='changed' where id='<id>';` and `delete from care_plan_versions where id='<id>;` **Expected Result.** Both **raise**. Only `is_current` and a **first** `expires_at` may change — the offer window is stamped once, at approval, and moving one already set raises too. This is enforced by trigger, not by RLS — every route writes with the service role, which bypasses RLS entirely.

THR-CARE-005 — Withdraw one's own recommendation · P2

Steps. Tap the withdraw control on an unpurchased recommendation, then on one still **waiting for the clinic's approval**. **Expected Result.** Both close; the patient's offer disappears where there was one. **A purchased plan cannot be withdrawn at all.**

THR-CARE-007 — The clinic's decision reaches the therapist · P0

Feature. A recommendation turned down silently is one that never happened, and the therapist is the only person who can put it right — they rewrite.

Preconditions. `ADM-CARE-005` has turned down this therapist's recommendation with the reason `This patient still has four unused sessions on their current plan.`

Steps. Open `/therapist/dashboard`. Then open that patient's chart. **Expected Result.**

- The activity feed carries a **needsYou** item: *"The clinic turned down your recommendation for QA Patient A"*, with the admin's reason as its detail. It is the **only** care-plan feed item marked `needsYou` — an approval is the expected outcome and marking it so would train the therapist to ignore the badge.
- An **approval** and an **approve-with-changes** also appear, not marked `needsYou`: a submission that vanishes into a queue and never reports back teaches a clinician to stop trusting the queue.
- On the chart, the thread reads **Not approved with the clinic's reason printed beneath it**, in red. The reason is the actionable half — "Not approved" says the recommendation is gone, and only the reason says what to write instead — and the feed item scrolls away while the chart is where a clinician goes to rewrite. A queued thread reads **Waiting for the clinic to approve**. Both are visible to the clinician and **neither is visible to the patient**.
- The thread being closed frees the one-open-plan slot, so a fresh recommendation can be written straight away.

THR-SUGG-001 — Suggest a session · P0

Preconditions. `therapist_suggestions_enabled` is **on** (it is on by default now; confirm in Settings → Booking Rules). A programme locked to this therapist with credits remaining.

Steps

1. On the programme's card, tap the suggest control.
2. Pick a date and time comfortably beyond the booking lead time.
3. Enter the note `Let's keep the two-a-week rhythm while it is working.`
4. Tap **Send suggestion**.
5. Tap **Send suggestion** twice more, rapidly, on a second programme.

Expected Result

- Step 4: the control changes to **Waiting on the patient** with a **Withdraw suggestion** option. **No slot is held** — a hold would need releasing, releasing would need a sweep, and there is no scheduled worker. The therapist's calendar is re-checked at acceptance instead.
- No appointment is created and **no credit is spent** — `sessions_used` is unchanged.

- Step 5: exactly **one** suggestion. The submit is guarded by a **synchronous ref** (a `disabled` attribute lands a render too late), and a partial unique index allows at most **one pending suggestion per purchase**.
- Negative cases: a slot too soon → `That time is too soon to book. Pick a later slot.`; a programme not locked to this therapist → `That isn't your programme.`; an inactive/unpaid programme → `That programme isn't active.`; a slot after expiry → `That time is after the programme expires.`; no credits → `Every session in this programme is already used.`; the therapist already busy → `You already have a session at that time.` (409); a note over 500 characters → `Your note must be 500 characters or less.`
- With the toggle off: `Suggesting sessions is switched off.` (403).

THR-SUGG-002 — Withdraw a suggestion · P2

Expected Result. The pending suggestion is closed. The patient's Suggested Sessions entry disappears (and the sidebar entry disappears if nothing else waits).

12.6 The contact-leak scanner (therapist side)

Feature guide. Every string one role writes and another reads is scanned. **Two tiers, deliberately:** a `block` hit (UPI handle, payment link, payment app) refuses the write; a `flag` hit (phone, email, social handle, bare URL) is delivered and recorded. The two tiers exist because this text is **clinical** — a scanner that treats digits as suspicious fires on every dose and every exercise prescription, and a check that cries wolf is a check nobody reads. Phone matching is the Indian mobile shape specifically (ten digits starting 6-9, optional `0 / 91`), not a loose digit run.

THR-LEAK-001 — A payment handle is blocked · P0

Steps. In **Why this, for this patient**, enter `Pay me directly on 9876543210@okhdfc, it's cheaper` and submit. **Expected Result.** The write is **refused**. The care plan is not created. A `communication_flags` row records the attempt with `blocked=true` and the offending content.

THR-LEAK-002 — A payment link is blocked · P0

Steps. In a suggestion note, enter `https://rzp.io/l/abcd1234 pay here`. **Expected Result.** Refused, recorded.

THR-LEAK-003 — A phone number is delivered and recorded · P1

Steps. In a suggestion note, enter `Call me on 9876543210 before the session`. **Expected Result.** The suggestion **is created** and the patient sees the note. A `communication_flags` row exists with tier `flag` and `blocked=false`. Admin → Settings → Team & Access shows it.

THR-LEAK-004 — Clinical text with digits does not fire · P0

Steps. In a session note, enter `Grade III PA mobilisation x3 sets, 30s hold. 10 reps, 2x daily. Order ref 90210.` **Expected Result. No flag at all.** If this fires, the scanner is broken in the way that matters most — a false positive on ordinary clinical text is what makes the whole control useless.

THR-LEAK-005 — The patient direction is record-only · P1

Steps. As Patient A, put `Call me on 9876543210` in the `/book` notes field and complete the booking. **Expected Result.** The booking **succeeds**. A flag is recorded. **A 400 at the last step of checkout costs a real booking, and a patient is not who this control exists to catch.**

THR-LEAK-006 — The scan mode switch · P1

Steps. Admin sets `contact_scan_mode` to `flag_only`, then to `off`. Repeat THR-LEAK-001 after each. **Expected Result.** `flag_only`: the UPI handle is **delivered** and recorded rather than refused. `off`: nothing is scanned or recorded. The setting is read in its **own** call and **fails open** — if the read fails, writes are allowed. (`contact_masking_enabled` is the opposite: it fails **closed**. Both defaults are deliberate and opposite.)

THR-LEAK-007 — The evidence tables cannot be edited · P0 [SQL]

Steps. Attempt `update communication_flags set content='x'` and `delete from contact_reveal_log` in the SQL editor. **Expected Result.** Both **raise**. Append-only **by trigger**, not merely by RLS — every route here writes with the service-role client, which bypasses RLS entirely. *An evidence record the evidenced party could edit is not evidence.*

12.7 Earnings and payouts

THR-EARN-001 — Earnings shows delivered work only · P0

Preconditions. Therapist A has a revenue share of `60`, one **completed paid** session at ₹1,999, and one **paid but not completed** session at ₹1,799. **Steps.** Open `/therapist/dashboard/earnings`. **Expected Result.** The earned figure counts **only the completed** session: $₹1,999 \times 60\% = ₹1,199.40 \rightarrow 119940$ `paise` (rounded). The paid-but-not-completed session contributes **nothing** — a therapist's share is earned by **delivering**, not by being booked. The screen answers both "what am I owed" and "what have I been paid" — Earnings and Payout Receipts are **one screen**, not two entries.

THR-EARN-002 — Home-visit share and travel · P0

Preconditions. Therapist A has `home_visit_revenue_share_percent = 65`; Therapist B has none set. Each completes one home visit of ₹2,499 with a ₹150 travel fee. **Expected Result.** Therapist A: $2499 \times 65\% + 150 = ₹1,774.35$. Therapist B falls back to their **online** share (55%): $2499 \times 55\% + 150 = ₹1,524.45$. **The travel fee is paid through in full and is never revenue** — a therapist must never fund their own transport.

THR-EARN-003 — Request a payout · P1

Steps. Tap **Request Payout**. **Expected Result.** The button becomes **Request Pending**, then **Under Review** once an admin starts review. A second request returns `You already have a pending payout request`. With no revenue share set: `Ask admin to set your revenue share % before requesting a payout`. The online method is refused with `Online payouts aren't available yet – use cash for now`.

THR-EARN-004 — Cash held nets off the payout · P0

See `FIN-PAY-003` for the full cross-check. From the therapist's side, the figure shown as payable must be **net of cash they are still holding**.

12.8 Therapist authorization (negative)

THR-SEC-001 — Therapist A cannot touch Therapist B's schedule · P0

Steps. With Therapist A's cookie, call `POST /api/therapist/save-availability` with a body naming Therapist B's id. Then call `POST /api/admin/save-therapist-availability` for Therapist B. **Expected Result.** Both refused. The therapist route writes **only** the caller's own schedule (the id in the body is

ignored or rejected); the admin route returns **403 Forbidden**. Verify in the admin Roster that Therapist B's schedule is byte-identical afterwards.

THR-SEC-002 — Therapist A cannot read an unassigned patient · P0

Steps. With Therapist A's cookie, open `/therapist/dashboard/health-profile/<Patient B id>` (a patient assigned only to Therapist B). Then call `POST /api/therapist/pain-assessments/submit` and `POST /api/therapist/care-plan/submit` for Patient B. **Expected Result.** The page shows no clinical data. The routes return **403** with `You're not assigned to this patient.` / `That isn't your patient.`

THR-SEC-003 — A therapist cannot book as a patient · P0

Steps. Signed in as Therapist A, open `/book`. **Expected Result.** Instead of the form, the **wrong-account** panel renders, naming the role and routing them to what is theirs. A direct `POST /api/appointments/create` with the therapist's cookie returns **403** with `This account can't book sessions. Sessions are booked under a patient account.` The same applies to all four purchase routes.

THR-SEC-004 — A therapist cannot reach the admin dashboard · P0

Steps. Signed in as Therapist A, navigate to `/admin/dashboard`. **Expected Result.** Redirected to `/get-started` — **never** to `/admin/login`, which would confirm the back office exists and name its door.

THR-SEC-005 — A suspended therapist is locked out immediately · P0

Steps. Admin sets Therapist A inactive. With the therapist's still-valid cookie, load the dashboard and then call any therapist API route. **Expected Result.** The dashboard redirects to `/account-suspended`. The API returns **403** `Your account is not active.` A live session cookie must not outlive suspension.

13. Hospital / partner test plan

13.0 Feature guide — the partner's world

A hospital is a **referral source**, never a clinical actor. It is **provisioned by an admin** (there is no partner self-signup that produces a working account — the public `/hospitals` page collects an *enquiry*, which becomes a B2B lead an admin converts). Once provisioned it can refer patients, watch their referrals move through a status pipeline, and see the commission it has earned.

The lifecycle end to end:

```
/hospitals enquiry → admin converts the lead → hospital account + referral code
  → hospital submits a referral → admin reviews it
  → admin assigns a therapist → admin sends an invite
  → the patient registers (via the invite, or by typing the referral code at booking)
  → the patient books and pays → the session is delivered
  → the partner commission is computed on NET revenue
  → it appears on the hospital's Earnings and on the admin's Money screens
```

The commission rule, precisely: the hospital's cut is `round(net_revenue_paise × hospital_share_percent / 100)` per appointment, where `net = paid - processed_refund`. It is taken on **net**, so a refund reverses it. It is **not** taken on a session whose therapist share is unknown — that whole appointment is excluded from the split and surfaced as a named count instead. **Never guess a percentage to make the numbers tie.**

Data isolation: a hospital sees its own referrals and its own commission, and nothing else. It never sees another hospital's rows, another patient's clinical data, or any admin screen.

13.1 Provisioning

HOS-LEAD-001 — The public enquiry form · P1

Steps

1. Open `/hospitals`.
2. Scroll to the enquiry form.
3. Fill it with Hospital A's details from §8.9 and submit.

Expected Result. A confirmation appears. A `b2b_leads` row is created. It shows in Admin → People → Partners and raises that tab's badge. **No account is created and no login works yet** — an enquiry is not a partner. **Negative:** submitting with a blank required field, or an invalid email, is refused with a readable message and creates nothing.

HOS-AUTH-002 — Admin provisions the hospital · P0

(Executed by the admin; verified here because it produces the partner's credentials.)

Steps

1. As Admin Full, open **People → Partners**.
2. Find the lead `QA Sunrise Hospital`. Tap the onboard control.
3. Enter **Organisation Name** `QA Sunrise Hospital`, **Contact Person** `QA Hospital Admin A`, **Email** `qa.hospital@example.test`, **Revenue Share %** `10`.
4. Submit.
5. **Write down the generated password and the generated referral code.**

Expected Result. An account is created with `role='hospital'`, `active=true`, a generated `referral_code`, and `revenue_share_percent=10`. The one-time password is shown **once, in the response** — and **never written into the `admin\_activity\_log`'s `details`**, because that log is readable by every admin and a generated password there would be a credential leak. An `admin_activity_log` row records *who onboarded whom and when*, with no password. **Negative:** a share of `-5` or `150` is refused with `Revenue share must be a number between 0 and 100`. A missing email is refused. Re-submitting the same email is refused rather than creating a second account.

HOS-AUTH-001 — Hospital sign-in · P0

Steps. Open `/hospital/login`, sign in with `qa.hospital@example.test` and the generated password.

Expected Result. Lands on `/hospital/dashboard`. The sidebar reads **Overview, Refer a Patient, Your Referrals, Earnings, Edit Profile**, with **Back to Home** at the foot of the nav directly above Collapse (children: Logo, Organisation Details, Contact Preferences, Account Security). The money word on this sidebar is **Earnings** — matching the therapist. It must not read "Revenue & Payouts" or any third name for the same thing.

HOS-AUTH-003 — A suspended hospital is locked out · P1

Steps. Admin toggles the hospital inactive. With the hospital's cookie, load the dashboard, then call `POST /api/hospital/withdraw-referral`. **Expected Result.** Dashboard redirects to `/account-suspended`. The API returns **403**.

13.2 Referrals

HOS-REF-001 — Submit an online referral · P0

Steps

1. Open `/hospital/dashboard/refer`.
2. Tap **Patient Full Name**. Enter `QA Referred Patient C`.
3. Tap **Patient Phone Number**. Pick the country and enter `9876543210`.
4. Under **Session Type**, select `Online`.
5. Tap **Address**. Enter `8, 100 Feet Road, Indiranagar, Bengaluru`.
6. Tap **Preferred Language**. Enter `English`.
7. Tap **Medical Issue**. Enter `Right-sided weakness following a stroke six weeks ago`.
8. Tap **Treatment Needed**. Enter `Gait and balance retraining, twice weekly`.
9. Submit.

Expected Result. A teal confirmation: *"Referral submitted — our team will review and reach out."* The form resets and the Session Type returns to `Online`. The referral appears under **Your Referrals** with status **Pending Review**. It appears in Admin → People → Partners and raises the badge. **The Pincode field is not required for an online referral.** The phone field clears with the rest of the form. **Negative:** submitting with the phone blank or malformed is refused with `Enter the patient's phone number so our team can reach them.` The number is required because the clinic **rings this patient before sending the registration link** — see `ADM-REF-001`.

HOS-REF-002 — A home-visit referral requires a pincode · P1

Preconditions. The admin master switch **Home Visit enabled** is on (otherwise the Session Type option is absent, which is itself the expected behaviour when it is off — verify that first). **Steps.** Repeat `HOS-REF-001`, selecting `Home visit`, and submit with the **Pincode** field blank. Then submit `56003`. Then submit `560038`. **Expected Result.** Blank and `56003` are both refused with `Enter the patient's 6-digit pincode for a home visit referral.` `560038` succeeds. With the master switch off, the **Home visit** option is not offered at all — a partner must not be offered a delivery mode the platform has not turned on.

HOS-REF-003 — Duplicate referral prevention · P1

Steps. Submit the identical referral (same patient name, same medical issue) a second time. Also double-tap the submit button on a fresh referral. **Expected Result.** The double-tap produces exactly **one** row. For a genuine repeat submission, the second row appears in the list — **and the admin's queue makes the duplication visible** so it can be declined rather than silently creating two patient journeys. (See §19 — the exact duplicate-detection behaviour on a genuinely re-typed referral is one of the items flagged for confirmation.)

HOS-REF-004 — The referral status pipeline · P0

Steps. Watch one referral through every state, driving each transition from the admin side.

Stage	Driven by	Status shown to the hospital
Just submitted	Hospital	Pending Review
Therapist assigned	Admin → assign referral	Therapist Assigned
Invite sent	Admin	Invite Sent

Stage	Driven by	Status shown to the hospital
Patient registered	Patient uses the invite / referral code	Registered
Refused	Admin → decline referral (reason required)	Declined

Expected Result. Each transition is reflected on the hospital's **Your Referrals** screen. The hospital sees **status only** — never the patient's clinical record. Declining without a reason is refused with `A reason is required to decline.`

HOS-REF-005 — Withdraw a referral · P2

Steps. On a **Pending Review** referral, tap the withdraw control. Then try to withdraw one whose status is **Invite Sent**. **Expected Result.** The pending one is withdrawn. The invited one is refused with `An invite has already been sent for this referral, so it can't be withdrawn`. The same rule guards the admin's decline path with its own wording.

HOS-REF-006 — Referral attribution reaches the patient · P0

Steps

1. As Patient C, open `/book`.
2. Complete Step 1, then on Step 2 enter the Patient C details and, in **Referral Code**, enter Hospital A's code. Tab out of the field.
3. Read the validation line.
4. Complete the booking and pay with `success@razorpay`.

Expected Result. Step 3 shows `Checking code...` then, in teal, `Valid – referred by QA Sunrise Hospital`. After the booking, Patient C is linked to Hospital A. The hospital's **Your Referrals** shows the referral as **Registered**. **An unknown code (`ZZZZZZ`) blocks Continue** with `That referral code isn't recognized...`; a **blank** code does not block anything.

HOS-REF-007 — Registration through the invite link · P1

Steps. Use the invite link the admin copied (Admin → Partners → **Copy invite link**) in a private window, and register. **Expected Result.** The register card is pre-associated with that referral. The account is created with a session immediately (no email step). The referral becomes **Registered**. Attribution is set without the patient having to type a code.

13.3 Partner money

HOS-MONEY-001 — Earnings reflects delivered, paid, non-refunded work · P0

Preconditions. Patient C (referred by Hospital A, share `10%`) has **one completed paid** online session at ₹2,499 and **one paid, cancelled-and-refunded** session at ₹1,999. Therapist A's share is configured.

Steps. Open `/hospital/dashboard/revenue`.

Expected Result

- The completed session contributes `2499 × 10% = ₹249.90 → 24990 paise`.
- The refunded session's net is `₹1,999 – ₹1,999 = ₹0`, so it contributes **₹0** — a refund reverses the partner's commission, because the commission is a cut of money **kept**.
- The figures on this screen match Admin → Money → Breakdown's `hospitalCutPaise` for the same range exactly.

- Balances are **not date-filtered**; flows are. The screen labels which is which.

HOS-MONEY-002 — A patient whose therapist share is unset is excluded, not guessed · P0

Preconditions. Therapist B has **no** revenue share configured; Patient C had a completed paid session with Therapist B. **Expected Result.** That appointment is **excluded from the split entirely** — it contributes to Gross, Refunds and Net on the admin screens, but to **none** of therapist cut, hospital cut or clinic share. Admin → Money shows it in the named excluded count and excluded revenue. **No commission is estimated for the hospital.**

HOS-MONEY-003 — A hospital with a share of 0 is not the same as one with none · P1

Purpose. These are two different states and must not collapse. **Expected Result.** A patient **not** hospital-referred → hospital cut of 0, and the appointment stays in the split. A patient referred by a hospital whose share is **not configured** → the appointment is **excluded** from the split. If a 0% hospital cut and an unconfigured hospital produce identical figures, the split maths has lost the distinction and that is a P0 defect.

13.4 Hospital isolation and authorization

HOS-SEC-001 — Hospital A cannot see Hospital B's referrals · P0

Steps. Sign in as Hospital A. Open **Your Referrals**. Then, with Hospital A's cookie, call `POST /api/hospital/withdraw-referral` with a referral id belonging to Hospital B. **Expected Result.** The list contains only Hospital A's rows. The API returns **403/404** and Hospital B's referral is unchanged.

HOS-SEC-002 — A hospital cannot open a patient's clinical record · P0

Steps. As Hospital A, navigate directly to `/therapist/dashboard/health-profile/<Patient C id>` and to `/patient/dashboard/health-profile`. Then call `POST /api/medical-documents/view` with a document id belonging to Patient C. **Expected Result.** Both pages redirect to `/get-started` (a signed-in user of the wrong role is never shown another role's dashboard). The document route is refused — the metadata row does not come back under the hospital's own RLS-scoped client, and **the row coming back is the authorization.**

HOS-SEC-003 — A hospital cannot reach the admin dashboard · P0

Expected Result. `/admin/dashboard` redirects to `/get-started`, never to `/admin/login`.

HOS-SEC-004 — A hospital cannot book as a patient · P0

Steps. As Hospital A, open `/book`. **Expected Result.** The wrong-account panel renders and tells the hospital to **refer** instead. A direct `POST /api/appointments/create` returns **403**.

HOS-PROF-001 — Edit Profile · P2

Steps. Open `/hospital/dashboard/profile` and walk **Logo, Organisation Details, Contact Preferences, Account Security**. **Expected Result.** The page is named **Edit Profile** — not "Account Security", which named one section of the page rather than the page. Changing the organisation name updates what the admin's Partners screen shows. A password change signs the partner out of other sessions or requires re-authentication, per the security section's behaviour.

Expected Result. The same shape as every other dashboard: a strip of four figures, then the activity feed, then quick actions — in that order. Items still waiting on the partner are pinned to the top of the feed and counted.

14. Admin test plan — Part A (Today, Sessions, People, Catalog)

14.0 How the admin dashboard works

The whole back office is **one page** at `/admin/dashboard` making roughly forty queries. The screen you see is chosen by `?section=&tab=`. Three facts follow, and each has a testable consequence:

1. **Tab state is written with the History API, never a router navigation.** Moving between two already-rendered screens must **not** re-run the page's queries. If switching tabs shows a loading spinner or takes seconds, that is a defect.
2. **A deep link server-renders its screen.** `/admin/dashboard?section=money&tab=payouts` must paint Payouts directly — not paint Today first and jump.
3. **An unknown tab key falls back to the section's first screen.** So a stale link *looks* like it works while quietly landing somewhere else. Build links with the typed helper; never hand-write one.

Realtime. The dashboard subscribes to two channels: operational tables (bookings, payouts, profiles, care records) on a **short** cooldown, and catalog/settings tables on a **much longer** one. It fires on the **leading edge** — the first change lands immediately and only the burst behind it is collapsed. A refresh re-runs every query on every screen, which is why the cooldowns exist.

Every mutating admin route writes an `admin\_activity\_log` row, after the route's compare-and-swap claim, so the log cannot record a settlement or cancellation that lost its race. The write is **best-effort and never throws** — an audit failure must not block the action it describes. The log has a select policy and **deliberately no insert policy**: the service role is the only writer, so it is append-only from any session. **A generated password never appears in `details`.**

14.1 Today

ADM-TODAY-001 — The Today screen · P0

Feature. "Everything waiting on you, in one list." Figures, queues and the activity feed on **one** screen, because splitting them meant an admin checked one and missed the other.

Steps. Sign in as Admin Full. The dashboard opens on **Today → Today**. **Expected Result.** A strip of four figures, the inbox queues, and the activity feed — in that order. Each inbox row shows a **count**, a **one-line hint** saying why it matters, and links to the section/tab where that work is actually done. Rows representing **money at stake** render in red as urgent. The **Today** badge equals the sum of the inbox counts. Every row's destination must exist: tapping each row must land on a real screen, never on a section's first tab by accident.

ADM-TODAY-005 — A count opens the rows it counted · P1

Feature. A figure or a queue row that opened an unfiltered list made the reader redo the filtering by hand, and made the number look wrong. The link carries a `?view=` preset the target screen applies on arrival.

Steps

1. With at least one unassigned session on record, note the **Unassigned sessions** figure on Today.
2. Tap it.
3. Read the **All Sessions** heading count and the status filter.
4. Tap **Sessions today**, then **Cash to remit**, from Today.
5. Return to **Today** in the sidebar, then open **Sessions → All Sessions** from the sidebar.
6. Sign in as **Admin Clinical** (no Money scope) and read the same strip.

Expected Result

- Tapping **Unassigned sessions** opens All Sessions with the status filter on **Needs a therapist**, the header count equal to the figure, and the list starting at **page 1**. Every other filter — mode, payment, therapist, patient, date range — is cleared, including any remembered on this device: a remembered filter that hid rows the figure counted is the same bug in a subtler form.
- **Sessions today** filters the date range to today; **Cash to remit** opens Money → Payouts filtered to therapists with a balance.
- The preset is **one-shot**. Returning through the sidebar drops `view` from the URL, and re-opening All Sessions shows the filters as the admin last left them, not the preset again.
- On All Sessions itself, the **No therapist**, **Today** and **Home visits** figures filter the list **in place** (no page navigation) and tapping the applied one clears it.
- An admin whose scope cannot open the target section sees the figure **without a link** — never a link into a 403.

ADM-TODAY-002 — Inbox counts are live · P1

Steps. In a second browser, have a patient book a session. Watch the admin's Today screen without reloading. **Expected Result.** The unassigned count and the badge update within the operational channel's cooldown. The **first** change appears immediately (leading edge); a burst of ten bookings collapses into one refresh.

ADM-APPR-001 — Approve a pending patient · P0

Steps

1. Open **Today → Approvals**.
2. Find the row for `QA Patient B`.
3. Tap **Approve**.

Expected Result. The row leaves the queue and the badge decreases by one. `profiles.approved` becomes true. Patient B can now sign in and reach `/patient/dashboard` instead of `/pending-approval`. An `admin_activity_log` row records the approval with the actor, the target and the timestamp. **Approvals live under Today, beside the inbox that counts them — never on the patients directory.** A queue is not a person. **The screen states what it is deciding**, because the two halves are not the same decision: a therapist here is a credentials check, while a patient here registered *without* booking — anyone who genuinely attempts a payment is approved automatically at that moment. Approving a patient from this list changes what they can see, never whether they can pay. Confirm that line is present; without it a new admin cannot tell what they are being asked to judge.

ADM-APPR-002 — Approve a therapist · P0

Same as above for `QA Therapist A`. **Expected Result.** The therapist can sign in and reach the dashboard, and their availability routes stop returning 403.

ADM-APPR-003 — Decline an account · P1

Steps. Tap **Decline** on a pending row, leaving the reason blank; then with a reason. **Expected Result.** Blank is refused (A reason is required to decline.). With a reason, the row leaves the queue, the account does not become approved, and the decline is audited.

ADM-APPR-004 — Profile change requests · P1

Preconditions. PAT-PROF-001 submitted a gated field change. **Steps.** Approve one request; decline another with a reason. **Expected Result.** Approving writes the new value onto the profile; declining does not. Both are audited. A stale decision on an already-decided request is refused with This request has already been reviewed .

ADM-RISK-001 — The Risk queue · P1

Feature. Suspicious patterns surface here, written by a **bounded lazy sweep after the Today render** — a wall-clock budget checked between rules, and a five-minute minimum interval, because realtime refreshes this page on every booking.

Steps. Open **Today → Risk**. Read the list, then open one signal. **Expected Result**

- **A flag is never an accusation and never carries a penalty.** Nothing is suspended, held or hidden because a rule fired. **The Risk tab carries no action buttons** — acting on a finding means going to the screen that owns that action and doing it deliberately, with its own audit row. If an action button appears here, that is a defect.
- Each signal shows a severity (Low / Worth a look / Look now), a status (Needs a look / Being reviewed / Nothing in it / Acted on), and **links to the rows behind it** — evidence stores row ids, not a score, because an admin who can only see a verdict cannot disagree with it.
- The eight rules are contact_leak , completion_without_payment , early_completion , cash_variance , contact_reveal_volume , manual_adjustment_volume , plan_conversion_low , post_consultation_dropout . **The last two ship disabled** — a threshold invented before anyone knows the clinic's normal rate fires on everyone or on nobody.

ADM-RISK-002 — Reviewing a signal requires a real note · P1

Steps. Review a signal with a note of ok (2 characters), then with Checked the two sessions, both legitimate. **Expected Result.** The short note is refused — the minimum is **ten characters**, enforced by a CHECK, because "dismissed" with no reason reads the same as "not read". Reviews are **append-only**. Closing a signal frees its slot, so a repeat after a dismissal is raised **fresh** — that is correct, it is new information.

ADM-RISK-003 — Thresholds are editable, and the queue is full-scope only · P1

Steps. Edit a rule's threshold on the tab. Then sign in as Admin Ops and open Today. **Expected Result.** The threshold saves and the next sweep uses it. As Admin Ops, the **Risk tab is not shown, and the page does not even fetch the signals** — a signal names a colleague and quotes what they wrote.

14.2 Sessions

ADM-SCHED-001 — Schedule (calendar) · P1

Steps. Open **Sessions → Schedule**. Navigate months. Tap a day with sessions. Tap one session.

Expected Result. The calendar shows sessions by day. Tapping a day opens its panel; tapping a session

opens the **same** ``SessionDetailDrawer`` that All Sessions opens. **There is one detail surface, not two.**

ADM-SESS-001 — All Sessions is one filterable list · P0

Feature. All Bookings, Session Story, the calendar's day panel and the home-visit queue were four lists over the same rows. They are now **one** list plus the calendar, both opening the same drawer. **If you find yourself wanting a second list of sessions, the answer is a filter.**

Steps. Open **Sessions → All Sessions**. Apply a status filter, a mode filter and a date range. Reload the page. Then change the page size and reload again. **Expected Result**

- One list containing every session, video and home visit alike. Home-visit specifics (address, travel fee, cash) are a **panel inside the drawer**, not a parallel screen.
- Filters are **remembered per browser** — but **the date range is not**, because it goes stale.
- The list ends in the standard pager: a **Show N per page** field (default **25** on this screen — it is the one an admin lives on, where ten rows made every working day a paging exercise; other lists keep the shared default of 10, remembered per browser under this list's own key), Previous/Next that grey out at the ends, and an "x-y of n" count. There is **no arbitrary row cap with a "Show all" escape hatch** — that was the old behaviour, and "Show all" then painted every row anyway, which is the thing the pager replaced.
- Filtering, sorting, totals and **both exports** always run over the **whole filtered set** — only what is painted is paged. Otherwise a range total would start describing a page.

ADM-SESS-002 — Assign a therapist · P0

Steps

1. Open a paid, unassigned session's drawer.
2. Read the assign form.
3. Select `QA Therapist A`.
4. Submit.

Expected Result. The drawer's control reads **Assign a therapist**, never "Reschedule / Reassign" — nothing has been reassigned on a session nobody has ever been assigned to. If the patient requested a specialist, that therapist is **preselected and marked "(requested)"**, with the line "Patient requested this therapist" above the picker. An unassigned row on **All Sessions** and on the **Schedule** day panel carries a **Tap to assign** chip, and the reschedule control below the assign form is the one to use when the time has to move too. A **home visit** is assigned from the visit panel in the same drawer, not from a second copy of the online form. On assignment the session becomes `confirmed`, a Google Calendar/Meet event is created (if credentials are configured), the therapist now sees it, and the patient's card shows the therapist's name. The unassigned badge decreases. **Negative:** assigning a therapist who already has an overlapping session is refused with `This therapist already has another session that overlaps this time slot.` A session already over is refused with `This session is already over and can't be modified`. A stale submit returns `This session's status changed – please refresh and try again.`

ADM-SESS-003 — Meet sync failure is recorded, retried and capped · P1

Feature. Google sync **must never block a booking**. Failures are recorded on the appointment, re-attempted by a **lazy sweep at the top of the admin dashboard render**, and retried by hand from Sync Health. Because that sweep makes outbound calls from inside a page render, it is capped **three ways**: a wall-clock timeout per attempt, a few appointments per sweep, and an attempts-per-appointment counter.

Steps. Remove or invalidate the Google credentials. Assign a therapist to a paid session. Then open **Settings → System Health**. Tap **Retry** on the failed row several times. **Expected Result.** The

assignment **succeeds** and the session is confirmed — the booking is never blocked.

`google_calendar_sync_error` is recorded and the row appears in Sync Health, raising that tab's badge. Retrying increments the attempt counter; at the cap the row stays flagged as **needing a person** rather than being retried forever. A **manual Retry resets the counter**. Two overlapping attempts must not both create an event — each claims the appointment first, with a staleness window so a render that dies mid-attempt releases its row. Retrying a session that is not confirmed-with-a-therapist is refused with `Only confirmed sessions with an assigned therapist can retry Meet sync`. A concurrent retry returns `A sync attempt for this session is already running. Try again in a moment.` **Note:** a **home visit still gets a calendar event even when `google\_meet\_enabled` is off** — that toggle gates the Meet conferencing only, not event creation, because the invite email is the only outbound notification this platform sends.

ADM-SESS-003b — Patient and therapist join without being admitted · P1

Feature. Meet holds anyone it does not recognise in a waiting room until the meeting's owner admits them, and a patient signs in with whatever Google account they have — so without this, every session needed the clinic's own account to let both parties in, one at a time. Each new session's meeting is opened at creation. Settings → Booking Rules → **Join Without Approval** is the switch; Settings → System Health → **Waiting Room** lists the sessions where it did not take.

Steps. With the switch **on**, confirm a paid session. Open its Meet link in a browser signed in as a Google account that is *not* on the invite. Then open **Settings → System Health. Expected Result.** The link goes **straight into the call** — no "asking to be let in", and nobody has to admit anyone. `meet_access_open` is `true` and the session is **not** in the Waiting Room panel. The Join button's caption reads `Opens straight into the call – sign in to Google if asked.` once the join window is open, and `Opens N minutes before your session.` before it. **Negative:** with a refresh token minted before the `meetings.space.settings` scope (or the Google Meet API not enabled on the Cloud project), the **booking still succeeds and the link still works** — only the waiting room stays on. The session appears under Waiting Room with the 403 explained, is retried a couple of times automatically, then flagged as **needing a person**. **Open** re-attempts it and re-arms those attempts; on a session with no Meet link yet it is refused with `This session has no Meet link yet – retry the Calendar sync first`, and on a cancelled one with `This session is cancelled – its Meet space is gone`. **Note:** open access removes the **knock**, not the **sign-in**. A meeting organised by a personal Gmail account still requires every participant to be signed in to *some* Google account; only moving the organising account to Google Workspace allows a patient with no Google account at all to join.

ADM-SESS-004 — Edit, cancel, reopen and restore a session · P1

Action	Expected
Edit a booking's time/therapist	Saves, re-checks conflicts, audits. A session already over: <code>This session is already over and can't be modified</code>
Cancel with refund	The session is cancelled, the refund is processed, Money → Summary's Net Revenue drops while Gross is unchanged
Reopen a completed session	Only a completed session can be reopened (<code>Only completed sessions can be reopened.</code>); audited
Restore a cancelled/no-show session	Allowed only from those two states (<code>Only a cancelled or completed (no-show) session can be restored.</code>); a second attempt: <code>This session has already been restored.</code>
Mark paid by cash	A session already paid: <code>This session is already marked as paid</code>

ADM-ROST-001 — The Roster opens on therapists, not on a date · P0

Feature. The Roster is the clinic's planning record. It opens on a **list of therapists** rather than a calendar date and an eighteen-column grid, and it uses the **same editor** the therapist's own screen uses.

Steps. Open **Sessions → Roster**. Read the landing view. Open QA Therapist A. **Expected Result.** A list of therapists with a summary of what each works, plus their leave state. Opening one shows the **period editor**, not an hourly grid. The periods match exactly what the therapist saved in THR-AVAIL-001.

ADM-ROST-002 — An admin saves a therapist's weekly schedule · P1

Steps. Change Therapist A's Friday to 09:00–12:00 and save. **Expected Result.** Saved through the same compare-and-swap function, with the same stale-save 409 and the same double-click no-op success. The therapist's own screen shows the change. **No appointment is moved.**

ADM-ROST-003 — Set a date exception · P0

Steps. For Therapist A, set 2026-09-15 to 14:00–18:00 with the reason Clinic audit in the morning. Save. Then look at 22 September (the next Tuesday) and at the weekly template. **Expected Result.** Only 15 September changes. **Every other Tuesday still shows the weekly template's hours, and the weekly template itself is untouched.** Setting a date exception replaces that **whole day** in one function — it is not a partial merge.

ADM-ROST-004 — Set leave · P1

Steps. Put Therapist C on leave 2026-09-14 – 2026-09-18. **Expected Result.** profiles.on_leave is set; the roster shows them off; **the weekly schedule is untouched** and is still there when leave is removed.

ADM-ROST-005 — Roster authorization on every route · P0

Steps. With a **therapist's** cookie, call POST /api/admin/save-therapist-availability and POST /api/admin/set-availability-exception. With **Admin Finance's** cookie, call both again. **Expected Result.** All four return 403. The roster is sessions scope; Finance cannot open Sessions at all.

ADM-DELIV-001 — Delivery answers operational questions, not financial ones · P1

Steps. Open **Sessions → Delivery**. **Expected Result.** No-show rate, cancellation rate, repeat-booking rate and sessions-per-therapist. **These live under Sessions, not Money** — a no-show rate is about how the clinic runs, not about its books. All three metric slices (summary, breakdown, delivery) are computed from **one pass** of the same maths, so a figure here can never contradict the same figure on Money.

ADM-MONEY-GW-001 — Goodwill: take an amount off one session · P0

Feature. The lane for a session cut short, a therapist who ran late, or a patient in genuine hardship. Without it these are settled outside the system, where nobody can see them and the books never learn they happened.

Preconditions. An **unpaid** session on a ₹1,200 category. Signed in as an admin with **Money** scope.

Steps

1. Open the session from **Sessions → All Sessions** and find **Goodwill** in the drawer.
2. Enter 200 and a two-character reason. Submit.
3. Replace the reason with Their last session was cut short by a connection problem. Submit.
4. Re-open the drawer.
5. Separately, try 2400 (more than the session costs) on another unpaid session.
6. Separately, try it on a **paid** session.

Expected Result

- Step 1: the panel states the session price and says plainly that this reduces what the patient is asked for, and refunds nothing, because nothing has been paid.
- Step 2: **refused**. Under ten characters is rejected by the route and by a CHECK on the column — a discount nobody can explain a month later is indistinguishable from a mistake.
- Step 3: applied. The row records **all four facts**: list price `120000`, discount `20000`, source `goodwill`, and the reason. A `payment.goodwill_discount` audit row is written.
- Step 4: the drawer shows the amount, what they now pay, and the reason. It is **not** re-editable — the patient has been quoted it.
- Step 5: **refused**, not applied at ₹1. This is a number typed with the price on screen beside it, so more than the price is a typo (2400 for 240), and quietly charging ₹1 is worse than saying no. (*A configured offer behaves differently and is floored — see `ADM-SET-023`.*)
- Step 6: **409** — "Refund it instead." Money that has moved comes back through the refund route, with its own Razorpay call and its own audit.
- A **Clinical** or **Operations** admin gets 403: this changes what somebody is charged, which is a money capability whatever the reason for it.

ADM-MONEY-GW-002 — What discounting cost · P1

Steps. Apply a goodwill discount and let a first-session offer run, then open **Money → Costs**. **Expected Result.** A **Discounts given** figure, split by rule — the offer, goodwill, promo codes and both halves of an invite, each line appearing only when it has something behind it. It is **stated, not deducted** — the note says so — because a discount means less was collected and is already inside gross revenue as a smaller number; subtracting it from profit would count it twice. If Operating profit drops by the discount amount, that is a P0 defect.

ADM-PROMO-001 — Create a campaign · P1

Steps. As a **Money** admin, open **Money → Costs → Promo codes**. Switch codes on. Create `WELCOME200`, ₹200 off, total 50 uses, 1 per patient. Then try each of: a code with a hyphen in it; a percentage of 150; an end date before the start date; a second code called `WELCOME200`. **Expected Result.** The first saves and appears as **Running**. Each of the four is refused with a sentence naming the problem, not a raw database error. A `promo.create` audit row is written for the one that saved. A **Clinical** or **Operations** admin gets 403 on the route: this decides what every patient who types the code pays, which is a money capability however much it reads like catalog data.

ADM-PROMO-002 — A used code is paused, never deleted · P1

Steps. Have a patient claim a code at checkout, then try to delete it. Then pause it and have another patient try it. **Expected Result.** Delete is **not offered** on a code that has been claimed, and the route refuses it with "switch it off instead" — a paid session pointing at a campaign nobody can name cannot answer which rule gave the money away. Pausing works, and the next patient is told the code is **no longer available** rather than "not recognised".

ADM-PROMO-003 — The cap means what it says · P0

Steps. Create a code capped at **1** total use. Open checkout for it as patient A and pay. Then try it as patient B. **Expected Result.** Patient B is refused with "That code has been fully claimed." Repeat with patient A abandoning checkout instead of paying: after **30 minutes** the claim frees up and patient B can use it. A cap that can be exceeded by opening two checkouts is a P0 defect.

PAT-PAY-FREE-001 — The payment screen quotes what it charges · P0

Steps. As a patient with a first-session offer running (say 25% off), reach step 3 of the booking wizard.

Expected Result. The summary shows the session fee **struck through**, the discount named on its own line, and a **Total**. The button reads **Pay ₹<total> Now** with that same total. If the button quotes the list price while Razorpay opens at the discounted figure — or the reverse — that is a P0 defect: quoting one number and charging another is the one thing a payment screen must never do.

PAT-PAY-FREE-001b — A brand-new visitor is quoted the offer too · P0

Steps. With a first-session offer running, open `/book` **signed out**, pick a condition and a slot, and reach

step 3 without creating an account yet. **Expected Result.** The summary already shows the offer — fee struck through, "First session offer — ₹X off", and the discounted **Total** — before any account exists. This is the path the offer is *for*: a self-signup patient makes their account, their booking and their payment with one tap on this screen. Showing list price here and then opening Razorpay at the offer price is a P0 defect.

PAT-PAY-FREE-002 — A 100%-off code books without paying · P0

Steps. Create a promo code at **100%** off. As a patient, apply it at step 3. **Expected Result.** Total reads **Free**, the lock line changes to *"Nothing to pay — your discount covers this session in full"*, and the button reads **Confirm booking — free**. Tapping it books the session with **no Razorpay screen at all**. The session appears in the patient's dashboard as confirmed or pending exactly like a paid one. Being charged ₹1 instead is a P0 defect — that was the old behaviour and it charges a figure nobody was quoted.

PAT-PAY-FREE-003 — Free is decided by the server, never the browser · P0

Steps. With no discount running, POST to `/api/appointments/confirm-free` with a real unpaid

appointment id (browser console or curl, signed in as that patient). **Expected Result. 409**, *"This booking still has an amount to pay."*, and the booking stays **unpaid**. If a booking can be confirmed free by asking, every session in the app is free.

ADM-MONEY-FREE-001 — A free session still shows in the books · P1

Steps. After `PAT-PAY-FREE-002`, open **Money → Costs** and the session's own drawer. **Expected Result.**

The booking records the full price as list price, the whole of it as the discount, and `promo_code` as the source — so **What discounting cost** includes it. Amount paid is **₹0** and there is no payment/transaction row, because no money moved. A free session that recorded nothing would make the giveaway invisible, which is the figure that decides whether the campaign continues.

ADM-INVITE-001 — Invites: the two halves · P1

Steps. As a **Full** admin, open **Settings → Booking Rules → Patient invites**. Switch on, set the friend's

welcome to ₹300 and the reward to ₹200, and save. Open a patient's dashboard. **Expected Result.** The panel previews the exact sentence the patient will read, and it says the reward arrives **once their friend has had a session** — not on a signup. The patient's dashboard shows their own code, formatted in two halves, with a copy button.

ADM-INVITE-002 — What an invite refuses · P0

Steps. As the code's owner, try to use your own code. As a patient who has already paid for a session, try

to use somebody's code. As a patient who has already used one, try a second. **Expected Result.** All three refused, each with its own sentence: *"That's your own invite code."*, *"An invite code can only be used before your first session."*, *"You've already used an invite code."* The entry field is **not shown at all** to a patient who has already paid or already claimed — a field that can only refuse is worse than no field. A patient reaching the API directly gets the same three answers; none of these rules lives only in the browser.

ADM-INVITE-003 — A promise already made is kept · P1

Steps. Have a patient claim an invite while the welcome is ₹300. Lower it to ₹100, then switch invites off entirely. Open that patient's checkout. **Expected Result.** They still get **₹300** off. Amounts are snapshotted at claim, and the switch stops new claims rather than withdrawing one already made. Charging them the new figure — or nothing — is a P0 defect.

PAT-PAY-DSC-001 — The patient sees what they were given · P1

Steps. As a patient who received a discount, open **Payments** and tap the receipt. **Expected Result.** Three lines: the session price struck through, the discount named and negative, and **You paid**. A receipt that silently printed the lower number would tell the patient nothing about having been given something, which is the entire value of an offer.

ADM-CARE-001 — Recommendations: see every care plan · P0

Steps. Open **Sessions → Recommendations**. **Expected Result.** Three bands, in this order: **Waiting for your decision**, **Waiting on a patient**, **Answered and closed**. Every care plan in the clinic is listed with its patient, therapist, package, status and date. A care plan is now the **only** route by which a patient buys a programme, so the clinic must be able to see them all. The first band **renders even when empty**, saying so. A section that disappears when there is nothing in it gives an admin no way to tell "nothing waiting" from "I am on the wrong screen".

ADM-CARE-004 — Approve a recommendation · P0

Feature. A therapist's recommendation is a bill as well as a clinical note, and the clinic that carries it sees one before the patient is asked to pay it.

Preconditions. `care_plan_requires_approval` is **on** (default, at Settings → Booking Rules). `THR-CARE-001` has been submitted.

Steps

1. Open **Today → Overview** and read the Clinical group of the action inbox.
2. Follow **Recommendations waiting for approval** to the queue.
3. Read the card: patient, therapist, programme, sessions, price, frequency, the clinician's reasoning and their instructions to the patient.
4. Note the order of the cards and what the badge on each one says.
5. Tap **Approve**.
6. Check the patient's Suggested Sessions screen.
7. Attempt the same decision a second time.

Expected Result

- Step 1: the count is there, and it turns **urgent** only once something has been waiting over four hours — never merely because the queue is non-empty. A badge that is always on is a badge nobody reads, and this is the one queue with a patient on the other side of it who has been told nothing at all. The hint names how many are late.
- Step 2: the link lands on `?section=sessions&tab=recommendations` — the figure and the list it opens must agree.
- Step 4: the queue is **oldest first** — every other list on this screen is a record and reads newest-first, but this is work, and the person waiting longest is served next. Each badge reads **how long** it has waited (`Waiting 3 hours`), not the date it arrived; past four hours it turns red. A card that reads `2 September` when the thing arrived nine minutes ago tells an admin nothing they can act on.

- Step 4: where the patient still has unused sessions **or visits** on a live programme, the card says so in amber. The figure comes through the same ledger helper every other balance surface uses, so it cannot disagree with Catalog → Purchases after the ledger switch is flipped. It is **stated, never acted on** — a patient with sessions left may well need a different programme, and the clinician has seen them. This is the commonest reason to turn one down, and an admin previously had to leave the queue to find it out.
- Step 5: **one tap, no reason asked**. Approving is the outcome this queue exists to reach, and demanding a sentence meaning "fine" twenty times a day is how a reason column fills with `ok` and stops being worth reading; a plain approval's evidence is who and when, both already on the row. The plan becomes `active`; `reviewed_by` and `reviewed_at` are stamped; a `care_plan_reviews` row records the decision and the reviewer with a null reason; a `care_plan.approve` audit row is written. **`care\_plan\_versions.expires\_at` is stamped now** — the patient's answering window starts at approval, not at authoring, so a plan that waited in the queue does not arrive with its time already spent.
- Step 6: the recommendation is now visible and purchasable.
- Step 7: **409** — `Someone else decided this one first.` The decision is a compare-and-swap on `pending_review`, so two admins in the queue together cannot both decide.

ADM-CARE-008 — A stale offer is caught before the patient meets it · P0

Feature. Checkout re-reads the package and refuses on a mismatch rather than charging a different amount. On its own that means the *patient* discovers the clinic's stale data, by having their payment refused at the last step.

Steps. With a recommendation queued, go to **Catalog → Packages** and change that package's price. Come back and tap **Approve**. Then tap **Turn it down**. **Expected Result.** The approval is **refused** with a sentence naming the drift — *"This was written at ₹9,000 and the programme now costs ₹9,500. Approving it would quote one figure and charge another."* The plan is still queued and `reviewed_by` is still null: refused, not half-applied. Deactivating the package or clearing its **recommendable** flag refuses the same way. **Turning it down still works.** Refusing to let an admin close a thread because its package moved would trap exactly the recommendation that most needs closing.

ADM-CARE-005 — Turn a recommendation down · P0

Steps. On a queued recommendation, tap **Turn it down**. Submit with the reason `ok` (two characters), then with `This patient still has four unused sessions on their current plan.` **Expected Result.** The two-character reason is **refused**, by the route and by a CHECK on `care_plan_reviews` — a rejection is what the therapist acts on, so it owes them a sentence, and one with none reads the same as one nobody got round to. With a real reason the plan becomes `rejected`; a `care_plan_reviews` row and a `care_plan.reject` audit row are written; the patient never sees it. The therapist sees it as something needing them, with the reason (`THR-CARE-007`) — **they rewrite; an admin does not edit their judgement**. The closed thread frees the one-open-plan slot immediately.

ADM-CARE-006 — Approve with different numbers · P0

Feature. The middle case, and the one whose honesty is in the plumbing rather than the button.

Steps. On a queued recommendation, tap **Approve with changes**. Change the session count chip and the frequency. Submit with the reason `Frequency reduced to match what this patient can attend.`

Expected Result

- The plan becomes `active` and the patient is offered the **new** numbers.
- **The therapist's original version is untouched** and still readable in the thread, marked superseded. A version is append-only; rewriting one under a clinician's name would be a lie about who decided what, and the trigger refuses it regardless.

- The new version carries `authored_by` = the **therapist** and `entered_by` = the **admin**. Check both in the database — this is the assertion the whole feature turns on.
- The `care_plan_reviews` row records `edited_and_approved`, and a `care_plan.edit_and_approve` audit row is written.
- The programmes offered in the change panel are **narrowed to that session's own condition**, exactly as on the therapist's own dialog.

ADM-CARE-007 — The switch · P1

Steps. At **Settings → Booking Rules**, turn **Approve recommendations before the patient sees them** off. Have a therapist submit a recommendation. **Expected Result.** It publishes on save and the patient sees it immediately, exactly as before the review step existed. The therapist's panel copy changes to match. Turn it back on afterwards — the rest of the suite assumes the default. The setting **fails closed**: with the column unreadable, a submission is held rather than published. That is the opposite direction from `contact_scan_mode`, and deliberately so.

ADM-CARE-002 — Withdraw a recommendation · P0

Steps. Withdraw an **active, unpurchased** plan with the reason `Therapist on extended leave; will re-review`. Then attempt to withdraw an **accepted (purchased)** plan. **Expected Result.** The active one closes; the patient's offer disappears; a `care_plan.withdraw` audit row is written; the route required `sessions` scope, a **mandatory reason**, and a compare-and-swap on `status='active'` (a stale attempt returns `Someone else closed this recommendation. Refresh to see it.`). **The purchased plan cannot be withdrawn at all** — the patient has paid and the sessions exist, so the honest lane is a refund or a credit adjustment, each of which has its own screen. Withdrawal also covers a plan **still waiting for approval** — refusing would leave the queue holding a thread nobody intends to approve while the patient's one-plan slot stayed taken. **There is no admin path to edit a version. Approving with different numbers ('ADM-CARE-006') writes a new* one attributed to the clinician; it does not change theirs, and no route anywhere can set a price.*

ADM-CARE-003 — Write a recommendation on a therapist's behalf · P0

Feature. One authoring implementation, three doors. This one exists for when a therapist cannot reach their dashboard — on leave, off sick, gone — and a patient is still waiting to hear.

Steps

1. On **Sessions → Recommendations**, open the authoring panel.
2. Choose the session — it must be a **completed session that the named therapist ran**.
3. Read which programmes are offered.
4. Fill the same four clinical fields as `THR-CARE-001`.
5. Read the text at the submit button.
6. Enter the reason `Therapist on leave; patient waiting since Tuesday.` and submit.
7. Then change the chosen session to a different one and observe the draft.

Expected Result

- Step 3: programmes are **narrowed to that session's own condition**, exactly as on the therapist's dialog.
- Step 5: **whose name it goes out in is stated at the button**, not in a subtitle two screens up.
- Step 6: the write **publishes directly**, without passing through the review queue — the admin writing it is the approver, so their own queue would decide nothing. It succeeds with **split attribution** — `authored_by` is the clinician whose judgement it is, `entered_by` is the admin who typed it. Naming only

the therapist would be a quiet lie about who was at the keyboard; naming only the admin a louder one about whose judgement it is. A `care_plan.author_on_behalf` audit row is written. The route required `sessions` scope and a mandatory reason.

- Step 7: **the draft is dropped** when the session changes, so a package for someone else's condition cannot be carried across.
- With **no** eligible session or **no** recommendable package, the panel **still renders** and says which of the two is missing. An admin opens this screen because a patient is waiting; a panel that is simply absent reads as a feature that does not exist.
- The rules are **not weaker than the therapist's door**: the package still comes from the admin whitelist, the source must still be a completed session that therapist ran, and the text is still scanned.

ADM-NEWB-001 — New Booking · P1

Steps. Open **Sessions → New Booking**. Create a booking for `QA Patient A` with `QA Therapist A` at a chosen slot. **Expected Result.** The booking is created server-side with the same re-derivation as the patient route. **An admin has a lead-time override** (there is somebody on the phone arranging the exception) where the patient route has none. Missing fields are refused with `Missing appointmentId`, `therapistId`, or `slotDateTime` / `Choose a patient.` / `Choose a treatment category.` The booking is audited. **The date and time are the patient's own calendar**, not native date/time boxes: a month grid plus hour cells, opened on the earliest eligible slot. Ticking **Book inside the N-hour window anyway** re-opens the grid down to the current hour — the one thing this screen may do that `/book` may not — and the caption under it changes to say the lead-time rule does not apply. It still never offers a past slot.

ADM-CAL-001 — One calendar, everywhere a session time is chosen · P1

Steps. Open each of these and compare the date grid and the hour cells: `/book` Step 1; the patient's package **bulk scheduler**; the home-visit bulk scheduler; the therapist's **Suggest next session**; admin **Sessions → New Booking**; admin **Reschedule / Reassign** on a session; admin **People → Partners → Patient Referrals → Pick a time**. **Expected Result.** All seven render the **same** control — same month grid, same weekday headers, same cell colours and states (available / selected / struck-through), same hour cells. Nothing on this list is a native `date`, `time` or `datetime-local` box, and nothing opens in a pop-up on the admin screens. The bulk schedulers additionally dot any day already holding a chosen slot; that dot outranks the today marker. **Where the lead time differs, and why:** `/book`, both bulk schedulers, Suggest and Assign-a-referral use the platform's 12-hour rule (home visits use their own, longer, setting). **Reschedule** uses zero — it moves a session that already exists, which is the admin override lane — and **New Booking** uses zero only while its override box is ticked. Zero never means "the past": no screen offers a slot before now. **The hour rule is enforced server-side too.** POST any of these with a slot carrying minutes (e.g. `...T18:52:00+05:30`) and it is refused **400** with `Sessions start on the hour. Pick a time like 6:00 or 7:00.` : `/api/appointments/create`, `/api/appointments/book-package-sessions`, `/api/admin/create-booking` (including with the lead-time override ticked — the override is about the lead time, never about landing between hours), `/api/admin/update-appointment`, `/api/admin/assign-referral`, `/api/therapist/suggest-session`, `/api/home-visit/book-visits`, `/api/home-visit/book-cash`, `/api/home-visit/verify`. **Critical check — the timezone:** the minute is read in the **booking's own** timezone, not the server's. 6 PM IST is 12:30 UTC, so a correct IST booking arrives as `...T12:30:00Z`; if that is refused, the check is reading UTC and every booking in the clinic is broken. Conversely `...T13:00:00Z` (6:30 PM IST) must be refused. **Deliberately not enforced at the consuming end:** `/api/patient/respond-suggestion` and `/api/patient/register-via-referral` book a slot somebody already agreed to, so a row created before this rule still books rather than being stranded. **Deliberately unchanged:** dates that are not session slots keep their native inputs — a report's date, a leave range, a schedule exception, a promo campaign's window, and every from/to range filter (Metrics, Costs, Activity Log, All Sessions, Payment History, Earnings, the Calendar tab's day). They have no hours and no lead time, and a grid whose greyed-out cells mean "too soon to book" would be lying on all of them.

14.3 People

ADM-PEOP-001 — Patients directory · P1

Steps. Open **People → Patients**. Filter, page, and export. **Expected Result.** A paged list ending in the standard pager. **Approvals are not here** — they are under Today. Below the directory sits the **condition requests** area, whose badge counts what is waiting. **Exports:** every admin export offers **CSV and PDF** from one column definition, so the spreadsheet and the printable document cannot describe different tables. Both cover the **whole filtered set**, not the current page. Every export carries a **subtitle naming what the rows are scoped to** — a printed table nobody can date is worthless. **Nothing in the admin dashboard exports JSON.**

ADM-PEOP-002 — Global search · P2

Steps. Use the admin search to find `QA Patient A` and `QA Sunrise Hospital`. **Expected Result.** Results are grouped by entity type and link to the right detail surface.

ADM-PEOP-003 — Patient detail · P1

Steps. Tap a patient row. Then open `/admin/dashboard/patients/<id>` directly. **Expected Result.** Tapping opens an **overlay modal**; the direct URL renders the **same content as a full page**. The detail shows sessions, purchases, notes, ratings, contact edit and password reset. `ProfileSessionList` and the purchase modals take `canSeeMoney` / `canManageSessions` — **a control an admin's scope cannot call must not render**, or they get a 403 with nothing to explain it.

ADM-PEOP-004 — Condition access grants and change requests · P0

Steps. Approve a therapist's access-grant request. Then approve a patient's condition change request. Then approve a **therapist-submitted** edit for a re-triaged patient. **Expected Result.** The grant lets the therapist edit that patient's record. **Approving a change MERGES; it never replaces** — see `THR-HP-004`. Revoking a grant that is not approved is refused with `Only an approved grant can be revoked`. A stale decision returns `Someone else reviewed this. Refresh to see what they said.`

ADM-PEOP-005 — Therapists directory · P1

Expected Result. A paged list with approval state, active state, leave state, team visibility, revenue share and rating visibility.

ADM-PEOP-006 — Therapist detail and revenue share · P0

Steps. Open `QA Therapist A`. Set **Revenue share %** to `60`, and the home-visit share to `65`. Save. Then try `-5` and `150`. **Expected Result.** Valid values save and immediately change the therapist's Earnings and the Money screens' split. Invalid values are refused with `Enter a percentage between 0 and 100.` The change is audited.

ADM-PEOP-007 — Suspend and restore a therapist · P1

Steps. Toggle Therapist A inactive, then active. **Expected Result.** While inactive: their dashboard redirects to `/account-suspended`, their API routes 403, they disappear from `/team` and from `?therapist=` resolution, and they cannot be assigned. Restoring reverses all of it. **Their existing appointments are unchanged.**

ADM-PEOP-008 — Partners · P1

Covered by `HOS-AUTH-002`, `HOS-MONEY-*`. Additionally: **Copy invite link**, **Update revenue share**, **Set active/inactive**, **Reset password**, **Referral capacity note**, and **Decline referral** (reason mandatory) all work and are audited.

ADM-REF-001 — Contacting a referred patient before the link goes out · P1

Steps. Open **People** → **Partners** → **Patient Referrals** and read the card for the referral created in `HOS-REF-001`. **Expected Result.** Directly under the patient's name: their **phone number as a `tel:` link** and their **preferred language**, before the referring partner's name. That order is deliberate — an admin rings this patient to agree a time *before* the registration link is sent, so the number and the language they want to be spoken to in identify the referral rather than being detail about it. A referral taken before the phone field existed reads `No phone on file`; one with no language reads `Language not stated` — never a blank line. **Critical check:** the phone comes from its **own isolated query** (`patient_phone` is the newest column on `patient_referrals`), so a database missing that migration must lose the number on the card and nothing else — the capacity note beside it and the referral list itself must still render.

ADM-REF-002 — Assigning a referral uses the patient's own calendar · P0

Steps. On a **Needs triage** referral, pick a therapist and open **Pick a time**. **Expected Result.** An inline compact month grid plus hour chips — **the same control and the same 12-hour lead-time rule as `/book` Step 1**, not a `datetime-local` box and not a pop-up. Today (and any hour inside the next 12) is **not clickable**; the picker opens on the earliest eligible slot with a time already chosen. Changing to a date that does not offer the chosen hour falls back to that day's earliest, never to a time the rule has just ruled out. The line under it reads `Earliest bookable time is 12 hours from now – the same rule the patient's own booking screen follows`. **Negatives:** `POST /api/admin/assign-referral` with a slot inside the window is refused with `The assigned slot must be at least 12 hours from now.` — the rule is re-checked server-side, not trusted from the browser. A card left open past the boundary is refused in the browser first with `That time is no longer far enough ahead. Pick a later slot.` **Unchanged:** therapist conflict detection, the home-visit travel buffer, the concurrent-assignment tiebreak and the registration link are all exactly as before.

ADM-PEOP-009 — Reset a password · P1

Steps. Reset Patient A's password from the detail page. **Expected Result.** A new password is generated and shown **once**. **It is never written into the audit log's `details`** — the log is readable by every admin, so who reset what and when is the part with audit value. The patient can sign in with the new password and is prompted to change it.

14.4 Catalog

ADM-CAT-001 — Create a treatment category · P0

Steps

1. Open **Catalog** → **Conditions**.
2. Tap the create control.
3. Tap **Category Name**. Enter `QA Back & Spine Care`.
4. Tap **Price (₹)**. Enter `1999`.
5. Tap **Session Length (min)**. Enter `60`.
6. Tap **Order**. Enter `1`.

7. Tap **Button Text**. Enter `Book Assessment`.
8. Optionally paste a **Cover Image URL**.
9. Save.

Expected Result. The category is created and appears **immediately** on `/` and `/conditions` — the create route invalidates both ISR-cached pages, so a five-minute wait is now a defect, not expected behaviour — in the `/book` concern dropdown as `QA Back & Spine Care – ₹1,999 / 60 min`, and as an option when creating a package. The cover image is a **plain URL an admin pastes**, not a Storage upload, rendered through a plain `<img>`; a row with no image shows the shared **placeholder panel at the same height**, never a broken-image state. **Negatives:** `Missing title, priceInr, or durationMinutes`; `Price` must be a positive number; `Session length` must be a positive number of minutes; `Order` must be a number.

ADM-CAT-002 — Edit, reorder, deactivate, delete a category · P1

Expected Result. Editing the price changes what `/book` charges **for new bookings** and is re-derived server-side at booking time. Reordering changes the display order everywhere.

Reorder, in detail. The up/down arrows rearrange the list **in the browser only** — nothing is written until **Save order** is tapped. That button is **always visible** and is **disabled until something has actually moved**; while there are unsaved moves the screen reads `Not saved yet – the public pages still show the old order.` beside an **Undo changes** link. Saving posts the whole list, renumbers it `1..n`, and shows `Order saved – live on the site.` **Critical check:** reload the tab after saving and confirm the new order **survives**, then open `/` and `/conditions` and confirm they show the same order **immediately** (the pages are ISR-cached, and the save invalidates them). A reorder that reverts on reload, or one the public pages ignore, is the P1 defect this case exists for — it was caused by a pairwise `display_order` swap that did nothing whenever two categories held the same number, which every category created without typing an **Order** did. **Negatives:** with two browser tabs open, add or delete a category in tab B, then Save order in tab A — refused with `The condition list changed while you were reordering it. Refresh and try again.` **Also:** creating a category now defaults **Order** to one past the last existing category, so a new condition appends rather than appearing first. Deactivating removes it from public surfaces and refuses new bookings against it (`That concern isn't available any more. Please pick another one.`) while **leaving existing appointments untouched**. Deleting a category referenced by a live purchase must not silently break that purchase.

ADM-CAT-005 — Create a session package · P0

Steps. Create Package P1 exactly as specified in §8.11. **Expected Result.** Saved. The **Category is set once at creation and locked afterwards** — live purchases reference it. The package becomes available to therapists as a recommendable programme for that condition **only**. On the public site it appears as a card with **no Buy button**, showing instead *"Arranged by your therapist after your first session."* **Negatives:** the five validation messages in §8.11.

ADM-CAT-006 — Editing a package never changes what was already sold · P0

Steps. After Patient A has purchased P1 (6 sessions, ₹9,999), change P1 to 4 sessions and ₹12,000. Then open the patient's package widget and the admin's Purchases row. **Expected Result.** The purchase still reads **6 sessions** at the price paid. `sessions_granted` and `package_snapshot` are **frozen by trigger**. **Never resolve a purchased entitlement by joining the live catalog row** — if the patient's widget now says 4 sessions, that is a P0 defect. **[SQL] confirmation:** attempting `update patient_package_purchases set sessions_granted = 4 where id='<id>'` **raises**.

ADM-CAT-007 — Package rules reach the booking · P1

Steps. With P1's minimum gap 24h and max 3 /week, have the patient attempt to book two sessions 12 hours apart, and four in one week. **Expected Result.** Both refused by the batch rules layer, with readable messages. Changing the rule in the catalog changes the refusal for **new** scheduling attempts.

ADM-CAT-010 — Service areas · P0

Steps. Create Area 1 and Area 2 from §8.13. Then try to create a third area containing 560038. **Expected Result.** Both save. The third is refused with Another service area already covers that pincode. (or Every pincode in that list is already a service area.). Other negatives: Enter at least one pincode. , Enter a valid 6-digit pincode. , City is required. , Travel Fee must be zero or a positive number. **Dependency:** creating 560038 makes /book-home-visit accept it; deleting it makes the same pincode fall through to the waitlist. **Re-checked server-side at every purchase route** — never trust a serviceability answer the browser already has.

ADM-CAT-011 — The home-visit waitlist · P2

Expected Result. Entries from PAT-HV-003 appear with status new , raising the Service Areas badge. Updating a status clears the badge. Unknown status is refused for an invalid value.

ADM-CAT-014 — Purchases · P1

Steps. Open **Catalog → Purchases**. Open a package purchase's detail modal; then a home-visit purchase's. **Expected Result.** Both list every purchase with balances. The detail modals are **viewer-scoped, not role-branched** — the route queries with the caller's own RLS-scoped client, so a row coming back **is** the authorization. Money controls inside the modal render only for an admin with money scope. Admin actions available here: **extend expiry** (invalid dates refused with Invalid newExpiresAt), **reassign the locked therapist** (touches **future sessions only** — completed ones keep whoever actually ran them), **refund**, **restore a session**, and **grant / reverse / revive credits**.

ADM-CAT-015 — Credit adjustments require a reason · P0

Steps. Grant 2 extra sessions with the reason Goodwill (7 characters), then with Goodwill after a cancelled session. **Expected Result.** The short reason is refused — admin_adjust is the **only** entry type with free-form deltas and the **only** one requiring a reason, **ten characters minimum, enforced by a CHECK** so it holds for any caller. The valid one succeeds and appends a ledger row. **An admin can change any balance and cannot change any history.** [SQL]: update session_credit_ledger ... and delete from session_credit_ledger ... both **raise** — append-only by trigger, not by RLS, because every route writes with the service role.

15. Admin test plan — Part B (Settings, and configuration → dependent feature)

How to test a setting. Every settings case follows the same four beats: **change it → reload the admin screen (it must persist) → open the dependent feature and prove it changed → change it back.** A setting whose dependent feature you did not check has not been tested.

Every setting below is read through one shared settings module with defaults. **Every dashboard page selects the same column list**, so a new setting cannot silently read as its default on whichever page forgot it.

15.1 Settings → Brand & Contact

ADM-SET-001 — Brand & Contact Details · P1

What it controls. `site_name`, `site_tagline`, `site_description`, `contact_email`, `whatsapp_number`, `contact_phone`, `footer_copyright_text`. **What depends on it.** The **root layout** is the one place these are read (through a public/anon client, so ISR-cached pages under it are not forced dynamic) and it passes them into `Navbar` and `Footer` **as props**. Those two components must never fetch their own copy or hardcode a string.

Steps

1. Open **Settings → Brand & Contact**.
2. Change **Site name** to `QA Physio Clinic`. Save.
3. Reload the admin page. Confirm the value persisted.
4. Open `/` in a new tab.
5. Change **Footer copyright text** to `QA Physio Clinic. All rights reserved.` Save. Reload `/`.
6. Change **Contact email** to `not-an-email` and save.
7. Restore every original value.

Expected Result. Step 4: the navbar and the page title/description show `QA Physio Clinic`. Step 5: the footer shows the new line. Step 6: refused with a validation message (`Enter a valid email address.`). Step 3 and every reload: values persist. Existing records are unaffected — this is presentation only.

Interaction worth checking: the **splash** brand line is **blank by default and falls back to the site name**, so changing the site name also changes the splash greeting until an admin deliberately parts them.

15.2 Settings → Public Site

ADM-SET-004 — Ratings visibility · P2

Steps. Toggle **ratings visible publicly** off. Open `/team` and `/`. **Expected Result.** The public rating summary disappears from public surfaces. Individual therapist rating visibility is a **separate** per-therapist toggle on their detail page; verify both independently.

ADM-SET-005 — Home page walkthrough pace · P1

Steps. Set the walkthrough seconds to `2`. Open `/` and watch the "How the process works" widget. Then set it to `0`. **Expected Result.** At `2`, each step holds ~2 seconds before the next takes over. At `0` it does not advance on its own` — the same "0 is off" convention used by the session timeout. Values over 300 are refused (`Keep this to 300 seconds or less.`). **Accessibility interaction:** a visitor with **reduced motion** requested must not be subjected to auto-rotation. **Do not confuse this widget with the care-area showcase**, which **never advances by itself** — a second moving thing while you read the first is worse than either alone. Their `aria-label`s must stay distinct: "How the process works" vs "Areas of practice".

ADM-SET-006 — The opening splash, all five settings · P1

Feature. A teal sheet painted over the site for a beat. It greets a **cold open** — the first load of a browser tab — and a **return to a tab that has been in the background longer than the away threshold**. It deliberately does **not** show on every navigation, every reload or every tab focus, because a patient paying by UPI leaves the tab for their bank's app and comes back mid-checkout, and **splashing over a payment in progress is the one thing this must never do**.

Setting	Test
On/off	Off → no sheet on a cold open in a fresh tab
Name above the line	Blank → falls back to the site name. Set it → the splash and the navbar deliberately differ. Blank is a value here, not an error — it is how the override is undone.
The one line	Changing it changes the greeting text
Hold (seconds)	A longer hold visibly holds longer
Away minutes	Set <code>1</code> . Open a tab, switch away for 90 seconds, switch back → greeted again. Set <code>0</code> → first load only , and returning is never greeted. There is deliberately no value meaning "greet on every tab focus" .

Expected Result also to check: a **reload** of an already-greeted tab is **not** greeted. Someone who has asked for **reduced motion is skipped outright**. The overlay's markup is present in every page's HTML and visibility is driven by a `data-splash` attribute on `<html>` — not React state — so there must be **no hydration warning** in the console.

ADM-SET-007 — Testimonials · P1

Steps. Create a testimonial with patient name `QA Story` and quote `The exercises made a real difference in six weeks.` Save. Open `/` and `/mission`. **Expected Result.** It appears **immediately** in the same band on **both** pages (the route invalidates `/` and `/mission`, both ISR-cached) — one component serves both, because the two bands make the same claim and a visitor may see both in one session. The avatar is optional; with none, the **patient's initial** is shown, never a generic silhouette.

Critical check: the five rows the schema seeds are **illustrative copy, not real patients**, and the admin form must **say so at the point of entry**. Never present a seeded testimonial as real. The only place a **real** number is quoted is the public rating summary. **Negatives:** `Missing patientName or quote`; editing requires `Missing id, patientName, or quote`.

ADM-SET-008 — FAQ · P2

Steps. Create, edit, reorder and delete an FAQ. Open `/faq`. **Expected Result.** The public accordion reflects each change **immediately** — `/faq` is ISR-cached and every FAQ route invalidates it. **Negatives:** `Missing question or answer`, `Missing id, question, or answer`.

15.3 Settings → Booking Rules

This tab holds three groups: **Platform Rules**, **Package settings**, and **Home Visit settings**. They were on three different tabs before, which is how the online lead time ended up hardcoded while its home-visit twin was already a setting.

ADM-SET-010 — Online Booking Lead Time → the booking wizard · P0

Configuration. `online_booking_lead_time_hours`, default **12**. **Dependent features.** The `/book` Step 1 calendar and hour list **and** `/api/appointments/create`'s own validator — deliberately the same setting, so the picker can never offer a slot the server rejects.

Steps

1. Note the current earliest bookable slot on `/book` under simulated time `2026-09-10T10:00`.

2. In **Settings → Booking Rules → Online Booking Lead Time**, change `12` to `48`. Save.
3. Reload the admin page and confirm it persisted.
4. Reload `/book` under the same simulated time.
5. Attempt a booking at the old boundary (10 September 22:00) by any means, including a direct API call.
6. Change it back to `12`.

Expected Result. Step 4: the earliest offered slot is now **12 September at 10:00**, and 10–11 September are greyed. Step 5: the API refuses with **409** and `Please pick a slot at least 48 hours from now.` Step 6: the picker returns to the 12-hour boundary. **Existing bookings are untouched** — this rule applies to new bookings only. **Negatives:** `value must be a non-negative whole number`.

ADM-SET-011 — Online Cancellation Refund Window → the cancel dialog and the refund · P0

Configuration. `online_cancellation_refund_hours`, default **24**. **Steps.** Change it to `72`. Reload `/book`. Step 3 and read the cancellation notice. Then cancel a paid session whose slot is 48 hours away. **Expected Result.** Step 3's notice now reads *"Free cancellation up to 72 hours before your slot..."*. The 48-hour-away cancellation now falls **inside** the window: the dialog says it will not be refunded, and **no refund is processed**. Restore `24`. **Independence check:** this must **not** change the **home-visit** refund dialog, which reads its own setting.

ADM-SET-012 — Booking Languages → the Step 1 chips · P1

Steps. Add `Hindi` and `Kannada`. Save. Reload `/book`. Then remove every language and save. **Expected Result.** Three chips appear in Step 1 in the configured order; the first is auto-selected. A language not on the list is **dropped server-side** rather than stored as a preference nobody is matched on. Removing all is refused with `Keep at least one language – booking needs something to offer.` — booking must never present an empty language picker. Duplicates that differ only by case are de-duplicated.

ADM-SET-013 — Home Visit master switch → six surfaces at once · P0

Configuration. `home_visit_enabled`, default **off**. **Dependent features.** `/home-visit` (404s when off), the header nav, the footer Explore column, the home page connector grid, every "Where to go next" strip, the hospital's **Session Type** option, `/book-home-visit`, and `/api/care-plan/create-order`'s re-check.

Steps. With it **off**, check all seven surfaces. Switch it **on**, check all seven again. Then, with a **live unpurchased home-visit recommendation** outstanding, switch it off and have the patient try to buy. **Expected Result.** Off: `/home-visit` is a 404 and the entry is **dropped from every list rather than linking into a dead end**; the hospital cannot choose a home-visit referral. On: everything appears. The care-plan purchase with the switch off is refused with `Home visits aren't available right now. Please talk to your therapist.` — **an admin who switches home visits off has stopped the service, and a recommendation written before that must not stay purchasable.**

ADM-SET-014 — The remaining home-visit settings · P1

Setting	Default	Dependent feature to verify
Allow cash on visit	on	The cash option on <code>/book-home-visit</code> Step 4; <code>/api/home-visit/book-cash</code> refuses when off
Booking lead time	24 h	The wizard's Step 2 copy and its picker; must stay independent of the online 12 h
Travel buffer	45 min	A locked therapist's conflict check is padded by this on both sides of a new visit; online passes 0
Full-refund window	24 h	The home-visit cancel dialog and the refund actually paid

Setting	Default	Dependent feature to verify
Default package validity	90 d	A new home-visit purchase's expiry when the package leaves it blank
Bulk scheduling limit	8	Too many slots in one request. above this
Page heading / subheading	(defaults)	The /home-visit page's own copy

ADM-SET-015 — Session Timeout of Inactivity · P2

Steps. Set it to 1 minute. Sign in as a patient and leave the tab idle. **Expected Result.** The idle dialog appears and signs the patient out to **their own login page** (/patient/login, /therapist/login, /hospital/login — the door they came in through). **0` means off. Admins are exempt from the inactivity timeout entirely.**

ADM-SET-016 — Sign-out message duration · P3

Steps. Set farewell_banner_seconds to 2, then 0. Sign out each time. **Expected Result.** At 2, the banner clears after two seconds. At 0 it stays until dismissed.

ADM-SET-017 — Google Meet toggle, join window, and the Session Completed cutoff · P0

Setting	Default	Verify
Auto-Create Meet Links	on	Off → a newly confirmed online session gets no Meet link. A home visit still gets a calendar event — this toggle gates the Meet conferencing only, not event creation.
Join Button Window (before)	15 min	Set 5; the join control goes live 5 minutes before the slot on every surface
Join Button Window (after)	15 min	The short grace period for a late arrival
Session Completed Cutoff	60 min	Set 30; 31 minutes after the slot every join control on every surface — patient, therapist and admin — reads Session Completed

Note: the cutoff is **not** the same thing as the after-window. See XR-CUTOFF-001 for the cross-role check.

ADM-SET-018 — Package settings · P1

Setting	Default	Dependent feature
Assign a Therapist Automatically	off	See ADM-SET-021
Therapist Lock (site-wide)	on	Off → later sessions on a purchase are not auto-assigned to the first therapist

Setting	Default	Dependent feature
Session Balances From The Ledger	off	See <code>ADM-SET-019</code>
Therapist - Suggested Sessions	on for a fresh database	Off → <code>/api/therapist/suggest-session</code> returns <code>Suggesting sessions is switched off.</code> and the control is absent. The column default is now true, but that only applies to a new <code>site_settings</code> row — an existing database keeps its current value until an admin toggles it, or a reset restores defaults. Check the toggle before running <code>THR-SUGG-*</code> rather than assuming
Default Validity	90 d	A new purchase's expiry when the package leaves it blank
Bulk Scheduler Limit	8	<code>Too many slots in one request.</code> above this
Expiry Reminder Lead Time	14 d	When the expiry nudge appears on the patient's dashboard

`ADM-SET-022` — Recommendation settings · P0

At **Settings** → **Booking Rules**, above the package settings.

Setting	Default	Dependent feature
Approve recommendations before the patient sees them	on	Off → a therapist's submission publishes on save and the patient sees it immediately, as it did before the review step. On → it lands in Sessions → Recommendations and the patient sees nothing. See <code>ADM-CARE-004.008</code> . Fails closed: an unreadable column holds the recommendation rather than publishing it. Read in its own query, not through <code>SITE_SETTINGS_SELECT</code> — the same treatment Therapist-Suggested Sessions gets, so a database one apply behind loses this one control instead of resetting every setting on the page
How long a recommendation holds	30 d	The patient's answering window, counted from approval , not from when the therapist wrote it
Most sessions a week a clinician may ask for	5	A ceiling over the programme's own <code>max_sessions_per_week</code> , whichever is lower. Above it: <code>This programme allows at most N sessions a week.</code>

Note there is no longer a **Show programme prices publicly** switch. Programmes are not advertised on the public site at all — see `PUB-CAT-002`.

`ADM-SET-023` — First session offer · P0

At **Settings** → **Booking Rules**, above the recommendation settings.

Setting	Default	Dependent feature
First session offer	off	On → a patient who has never paid for a session is charged the offer price for a video consultation. Off → everyone pays list price
Offer type	A set price	A set price names what they pay ("₹499"); A percentage off adapts across categories priced differently
Offer value	0	Rupees or whole percent depending on the type. The panel previews what a real category's session would cost a new patient

Steps. Turn it on, set a set price of ₹499. Read the preview. Book and pay as a **brand-new** patient, then as one who has paid before. **Expected Result**

- The preview quotes a real category's price — "A ₹1,200 session would cost a new patient ₹499."
- The new patient is charged **₹499**; the returning patient is charged **₹1,200**. Eligibility is decided server-side from payment history, so it cannot be requested, repeated, or sent from the browser.
- Programmes and home visits are **not** discounted by it.
- Values the route must refuse: a type other than `fixed / percent`, a value of 0 or negative, a non-boolean for the switch.
- Set 100% off: the patient is charged the ₹1 minimum rather than zero — Razorpay refuses a zero-amount order, so an unguarded 100% would be a 500 at the last step of checkout.

ADM-SET-021 — Automatic therapist assignment · P0

Feature. When a session is paid for and **exactly one** therapist is unambiguously free for it, assign them and confirm the booking immediately instead of leaving it in the admin queue. It reads the roster (weekly template + that date's exceptions + leave) and the same conflict check the admin's assign form uses. **It does not change what times a patient is offered** — the roster still does not filter the picker.

#	Set up	Expected
1	Switch off . Book and pay a session.	Session is <code>requested</code> , unassigned , in the queue — the pre-existing behaviour.
2	Switch on . Roster only Therapist A for the slot's hour. Book and pay.	Assigned to Therapist A, <code>confirmed</code> , Meet link created, and the therapist sees it immediately. The unassigned badge does not rise.
3	Roster both Therapist A and B for that hour, neither busy. Book and pay.	Nothing is assigned. The session waits in the queue. Two free clinicians is a choice for a person.
4	Both rostered, but Therapist B already has a clashing session.	Assigned to A — one free candidate.
5	Both rostered and free, and the patient booked via <code>/book?therapist=</code> .	Assigned to B , not A. A stated preference beats the count.
6	Patient requested B, but B is busy; A is free.	Assigned to A . The preference is dropped rather than the session waiting.
7	Patient requested a therapist who is not rostered for that hour.	Falls back to the count. A stale <code>?therapist=</code> link never overrides the roster.
8	Nobody rostered for that hour.	Nothing assigned; queue as before.
9	Therapist A rostered but on leave .	Not a candidate.
10	Therapist A rostered but unapproved or inactive .	Not a candidate.

#	Set up	Expected
1 1	A home visit , with two therapists free but one finishing a visit within the travel buffer.	The buffered one is treated as busy — the conflict check is padded by <code>home_visit_travel_buffer_minutes</code> on both sides.
1 2	Pay, then close the tab before the callback lands (webhook configured).	The webhook applies the same assignment. Both paths use one decision, so they cannot disagree about who is free.

Expected Result throughout. No session is ever assigned to a therapist who is unavailable, on leave, unapproved, inactive or already booked. When it declines to choose, the outcome is **identical to the switch being off**. A failure inside this logic must never fail the payment — the appointment is still marked paid either way.

Cross-check `XCFG-ROSTER-001` afterwards: rostering changes must still leave `/book`'s picker byte-identical.

ADM-SET-019 — The ledger authority switch · P0

Feature. Whether a balance shown and offered is read from the **credit ledger** or from the older `sessions_used` / `visits_used` counters is **one admin switch**, off by default and **reversible in a second** — both are written either way.

Steps

1. Open **Settings → System Health** and confirm the accounting check reports **no disagreements**.
2. Turn **Session Balances From The Ledger on**.
3. Check every surface that shows a balance: the patient's package widget, the therapist's programme list, both purchase detail modals, the admin Purchases table, and the bulk scheduler.
4. Turn it back off and check them all again.

Expected Result. Every surface reads the same shape and follows the switch **together** — the substitution happens once, where the row is loaded. A refunded package still reads its original `6 sessions` with none pending rather than becoming a 1-session package. A purchase with **no** entitlement behind it is untouched, so a database without the backfill behaves exactly as before. **Important:** the switch **does not change how a session is claimed**. The counter's compare-and-swap still wins the booking race, with the ledger's row lock beside it. Flipping the switch must not change any booking outcome. The screen warns you to turn it on only once System Health has been clean.

ADM-SET-020 — Clinical Questions · P0

Feature. The Health Profile question sets and the Pain Map templates. **Editing these changes what is asked from here on; answers already submitted are untouched.**

Steps

1. Open **Settings → Clinical Questions**.
2. Confirm the question bank is presented as **one tab per specialty**, not three stacked sections.
3. On the **Orthopaedic** tab, change the wording of `severity` and save.
4. Open a patient's intake wizard.
5. Open an existing patient's already-answered profile.
6. On **Enabled condition types**, switch **Paediatric** off.
7. As a therapist, open triage for a new patient, and then re-triage an existing paediatric patient.
8. Attempt to switch **Orthopaedic** off.

Expected Result

- Step 2: tabs, because twenty-odd textareas stacked is the wall-of-fields shape this product keeps correcting.
- Step 4: the new wording is shown. A new question requires ``helpText`` (why this answer matters, in the patient's words) and a ``shortLabel`` alongside its label — a question added without them is a defect.
- Step 5: **the already-submitted answer is unchanged.**
- Step 7: Paediatric is **removed from the triage picker** — and **an existing profile carrying it still renders**, and a therapist **re-triaging such a patient is still offered it**. If a live paediatric chart blanks, that is a P0 defect.
- Step 8: **Orthopaedic can never be switched off.**
- `schema_version` is **per specialty**, so changing a neuro question must **not** fire the "we've changed some of these questions" banner at orthopaedic patients.
- Pain Map templates edit per region and question; unknown values are refused with `Unknown region / Unknown questionKey for this region`.

ADM-SET-025 — Team & Access: scopes · P0

Steps

1. Open **Settings → Team & Access**.
2. Read the admin list.
3. Attempt to change **your own** scope.
4. Narrow every other `full` admin, then attempt to narrow the last one.

Expected Result. Step 3: refused with `You can't change your own access. Ask another Master Admin.`
 Step 4: the last `full` admin (**Master Admin** in the picker) **cannot be narrowed** — otherwise a single mis-click locks everyone out permanently. Only a `full` admin can change scopes or mint another admin.

ADM-SET-026 — Create the three scoped admins · P0

Steps. Create `qa.admin.ops@example.test` (Operations), `qa.admin.finance@example.test` (Finance), `qa.admin.clinical@example.test` (Clinical). **The Account type picker is one control, not two.** It lists six entries in two groups — **Clinic:** Patient, Therapist · **Back office:** Master Admin, Operations, Finance, Clinical — using the same four names the dashboards call themselves. There is no separate **Access level** dropdown; picking a back-office desk shows that desk's one-line description under the picker. As a **non-`full`** admin, the whole Back office group is **absent** (only a Master Admin may mint an admin, and `create-account` enforces that with a full-only check, not a section gate — see §2). **Expected Result.** Each is created with a one-time password shown once and **never logged**. Signing in as each shows only their allowed sections in the sidebar — Operations: Today, Sessions, People, Catalog. Finance: Today, People, Money. Clinical: Today, Sessions, People.

ADM-SET-026a — Each scope opens on its own dashboard · P1

There is one admin login (`/admin/login`) and one dashboard route; the scope decides what it opens on. Sign in as each of the four in turn and read the Today screen without tapping anything.

Admin	Sidebar brand and header read	Today leads with	Quick actions
Master Admin	Master Admin	Sessions today	Approvals · All sessions · Money summary
Operations	Operations	Unassigned sessions	Assign a session · Approvals · Roster · New booking

Admin	Sidebar brand and header read	Today leads with	Quick actions
Finance	Finance	Owed to therapists	Money summary · Payouts · Transactions · Costs
Clinical	Clinical	Recommendations	Recommendations · Patients · All sessions · Roster

Expected Result. The dashboard names itself in two places on every screen — the sidebar brand (above **Admin Panel**) and the eyebrow above the section heading — so an admin never has to work out which dashboard they are on from which sidebar entries are missing.

Three further checks, each one a bug this replaced:

- **Every quick action lands somewhere.** Tap all of them for each scope. None may bounce to a different screen — a link into a section the scope cannot open silently falls back to the first allowed one, which looks like it worked.
- **"Needs you" agrees with the list below it.** The figure counts only the queues in the **Queues** card beside it. Add up the badges; they must match. It must not count queues this scope cannot open.
- **A limited scope gets a "Your access" card** under Queues, naming the sections it opens and the ones it does not. **Master Admin gets no such card** — nothing is withheld, so there is nothing to account for.

ADM-SET-027 — Scope is enforced at the route, not the sidebar · P0

For each scoped admin, do **both**: navigate to a forbidden section by URL, **and** call a route in that section directly.

Admin	Forbidden URL	Forbidden route	Expected
Operations	? section=money&tab=payouts	POST /api/admin/settle-therapist-payout	Page falls back to an allowed screen; route 403
Operations	? section=settings&tab=booking	POST /api/admin/update-setting	Same
Finance	? section=sessions&tab=all	POST /api/admin/assign-appointment	Same
Finance	? section=catalog&tab=packages	POST /api/admin/create-package	Same
Clinical	? section=money&tab=summary	POST /api/admin/refund-package	Same
Clinical	? section=settings&tab=team	POST /api/admin/set-admin-scope	Same
All three	? section=today&tab=risk	—	The Risk tab is not rendered and its data is not fetched

Expected Result. Every one returns 403 at the route. The sidebar hiding a section is presentation only — a session cookie can call any route directly. **Do not report the three full-only routes as violations.** `set-admin-scope`, `debug-reset` and `create-account` guard with an explicit `scope !== "full"` check rather than `requireAdminScope`, deliberately: a section gate would let a scoped admin widen its own access or mint a full admin. They are stricter than the rule, not exceptions to it.

ADM-SET-028 — The section is chosen by the capability, not the button's location · P1

Purpose. A refund is `money` scope **even though its button lives on a Catalog screen.** **Steps.** As **Admin Ops** (who *can* open Catalog), open **Catalog → Purchases** and look for the refund control. Then call `POST /api/admin/refund-package` directly. **Expected Result. The refund control does not render** — a control an admin's scope cannot call must not be shown, or they get a 403 with nothing to explain it. The route returns **403**.

ADM-SET-029 — Contact controls · P1

Steps. On **Settings → Team & Access**, change `contact_scan_mode` through `flag_and_block` → `flag_only` → `off`, and toggle `contact_masking_enabled`. **Expected Result.** As per `THR-LEAK-006` and `THR-SESS-003`. Note the deliberate asymmetry: `contact_scan_mode` fails open,` `contact_masking_enabled` fails closed` — the safe answer to "I don't know" is opposite for the two, on purpose. This tab also surfaces the `communication_flags` and `contact_reveal_log` evidence, **read-only**.

ADM-SET-030 — System Health · P0

Steps. Open **Settings → System Health**. **Expected Result.** Two panels: **Sync Health** (failed Meet syncs, with Retry) and the **accounting check**. The accounting check reports where the entitlement **cache**, the **ledger** and the **legacy counter** disagree. **It reports and never repairs** — a silent auto-fix on a money record is how a discrepancy becomes permanent. The badge on this tab is the sum of sync issues and accounting problems.

ADM-SET-031 — Sync Health retry · P1

Covered by `ADM-SESS-003`.

ADM-SET-033 — Activity Log · P0

Steps

1. Open **Settings → Activity Log**.
2. Confirm each of these earlier actions appears with actor, action, target and timestamp: account approval, therapist revenue-share change, care-plan withdrawal, care-plan authored on behalf, payout settlement, cash amount correction, credit adjustment, hospital onboarding, password reset.
3. Search the log for any of the generated passwords from `ADM-SET-026` or `ADM-PEOP-009`.
4. **[SQL]** Attempt `insert into admin_activity_log ...` as an authenticated (non-service-role) session, and attempt `update / delete`.

Expected Result. Step 2: **every one is present.** `payout.settle` is the largest money move in the application and must be attributed — if it is missing, that is a P0 defect. Step 3: **no password appears anywhere in the log.** Step 4: the insert is refused (there is a select policy and deliberately **no insert policy**), and the log is append-only from any session. **Ordering guarantee:** each log row is written **after** the route's compare-and-swap, so the log can never record a settlement or cancellation that lost its race.

ADM-SET-035 — Account Security · P2

Steps. Open **Settings → Account Security** and change the admin's own password. **Expected Result.** The change succeeds and the new password works. Admins are exempt from the idle timeout, so no

timeout dialog appears while working here.

15.4 Configuration → dependent feature: the full matrix

Every row here is a required test. The **Verify** column is what proves the change actually landed.

#	Configuration	Where it is changed	Dependent feature	Verify	Test
1	Online booking lead time	Settings → Booking Rules	<code>/book</code> picker and the create-appointment validator	Earliest offered slot moves; a boundary API call returns 409	ADM-SET-010
2	Online cancellation refund window	Settings → Booking Rules	<code>/book</code> Step 3 notice; the cancel dialog; the refund paid	Notice text changes; a 48h-away cancel stops refunding at 72h	ADM-SET-011
3	Booking languages	Settings → Booking Rules	Step 1 chips; the stored <code>preferred_language</code>	New chips appear; an off-list language is dropped	ADM-SET-012
4	Treatment category price	Catalog → Conditions	<code>/book</code> header, Step 3 fee, the Razorpay amount	All three show the new price; an old cached page still charges the new one	ADM-CAT-002
5	Treatment category duration	Catalog → Conditions	The appointment's <code>duration_minutes</code> ; overlap checks	A new booking stores the new duration	ADM-CAT-002
6	Category active/inactive	Catalog → Conditions	The concern dropdown; the create route	Absent from the dropdown; API returns 409	PAT-BOOK-013
7	Package definition	Catalog → Packages	The therapist's recommendation dropdown	The new package is offered for its condition only	THR-CARE-001
8	Package price	Catalog → Packages	The patient's offer card and what is charged	Card and charge both move — for new plans only	PAT-CARE-002
9	Package edited after purchase	Catalog → Packages	An existing purchase	Nothing changes — snapshot frozen	ADM-CAT-006
10	Package min gap / max per week	Catalog → Packages	The bulk scheduler	Violating slots are refused	ADM-CAT-007
11	Package default validity	Settings → Booking Rules	A new purchase's expiry	Expiry date matches	ADM-SET-018
12	Bulk scheduler limit	Settings → Booking Rules	<code>/api/appointments/book-package-sessions</code>	Too many slots in one request.	ADM-SET-018
13	Therapist lock switch	Settings → Booking Rules	Auto-assignment of later package sessions	Off → later sessions are not auto-assigned	ADM-SET-018
14	Therapist suggestions switch	Settings → Booking Rules	The suggest control and its route	Off → control absent, route 403	THR-SUGG-001

#	Configuration	Where it is changed	Dependent feature	Verify	Test
1 5	Ledger authority	Settings → Booking Rules	Six balance surfaces	All six follow together	ADM-SET-019
1 6	Service area created/deleted	Catalog → Service Areas	/book-home-visit check; every purchase route	Serviceable ↔ waitlist	ADM-CAT-010
1 7	Travel fee per area	Catalog → Service Areas	The quoted total and the therapist's payout	Total = programme + fee × visits	PAT-CARE-003
1 8	Home visit master switch	Settings → Booking Rules	Seven surfaces + care-plan purchase	404 / entries dropped / purchase refused	ADM-SET-013
1 9	Cash on visit	Settings → Booking Rules	Step 4 option; book-cash	Option absent; route refuses	PAT-HV-007
2 0	Home visit lead time	Settings → Booking Rules	The home-visit picker only	Online picker unchanged	ADM-SET-014
2 1	Travel buffer minutes	Settings → Booking Rules	The locked therapist's conflict check	Padded both sides for visits, 0 for online	ADM-SET-014
2 2	Home visit refund window	Settings → Booking Rules	The home-visit cancel dialog only	Online dialog unchanged	PAT-CANCEL-003
2 3	Join window before/after	Settings → Booking Rules	Every join control	Goes live earlier/later everywhere	ADM-SET-017
2 4	Session Completed cutoff	Settings → Booking Rules	Every join control on every role	All three read Session Completed together	XR-CUTOFF-001
2 5	Google Meet toggle	Settings → Booking Rules	New online sessions' Meet link	No link; home visit still gets an event	ADM-SET-017
2 6	Session timeout	Settings → Booking Rules	The idle dialog on all three non-admin dashboards	Signs out to the right login; admins exempt	ADM-SET-015
2 7	Farewell banner seconds	Settings → Booking Rules	The post-logout banner	Duration changes; 0 = until dismissed	ADM-SET-016
2 8	Clinical question wording	Settings → Clinical Questions	The intake wizard	New wording; old answers untouched	ADM-SET-020
2 9	Enabled condition types	Settings → Clinical Questions	The triage picker only	Removed from triage; existing charts render	ADM-SET-020
3 0	Pain Map templates	Settings → Clinical Questions	The exam dialog	New questions per region	ADM-SET-020
3 1	Admin scope	Settings → Team & Access	Every admin route and the sidebar	Route 403 + control hidden	ADM-SET-027

#	Configuration	Where it is changed	Dependent feature	Verify	Test
3 2	Contact scan mode	Settings → Team & Access	Every cross-role free-text write	block → flag → none	THR-LEAK-006
3 3	Contact masking	Settings → Team & Access	Therapist session cards	Masked ↔ plain; fails closed	THR-SESS-003
3 4	Risk signals on/off + thresholds	Today → Risk	The detector sweep	Sweep stops; thresholds change what fires	ADM-RISK-003
3 5	Brand & contact details	Settings → Brand & Contact	Navbar, Footer, page metadata, splash fallback	All update	ADM-SET-001
3 6	Walkthrough seconds	Settings → Public Site	The home page walkthrough	Pace changes; 0 = static	ADM-SET-005
3 7	Splash (5 settings)	Settings → Public Site	The opening splash	Each behaves as documented	ADM-SET-006
3 8	Testimonials	Settings → Public Site	/ and /mission bands	Both update from one component	ADM-SET-007
3 9	FAQ	Settings → Public Site	/faq	Accordion updates	ADM-SET-008
4 0	Public ratings visibility	Settings → Public Site	/team, /	Summary hidden	ADM-SET-004
4 1	Therapist revenue share	People → Therapists	Earnings, Payouts, Money split	All three move together	ADM-PEOP-006
4 2	Therapist home-visit share	People → Therapists	Home-visit payout maths	Falls back to online share when unset	THR-EARN-002
4 3	Hospital revenue share	People → Partners	Partner Earnings, Money breakdown	Both move; unset ⇒ excluded, not guessed	HOS-MONEY-002
4 4	Therapist team visibility	People → Therapists	/team, ?therapist= resolution	Hidden ⇒ link resolves to nothing, silently	PAT-BOOK-008
4 5	Payment gateway fee %	Settings (Costs context)	Operating profit on Money → Costs	The automatic fee line moves	FIN-COST-002
4 6	Therapist roster (any change)	Sessions → Roster	`/book` picker	Nothing changes — this is the guard	XCFCG-ROSTER-001

16. Finance test plan (Money) and payment integrity

16.0 Feature guide — the money model

One word, one money figure. If a new figure needs a word that is already taken, the figure gets renamed — the word is never overloaded. If two figures end up with the same meaning, one is deleted rather than explained. `MoneyGlossary` renders on **every** Money screen, not only Summary, because an admin reading "Net payable" on Payouts is the one who needs it.

Word	Means
Gross Revenue	Every paid session in range, before refunds
Refunds	Refunds that actually processed
Net Revenue	<code>Gross - Refunds</code>
Splittable Net	The part of Net whose split is knowable . Always $\leq$ Net.
Therapist share	Earned by delivering — completed and paid only. Includes a home visit's travel fee in full.
Partner share	A commission on net revenue
Clinic share	<code>Splittable Net - Therapist share - Partner share</code> . A gross figure.
Operating profit	Clinic share less the gateway fee and hand-entered business expenses. The only figure that may be called profit — and only because costs exist.
Package cash collected	What came into the bank up front
Recognised revenue	The same money, recognised one session at a time
Owed to therapists	An all-time balance , net of cash held. Never date-filtered.

The two identities that must always hold:

```
net           = gross - refunds
clinic share  = splittable net - therapist share - partner share
```

Three split rules, each a correction of a real misstatement:

- 1. **A therapist's share is earned by delivering, not by being booked.** Only a `completed` **and** paid session adds to it. Counting every paid session deducted a share nobody would ever be paid, which understated the clinic's take on every forfeited late cancellation.
- 2. **A home visit's travel fee is part of the therapist's share and is never revenue.** The Money screens must be passed the payout-enriched appointments (visit mode, travel fee, the cash columns) and the per-therapist home-visit rate. Passing the plain array silently moved the whole travel bill into the clinic's share.
- 3. **Refunds reverse the partner's commission, not the therapist's.** A refunded session was cancelled, so it never earned a therapist share; a hospital's cut is taken on net.

Different eligibility for revenue and for the split. Gross, refunds and net count **every** paid session. A session whose split is **unknowable** — no therapist share set, or a hospital-referred patient whose hospital has no share configured — is excluded **from the split alone** and surfaced as a **named count**. **Never guess a percentage to make the numbers tie.**

Flows are range-scoped; balances are not. "Owed to therapists" is all-time and net of cash held, matching what the Pay button actually transfers. Scoping it to the range in view once let an admin read "nothing owed" off a quiet week while a real debt sat outside the window. The label has to say which it is.

16.1 The reference dataset

Build this exact dataset before running §16.2 onward. It is small enough to compute by hand and exercises every rule.

#	Patient	Therapist	Mode	Paid	Status	Refund	Notes
S 1	A	A (60%)	online	₹1,999	completed	—	Ordinary delivered session
S 2	A	A (60%)	online	₹1,799	confirmed (not completed)	—	Paid, not delivered
S 3	A	A (60%)	online	₹1,999	cancelled	₹1,999 processed	Refunded outside the window
S 4	C (Hospital A, 10%)	A (60%)	online	₹2,499	completed	—	Partner attribution
S 5	C (Hospital A, 10%)	B (no share set)	online	₹1,999	completed	—	Excluded from the split
S 6	A	A (home 65%)	home visit	₹2,499	completed	—	Travel fee ₹150, paid online
S 7	B	A (home 65%)	home visit	₹2,499	completed	—	Travel ₹150, cash at the door , not remitted

Costs: the three expenses from §8.15. Gateway fee: **2%**.

Hand-computed expectations for the whole range (all figures in rupees):

- **Gross** = 1999 + 1799 + 1999 + 2499 + 1999 + 2499 + 2499 = **₹15,293** (S7 is a cash home visit: include it only once the cash is recorded as collected — see `FIN-SUM-002`.)
- **Refunds** = **₹1,999** (S3)
- **Net** = 15,293 – 1,999 = **₹13,294**
- **Excluded from the split**: S5 → count **1**, excluded revenue **₹1,999**
- **Splittable Net** = 13,294 – 1,999 = **₹11,295**
- **Therapist share** (completed **and** paid only):
 - S1 $1999 \times 60\% = 1,199.40$
 - S4 $2499 \times 60\% = 1,499.40$
 - S6 $2499 \times 65\% + 150 = 1,774.35$
 - S7 $2499 \times 65\% + 150 = 1,774.35$
- S2 (not completed) → **0**; S3 (cancelled) → **0**; S5 (excluded) → **0**
- **Total** ≈ **₹6,247.50**
- **Partner share** (10% of **net**, hospital-referred, non-excluded): S4 only → $2499 \times 10\% = ₹249.90$. S5 is excluded entirely, so it contributes nothing.
- **Clinic share** = 11,295 – 6,247.50 – 249.90 = **₹4,797.60**

- **Gateway fee** (2% of **online** collections, charged on **gross, skipped for cash-on-visit**): online gross = 15,293 – 2,499 (S7 cash) = 12,794 → $\approx ₹255.88$
- **Expenses in a September range** = 25,000 + 4,000 = **₹29,000** (the ₹6,000 August row is **outside** and must not be counted)
- **Operating profit** = 4,797.60 – 255.88 – 29,000 = **–₹24,458.28** (a loss — which is the honest answer for this dataset)

Rounding: each per-appointment share is rounded to the nearest paise **individually** before summing. Expect ± 1 paise against a spreadsheet that rounds at the end.

16.2 Money screens

FIN-SUM-001 — Summary and the two identities · P0

Steps. Open **Money** → **Summary**. Set the date range to cover the dataset. Read every figure. Compute the two identities by hand. **Expected Result.** `net = gross – refunds` and `clinic share = splittable net – therapist share – partner share` **hold exactly**. The figures match \$16.1. The excluded count reads `1` with `₹1,999` named. Every screen ends with the **MoneyGlossary**. No figure is labelled "approximate" — the split is exact over a stated subset.

FIN-SUM-002 — Paid vs unpaid, completed vs not · P0

Steps. Confirm S2's treatment. **Expected Result.** S2 (paid, **not** completed) **is** in Gross/Net, and contributes **nothing** to the therapist share. If the therapist share includes S2, the "earned by delivering" rule has regressed — that is a P0. **Cash check:** S7's cash home visit sits at `payment_status='unpaid'` for its whole life. **Read `payment\_mode` first.** It must not be presented anywhere as a failed or outstanding online payment.

FIN-SUM-003 — Date filtering, and what is never filtered · P0

Steps. Narrow the range to one day containing only S1. Read every figure on Summary. Then read **Owed to therapists**. **Expected Result.** Gross, refunds, net, and the split all narrow to S1. **"Owed to therapists" does not change** — it is an all-time balance, net of cash held, matching what the Pay button transfers. Its label says so. If it moves with the range, that is a P0: an admin could read "nothing owed" off a quiet week while a real debt sat outside the window.

FIN-BRK-001 — Breakdown agrees with Summary · P0

Steps. Open **Money** → **Breakdown** for the same range. **Expected Result.** Every figure and every chart segment matches Summary **exactly** — both come from **one pass** of the same maths. A discrepancy of any size is a P0.

FIN-TXN-001 — Transactions · P1

Steps. Open **Money** → **Transactions**. Reconcile every row against the dataset. Export CSV and PDF. **Expected Result.** One row per payment, with the order id and payment id. **No payment id appears on two rows.** The CSV and the PDF describe the **same table** — they are generated from one column definition, and the PDF route is sent the exact filtered rows the browser rendered. Both cover the **whole filtered set**. The PDF carries a subtitle naming the scope and the date. **No JSON export exists.**

FIN-PAY-001 — Payouts · P0

Steps. Open **Money → Payouts**. Read Therapist A's row. **Expected Result.** `Owed` matches \$16.1's therapist share for A, **less anything already settled**. The **Pay** button shows the net figure — owed minus cash held — not the gross owed. A therapist with **no revenue share set** shows the "not set" state rather than `₹0`; paying them is refused with `Set this therapist's revenue share % before paying out.`

FIN-PAY-002 — Settling a payout · P0

Steps. Settle Therapist A's payout. Then attempt to settle again immediately. Then double-click Settle on Therapist B. **Expected Result.** The transfer amount is recorded, `therapist_payout_paid_at` is stamped on exactly the sessions settled, and a ``payout.settle`` **audit row** is written **after** the compare-and-swap claim. A second attempt is a no-op or is refused — **no double payout**. The Owed figure drops to zero for the settled sessions.

FIN-PAY-003 — Netting cash off a payout is a remittance · P0

Purpose. Without this, the same rupees are deducted again on the next payout and the Cash Ledger goes on asking someone to chase money already recovered. **Preconditions.** Therapist A is holding S7's cash (₹2,499 + ₹150 travel, per the reconstructed total), un-remitted. **Steps.** Read the Pay button's figure. Settle. Then open the **Cash Ledger** panel on the same screen. Then run a second payout cycle. **Expected Result.** The transfer is reduced by the cash held. **The same run marks exactly those visits `cash\_remitted\_at`.** The Cash Ledger stops listing them. The second payout cycle does **not** deduct that cash again.

FIN-PAY-004 — A therapist holding more cash than they are owed · P0

Preconditions. Arrange cash held > amount owed. **Expected Result.** The transfer **floors at zero** (never negative). The difference is shown as **still owed to the business**, and **those collections deliberately stay open on the Cash Ledger** for a person to chase. They must not be silently marked remitted.

FIN-PAY-005 — Payout requests · P1

Steps. With a therapist request outstanding, tap **Start review**, then **Complete**. Then try to complete a request that was never reviewed. **Expected Result.** The states move `pending → reviewing → completed`. Completing without review is refused with `Start review on this request before marking it completed.` A completed request re-submitted returns `This request is already completed.` A stale action returns `This request is no longer pending – please refresh.` All audited.

FIN-PAY-006 — Correcting a cash amount is the admin's job, not the therapist's · P0

Steps. As Admin Full, correct S7's cash amount to `₹2,000` with the reason `Patient short ₹649 at the door; agreed balance next visit.` Then try with a blank reason, then on an **already remitted** visit. **Expected Result.** Valid: succeeds with a CAS on the figure being replaced, and writes a ``cash.correct_amount`` audit row. Blank reason: refused. Already remitted: **refused** — that transfer has gone out, so the fix is an adjustment against the next payout, not a silent edit of a settled one (`This session isn't a cash-on-visit home visit.` / the remitted refusal). A stale figure returns `Someone else changed this figure.` Refresh and try again. As Admin Ops (no `money` scope): **403**, and the control does not render.

FIN-COST-001 — Costs · P1

Steps. Open **Money → Costs**. Add the three expenses from \$8.15. Set the range to September 2026. **Expected Result.** Only the two September rows count; the 28 August row is excluded — expenses are dated by **when they were incurred**, not when they were typed in. Negatives: `Enter an amount greater than zero.`, `Pick the date this cost was incurred.` Deleting an expense removes it from the total.

FIN-COST-002 — Operating profit and the gateway fee · P0

Steps. Read Operating profit. Then change the gateway fee percentage and re-read. **Expected Result.** The gateway fee is **derived automatically** from what was collected **online**, charged on **gross** (a processor keeps its fee through a refund), and **skipped for cash-on-visit**, which never touches a gateway. Operating profit = clinic share – gateway fee – expenses. Changing the percentage moves it. **With no costs recorded for a range, Operating profit is a ceiling and the screen must say so** rather than implying a number it cannot know. **Nothing here is post-tax — the label must never read "net profit"**.

FIN-REF-001 — Refunds across the screens · P0

Steps. Refund S3 in full. Then partially refund another session by ₹500 with the reason Session cut short by a connection failure. Then attempt a partial refund of ₹0, and one with no reason. **Expected Result.** Full: Gross unchanged, Refunds +₹1,999, Net –₹1,999, **therapist share unchanged, partner share reduced**. Partial: the same shape at ₹500. ₹0: Enter a refund amount greater than zero. No reason: Say why this refund is being made. A second full refund: This session has already been refunded in full.

FIN-REF-002 — Refund failure at the gateway · P1

Steps. Force a Razorpay refund failure (use a payment that cannot be refunded in test mode). **Expected Result.** Razorpay refused the refund. Nothing was refunded – check Razorpay and retry. **Nothing is marked refunded locally** — the local state must never claim a refund the gateway did not make.

FIN-REF-003 — A cash refund becomes a manual pending item · P1

Steps. Refund a cash-on-visit home purchase. **Expected Result.** With no Razorpay payment behind it, the refund becomes refund_status='manual_pending' and is **surfaced on the admin Cash Ledger** until an admin confirms the cash was handed back. Marking it returned clears it and is audited. Attempting to refund a cash purchase as if it were online: Cash-on-visit packages have no single payment to refund – cancel visits individually instead.

FIN-REF-004 — A refund voids available credits, never delivered ones · P0

Preconditions. A 6-session purchase with 2 sessions **completed** and 4 available. **Steps.** Refund the package. **Expected Result.** The **4 available** credits are voided; the **2 delivered stay delivered**. The ledger records a void for exactly 4. The patient's widget shows the programme as refunded with nothing available. **A delivered session is never un-delivered.**

16.3 Payment integrity (duplicates, concurrency, webhooks)

PAY-DUP-001 — One payment, one appointment · P0

Steps. Complete PAT-B00K-003, then reload and inspect Sessions → All Sessions and the payments table. **Expected Result.** Exactly one appointment, exactly one payments row. **The appointment is created before the order, and the order is minted against it**, so there is no path by which one payment creates two appointments.

PAY-DUP-002 — One order, one purchase · P0 [SQL]

Steps. Attempt to insert a second payments row with an existing razorpay_order_id. **Expected Result.** The unique index rejects it. **Do not drop that index to make an import succeed** — a collision means a duplicate already exists and wants investigating. Repeat for razorpay_payment_id.

PAY-DUP-003 — One purchase, one entitlement · P0

Steps. After a care-plan purchase, trigger the verify route twice (retry the callback). **Expected Result.** Exactly one entitlement, `sessions_granted = 6`. The grant function is idempotent.

PAY-DUP-004 — Duplicate webhook · P0

Preconditions. `RAZORPAY_WEBHOOK_SECRET` is set. **Steps.** Capture a real webhook body and its `x-razorpay-signature` header from the Razorpay dashboard's webhook log. POST it to `/api/razorpay/webhook`. POST the identical body again. **Expected Result.** The first is processed. The second collides on `razorpay_event_id` in `payment_webhook_events` and is treated as already-seen — **that insert is the deduplication, and it happens before any work is done**, so a retry arriving mid-flight cannot do the work twice. Money → Transactions shows the payment **once**. Gross Revenue does not double.

PAY-WH-001 — Signature verification · P0

Steps. POST a webhook body with (a) no signature header, (b) a wrong signature, (c) the correct signature but a **re-serialised** body (`JSON.parse` then `JSON.stringify`). **Expected Result.** (a) `400 Missing signature`. (b) `400 Invalid signature`. (c) `400 Invalid signature` — and this is correct: the signature is checked against the **raw** body, because a re-serialised body does not round-trip byte-for-byte. **The fix for a legitimate webhook failing here is never to skip the check.**

PAY-WH-002 — Webhook races the browser callback · P0

Steps. Complete a payment and, as close to simultaneously as you can manage, POST the webhook body and let the browser callback fire. **Expected Result.** Whichever arrives first applies the capture; the second finds it captured and **changes nothing**. The appointment is paid once, `payments` has one row, and the Meet event is created **once** (the webhook only creates one if the appointment does not already carry an event id).

PAY-WH-003 — Non-capture events are recorded and ignored · P2

Steps. POST a `payment.failed` webhook with a valid signature. **Expected Result.** The event row is recorded and marked processed. No capture is applied.

PAY-WH-004 — Without the webhook secret, a closed tab loses the confirmation · P0

Steps. Unset `RAZORPAY_WEBHOOK_SECRET`, restart, and POST any webhook. **Expected Result.** `503 {"error":"Webhook not configured"}`. **Consequence to state in your report if you see it in the wild:** a patient who pays and closes the tab before the callback lands leaves a **paid Razorpay order against an unpaid booking**. Restore the secret.

PAY-DUP-005 — Double settle · P0

Covered by `FIN-PAY-002`. **No double payout.**

PAY-DUP-006 — Partner commission is not accumulated twice · P0

Steps. Re-run the webhook and the callback for S4, then read Money → Breakdown and the hospital's Earnings. **Expected Result.** `₹249.90` appears **once** on both.

PAY-DUP-007 — Credit idempotency keys are derived, never random · P0 [SQL]

Steps. Inspect `session_credit_ledger` for a booked package session. **Expected Result.** Keys read `reserve:<appointment_id>` and `consume:<appointment_id>`. **A random key would make every retry look like a new event, which is the exact bug the key exists to prevent.** Availability is checked

after idempotency in the reserve function, deliberately — checking availability first would answer "no credits available" for a booking that in fact succeeded.

PAY-CONC-001 — Twelve concurrent reserves against one credit · P0 [script]

Steps. With a purchase holding exactly **one** remaining credit, fire twelve concurrent booking requests for distinct slots. **Expected Result. Exactly one succeeds.** The rest are refused. The row lock in the reserve function is real, and the CHECK constraint on the cached counts makes an overdrawn balance impossible rather than merely unwritten.

PAY-CONC-002 — Two admins settle the same payout at once · P0

Expected Result. One wins; the other is refused. The audit log records **only the winner** — the log write happens after the CAS claim.

PAY-AMT-001 — The amount is server-derived · P0

Steps. Intercept the `create-order` request and change any client-supplied amount or package id. Also attempt `create-order` with another patient's `appointmentId`. **Expected Result.** The amount charged is **re-derived server-side** from the category or the catalog row. A tampered amount has no effect. Another patient's appointment is **not found** under the caller's own scoped client → `Appointment not found` (404). Attempting to pay an already-paid booking: `This booking is already paid`.

PAY-AMT-002 — A care-plan price cannot be tampered with · P0

Steps. Accept a recommendation, intercepting the request to change the package id to a cheaper one. **Expected Result.** The route **re-derives the price from the plan's own recommended package** and refuses a catalog mismatch (`That recommendation is incomplete.` / a mismatch 409). The programme granted is the one recommended, at the price recommended.

17. Public marketing site test plan

17.0 Feature guide

The eight public pages are **one template, not eight layouts**. Every page assembles from the same design system: a hero (photo right, one headline, one sentence, up to two calls to action), a trust bar, some section bands, an "explore" strip, and a closing call to action. **Every page ends the same way on purpose** — wherever a visitor stops reading, the next step is in the same place.

The site's own index lives in **one array**, which the header nav, the footer's Explore column, the home page's connector grid and every "Where to go next" strip all read. So a page cannot exist in the header and be missing from the index, and a renamed page cannot leave a stale description behind.

Every Explore band ends on Book a session, on all eight pages, in the same full-width photo-beside-text tile — booking was on the home page's grid alone, so the six inner pages ended their index on another page to read. And **the page tiles above it square up**: the count varies (the page you are on is always missing, and Home Visit drops out when the clinic switches it off), so a row that would end short stretches its leftover tiles across it rather than leaving dead cells on the right.

PUB-EXP-001 — The Explore band, on every public page · P1

Steps. Open each of `/`, `/conditions`, `/how-it-works`, `/home-visit`, `/team`, `/mission`, `/faq`, `/hospitals` and read the Explore band at the foot. **Expected Result.** Every one ends with **Book a**

session → `/book`, as the last tile, full width, photo beside the text. Above it: every other page, never the one you are on, and never Home Visit while the master switch is off. **Alignment.** With the usual **seven** page tiles: two rows of three, then the seventh **stretched across the whole row** in the same wide layout — no empty cells to the right of it. Switch Home Visit off and reload: **six** tiles, two clean rows of three, nothing stretched. Narrow the window to the two-column breakpoint and repeat both: the last row must still be full. **Critical check:** this is arithmetic, not a hand-placed exception (`src/lib/exploreGridSpans.ts`, unit-tested). A tile that is full width on a tablet and half width on a desktop must **not** switch to the photo-beside-text layout — that would read as two designs rather than one stretched tile.

Word budgets are numbers, not a vibe — the rewrite exists because visitors could not tell what the site was, and the second round of feedback was that there was still too much to read:

Slot	Budget
Hero subtitle	12 words
Section lede	9 words — and dropped entirely when the heading already says it
Icon card / step / split-feature body	10 words
Bullet / check	5 words
Care-area blurb 8 words · detail 14 words	
Page blurb	8 words
Closing call-to-action body	12 words
Mission / vision sentence	15 words

Photography is load-bearing, not decoration. A visitor should be able to tell what a page is about with the text blurred out. Three rules hold:

1. **Every photograph shows a screen** — a laptop, tablet or phone in frame — **except the two home-visit images**, which show hands-on treatment. This clinic sells video consultations; a site of clinic photography reads as a walk-in practice.
2. **Every photograph shows a face, and the face is glad to be there.** The one exception is the clinician reading a scan, who is concentrating — a physiotherapist grinning at an X-ray is the opposite of reassuring.
3. Photos are **static imports**, never `/photos/x.jpg` strings and never remote URLs, so a missing file is a compile error.

PUB-CTA-001 — The closing band asks for the sale · P1

Steps. Open each of `/`, `/conditions`, `/how-it-works`, `/home-visit`, `/team`, `/mission`, `/faq` and read the very last band. (`/hospitals` deliberately ends on its referral form instead.) **Expected Result.** A teal band with the copy on the left — headline, one line, one or two buttons — and a **photograph on the right**: a patient at home, smiling, booking on her phone. The same photograph on all seven; only the headline, body and buttons change per page. Over the photo, a white chip reading **Session confirmed / Tuesday, 6:00 PM / Calendar invite on its way.** and, underneath it, **Example of what you get** — the chip must always say it is an example, on every page. **Assurances.** Three lines under the buttons: *One-to-one, never a group, Reports read before your session, Secure UPI payment.* **Critical check:** none of those three states a **number**. A session's length is set per treatment category and the cancellation window is an admin setting, so a fixed "60 minutes" or "free cancellation up to 24 hours" printed here would be a promise the settings can contradict. If you see a number in this band, that is a bug. **Layout.** Narrow to a phone width: the copy and buttons come **first**, the photograph below them — nobody should have to scroll

past a picture to reach the button. The image keeps the same landscape crop at every width; it must never go tall and leave the band mostly empty beside the text.

PUB-HOME-001 — The home page · P1

Steps. Open `/`. Scroll to the bottom. **Expected Result.** Hero → trust bar → care-area showcase → walkthrough → programmes → testimonials → mission band → connector grid. The **mission band gives the mission and vision in full** (they are two sentences; paraphrasing would make the home page a weaker version of the same claim) while the **four promises appear as titles only**, each linking to the mission page's promises anchor. The connector grid shows the other seven pages **plus booking** — the index of the site always ends on the one action the site exists for.

PUB-COND-001 — Care areas show one photograph at a time · P1

Steps. On `/` and `/conditions`, use the care-area showcase: swipe, the arrow buttons, and the picker. **Expected Result.** **One panel at a time** — photograph left, the answer right, the other five one tap away. All three controls go through one selection path so they cannot disagree about what is showing. The picker is a real tablist with **roving focus and arrow keys**. **It never advances by itself** — the home page already carries the auto-rotating walkthrough, and two moving things is worse than either alone. Its accessible name is **"Areas of practice"**, distinct from the walkthrough's **"How the process works"**.

PUB-NAV-001 — The section rail and the scroll arrow · P1

Steps. On each public page, use the section rail's entries and then the bottom-right scroll arrow repeatedly. **Expected Result.** Every rail entry corresponds to a section that **actually rendered** (several bands are conditional on admin-controlled catalog data), and the entries are in **DOM order**. The arrow walks the list **top to bottom** — if it ever sends you backwards, an entry is out of order.

PUB-CAT-001 — A catalog card opens a dialog; booking is its own button · P1

Steps. On `/` and `/conditions`, tap a programme (treatment category) card's **body**. Then tap its **Book ...** link. **Expected Result.** The card body is **one tap target that opens a detail dialog** carrying the long description and what the programme covers. The **Book ...** link sits **below the card** and again at the foot of the dialog, **outside** the tap-target button (a link nested inside a button is invalid markup and behaves differently per browser). It goes to `/book?category=<id>` — a **first session**, never a course of them. **Cover images:** a card with no photo shows the **shared placeholder panel at the same height** as one with a photo. It must never look like an image that failed to load.

PUB-CAT-002 — No course of treatment is advertised anywhere public · P0

Feature. A course of treatment is a clinical recommendation, so the public site does not carry a price list of them. Removed outright rather than hidden behind a setting: a toggle somebody can flip back on is not the rule being gone.

Preconditions. Packages P1-P3 exist, are `active`, and have `visible_on_home` and `visible_on_conditions` on. That matters — this is an absence tested against rows that genuinely could have rendered.

Steps. Read `/` and `/conditions` end to end, including inside every programme detail dialog. Then read `/home-visit`. **Expected Result**

- **No session package appears anywhere** — no card, no title, no price, and no "Where this usually leads" list inside a programme's dialog.
- **No link anywhere matches `/book?package=`**. A card still linking to package checkout would take money for something the server now refuses; this is the assertion that catches a partial revert.

- `/home-visit` shows **single**-visit packages only. `HV1` (1 visit) is there and books directly; `HV2` (4 visits) is **absent** — it is a recommendation, not a product.
- There is **no admin switch** for any of this. "Show programme prices publicly" no longer exists on Settings; if you find it, that is a defect.

`PUB-TEAM-001` — Team · P2

Expected Result. Only approved, active, team-visible therapists appear. Tapping one opens a popup with their profile and a **Book with ...** action carrying `?therapist=` into the wizard.

`PUB-FAQ-001` — FAQ · P2

Expected Result. The accordion renders the admin-managed FAQs in order. With none configured, the page shows a sensible empty state rather than a broken band.

`PUB-HV-001` — Home visit page · P1

Covered by `PAT-HV-001` and `ADM-SET-013`.

18. Security and authorization test plan

The rule for this whole section: do not only verify that the UI hides something. Every case has a route-level twin. The application enforces its gates in two places — the proxy for navigation, and `requireActiveProfile` / `requireAdmin` / `requireAdminScope` inside the routes — because a valid session cookie can call the API around the UI.

How to call a route as a given user. Sign in as that user in a browser, copy the session cookie from DevTools → Application → Cookies, and use it with curl:

```
curl -i -X POST http://localhost:3000/api/<route> \
  -H 'Content-Type: application/json' \
  -b '<paste the cookie header>' \
  -d '{ ...body... }'
```

18.1 Signed-out access

`SEC-ROUTE-001` — Every protected route redirects a signed-out visitor · P0

Steps. In a private window, navigate to `/patient/dashboard`, `/patient/dashboard/health-profile`, `/therapist/dashboard`, `/therapist/dashboard/earnings`, `/hospital/dashboard`, `/hospital/dashboard/revenue`, `/admin/dashboard`, `/admin/dashboard/patients/<id>`. **Expected Result.** Each redirects to that role's own login page. **No protected content is rendered even for a frame.**

`SEC-ROUTE-002` — Every mutating route refuses an anonymous caller · P0

Steps. Call a representative route from each family with **no cookie**: `/api/appointments/create`, `/api/appointments/cancel`, `/api/patient/condition-profile/submit`, `/api/therapist/save-availability`, `/api/therapist/care-plan/submit`, `/api/hospital/withdraw-referral`, `/api/admin/approve-account`, `/api/admin/settle-therapist-payout`, `/api/medical-documents/view`, `/api/razorpay/create-order`. **Expected Result.** **401 `Not signed in` or 403 `Forbidden`** on every one. **None returns 200, and none leaks data in the error body.**

18.2 Cross-role access

SEC-ROUTE-003 — A signed-in user of the wrong role is sent to Get Started · P0

Steps. As each of patient / therapist / hospital, navigate to each of the other three dashboards. **Expected Result.** Every one redirects to `/get-started``.

SEC-ROUTE-004 — The back office is never named to an outsider · P0

Steps. As a signed-in **patient**, navigate to `/admin/dashboard`. Then view the page source of `/`, `/patient/dashboard` and `/team`, and search for `/admin/login` and `/admin/dashboard`. **Expected Result.** The navigation redirects to `/get-started``, never to `/admin/login` — which would confirm the back office exists and name its door. **No admin path appears in any public client bundle:** the `role→dashboard` mapping is resolved **server-side** at `/dashboard`, and `Navbar` and the wrong-account panel link to `/dashboard`. **The Debug Bar is the deliberate exception** — it still lists the admin routes, and is deleted before release. If you find an admin path in a public bundle **outside** the debug bar, that is a P0.

SEC-ROUTE-005 — `/admin/login` is not indexed · P2

Expected Result. The page carries `robots: noindex`.

SEC-ROUTE-006 — `/dashboard` routes by role, server-side · P1

Steps. As each role, open `/dashboard`. Then open `/dashboard?hash=<something>` and `/dashboard?hash=//evil.example.com`. **Expected Result.** Each role lands on their own dashboard. A legitimate `hash` becomes a real fragment (the anchor-based shells need it) and is **pattern-checked**; a value that could smuggle a host is rejected. **No open redirect.**

18.3 Horizontal privilege (one user reaching another's data)

SEC-DATA-001 — Patient A cannot read Patient B · P0

Steps. As Patient A: (a) call `/api/packages/purchase-detail` with Patient B's purchase id; (b) call `/api/home-visit/purchase-detail` likewise; (c) call `/api/appointments/cancel` with Patient B's appointment id; (d) call `/api/medical-documents/view` with Patient B's document id; (e) call `/api/patient/condition-profile/export` and check whose record comes back. **Expected Result.** (a)–(d) all refused. The purchase-detail routes query with **the caller's own RLS-scoped client**, so **a row coming back at all is the authorization check** — there is deliberately no manual ownership branch duplicating what the policies guarantee. (e) returns **only Patient A's** record.

SEC-DATA-002 — Therapist A cannot reach Therapist B's data · P0

Covered by `THR-SEC-001`, `THR-SEC-002`. Additionally: `/api/therapist/record-cash-collection` with Therapist B's appointment id → refused; `/api/therapist/session-notes/submit` for Therapist B's session → refused; `/api/therapist/suggest-session` on a programme locked to Therapist B → `That isn't your programme.`

SEC-DATA-003 — Hospital A cannot reach Hospital B · P0

Covered by `HOS-SEC-001`.

SEC-DATA-004 — Private documents · P0

Steps. Upload a report as Patient A. Copy the signed URL. (a) Open it in a private window **within** 120 seconds. (b) Open it **after** 120 seconds. (c) Try to guess/construct a direct Storage object URL for the `medical-reports` bucket. (d) As Therapist A (assigned), view it. (e) As Therapist B (not assigned), request

it. **Expected Result.** (a) opens — a signed URL is a bearer token by design, which is why it is short-lived. (b) **fails**. (c) **fails — the bucket is private**, unlike `avatars`. A scan report is the most sensitive thing this application holds, and a public bucket would make the object URL itself the only secret. (d) allowed. (e) refused. **Also confirm:** `patient_medical_documents` has **no bytea/base64 column** — a handful of MRI PDFs stored inline would dominate the database's size and ride along on every read of a patient's chart.

SEC-DATA-005 — Contact information exposure · P0

Covered by `THR-SESS-003` (masking, and the plaintext number absent from the page source) and `THR-SESS-004` (the reveal log). Additionally: check the **admin export** of patients — a masked field must mask the same way there, and no export anywhere emits JSON.

18.4 Vertical privilege (scope)

SEC-ADMIN-001 — Scoped admins · P0

Covered by `ADM-SET-027` and `ADM-SET-028`. Run the whole table.

SEC-ADMIN-002 — A non-admin cannot call an admin route · P0

Steps. With **each** of a patient's, a therapist's and a hospital's cookie, call ten admin routes across different sections. **Expected Result. 403 on every one.**

SEC-ADMIN-003 — Scope changes are self-protecting · P0

Covered by `ADM-SET-025`: no self-change, and the last `full` admin cannot be narrowed.

SEC-AUTH-006 — A suspended account is locked out everywhere · P0

Steps. Suspend Patient A, Therapist A and Hospital A in turn. With each still-valid cookie: load the dashboard, then call a self-service route. **Expected Result.** Dashboard → ``/account-suspended``. Route → **403** (`Your account has been suspended.` / `Your account is not active.`). **A live cookie must not outlive suspension. Contrast:** an **admin** is refused on `active` but **deliberately not on `approved`** — an admin is promoted by hand rather than through the signup queue, so gating on approval would lock out the people it protects. Verify an admin with `approved=false` **can still sign in.**

18.5 Input tampering

SEC-TAMPER-001 — Manipulated ids · P0

Steps. For ten routes, substitute another user's id, a random UUID, an empty string, `null`, and a non-UUID string. **Expected Result.** Each returns a **readable 400/403/404. No 500 with a database message.** No response body contains a table name, a column name, a row id belonging to somebody else, or a stack trace.

SEC-TAMPER-002 — Manipulated prices · P0

Covered by `PAY-AMT-001` and `PAY-AMT-002`.

SEC-TAMPER-003 — Manipulated session counts · P0

Steps. Attempt to book more package sessions than remain by sending extra slots; attempt to set `sessions_granted` through any route. **Expected Result.** Refused. **There is no route that accepts a session count from the client.** The count comes from the frozen snapshot.

SEC-TAMPER-004 — Manipulated permissions · P0

Steps. As a patient, send `{"role":"admin"}` and `{"admin_scope":"full"}` in the body of a profile-update route. As a scoped admin, call `set-admin-scope` on yourself. **Expected Result.** All refused or ignored. **Never trust a role, an id, or an amount sent from the client — it is re-derived server-side.**

SEC-TAMPER-005 — Malformed bodies · P1

Steps. POST invalid JSON, an empty body, a deeply nested object, and a 5 MB string field to ten routes. **Expected Result.** `Malformed payload` or a specific field message, always **4xx**, never a 500 and never a crash.

SEC-TAMPER-006 — Duplicate submissions · P0

Covered by `PAT-B00K-017`, `PAT-SUGG-004`, `THR-AVAIL-004`, `FIN-PAY-002`, `PAY-DUP-004`.

18.6 Authentication behaviour

SEC-AUTH-004 — No email-confirmation step exists · P0

Steps. Register through **all four** sign-up call sites: `/patient/register`, the `/book` wizard, the `/book-home-visit` wizard, and `/therapist/login` → Apply to Join. **Expected Result.** Every one returns a session immediately. **None shows a "check your email" instruction.** If a sign-up returns no session, the app reports it as a **failure** (`Your account was created but we couldn't sign you in to finish this booking. Please sign in and try again.`) — because that state means the Supabase project has *Confirm email* switched on, which is a misconfiguration, not a step.

SEC-AUTH-005 — Session expiry · P2

Steps. Invalidate the session server-side (sign out elsewhere / delete the cookie) and then submit a form mid-flow. **Expected Result.** A readable message such as `Your session expired. Please refresh the page and try again.` — not a raw 401 body, and not a silent no-op.

SEC-AUTH-007 — A display code outlives the role that generated it · P2 [SQL]

Purpose. Every self-signup is inserted as a patient, so an account later promoted to admin or hospital **keeps its `PT####`**. **Steps.** Promote a patient to hospital, then register a new patient. **Expected Result.** The new signup succeeds. The uniqueness of codes is scoped to the **column**, not the role, and the sequence resync takes its max over **every** non-null code regardless of role. **A signup failing intermittently with a 500 from `auth.signUp` and an empty body is the symptom of this being broken.**

19. UX, responsive and accessibility test plan

19.1 Viewports to test

Device class	Viewport	Coverage
Desktop	1440 × 900	Everything
Tablet	820 × 1180	Booking, patient dashboard, admin All Sessions
Mobile	390 × 844	The full patient booking + payment flow, mandatory

19.2 Mobile booking flow

UX-MOB-001 — The critical path at 390 × 844 · P0

Steps. At 390 × 844, execute `PAT-B00K-002` → `PAT-B00K-003` end to end, including Razorpay checkout.

Expected Result — check every one of these:

- **No horizontal scrolling on any screen.** The page body never scrolls sideways. Wide content (tables, the calendar) scrolls **inside its own container**, not by moving the page.
- The Step 1 **calendar is legible and its day cells are tappable** — this is the control most at risk, because the calendar is a 7-column grid squeezed directly by the card's padding. Padding is deliberately reduced on phones for exactly this reason; each day cell must remain a comfortable tap target (≈ 44 px).
- Hour chips and language chips wrap rather than overflow.
- All text is readable without zooming; nothing is clipped mid-word.
- **The on-screen keyboard does not hide the field being typed into, nor the primary button.** Tap each Step 2 field in turn and confirm you can still see what you are typing and can reach **Review Booking** →.
- The Step 3 summary rows do not collide; the fee is fully visible.
- The **Razorpay checkout renders and completes** at this width.
- The **Payment Confirmed** panel and its **Go to Dashboard** button are fully visible.

UX-MOB-002 — Dashboards on mobile · P1

Steps. At 390 × 844, open each of the four dashboards.

UX-BUSY-001 — The app says it is working · P1

Steps. Tap anything that saves, creates or navigates — a Save on a settings form, **Done** on a session, a payout, a sidebar entry — and watch the very top of the viewport. **Expected Result.** A thin teal bar appears across the top for the whole time between the tap and the page answering, then completes and fades. It covers the part a button cannot: after `router.refresh()` the button is idle (and often unmounted with its row), while the server is still re-rendering — the gap that reads as a freeze. **Details that are the design, not decoration:**

- **No percentage.** The bar eases toward a ceiling and only completes when the work lands. A bar sitting at 90% is a defect, not a slow server.
- **It waits ~220ms before drawing.** An action that finishes faster must show **nothing at all** — a flash on every tap is the failure this delay prevents.
- **Reduced motion** (`prefers-reduced-motion: reduce`) keeps the bar and drops the travel: a static full-width band that fades. It must not disappear entirely — that setting means less movement, not less information.
- **Two overlapping actions:** start a second before the first finishes; the bar must stay up until **both** are done, not vanish with the first.
- The five money buttons (**Done**, **Collect ₹...**, **Request Payout**, **Confirm ... Payment**, **Assign & Confirm**) show a spinning ring beside their busy label rather than only swapping the text.

UX-BUSY-002 — The admin dashboard's second batch · P2

Steps. Load `/admin/dashboard`, then tap any button that saves. **Expected Result.** Noticeably quicker than before: eleven migration-dependent reads that used to run one after another (accounting health, the suggestion and recommendation switches, discounts, the first-session offer, promo and invite settings,

category covers and condition types, testimonial avatars, hospital notes) now run as one parallel batch, and both ledger-balance passes run together. **Critical check — the isolation is unchanged:** drop one of those columns (see `admin-degraded-schema.spec.ts`) and only that panel degrades. If the whole dashboard blanks, the batch has lost a `guard()` and that is a P0.

Expected Result. Each shell offers a **mobile drawer** for the sidebar. **Back to Home is present in all three renders** — expanded sidebar, collapsed rail, and mobile drawer. On all four shells it sits at the **foot of the nav, directly above Collapse** (above the profile/Log Out footer in the mobile drawer, which has no Collapse). Without it the only exit from a dashboard is Log Out, which also ends the session. It is a plain link, not a client-side transition, because transitions into a differently-chromed route were silently not completing.

UX-MOB-003 — Modals and drawers fit · P1

Steps. At 390 × 844 open: a catalog detail dialog, the admin session drawer, the intake wizard, the pain-exam dialog, the confirm dialog. **Expected Result.** Each fits the viewport, scrolls internally if it must, and its close control is reachable without scrolling.

UX-MOB-004 — The body map on a phone · P1

Expected Result. Front and back views **stack** rather than shrinking each tap target below a fingertip. Every region is tappable.

UX-MOB-005 — Admin tables on mobile · P2

Expected Result. Wide tables scroll inside an `overflow-x` container. The page itself never scrolls horizontally. The pager remains usable.

19.3 Accessibility pass

UX-A11Y-001 — Keyboard navigation of the booking flow · P1

Steps. With the mouse untouched, complete Step 1 and Step 2 using only Tab, Shift+Tab, arrows, Space and Enter. **Expected Result.** Every interactive control is reachable in a sensible order. **A visible focus ring is present on every focused control.** The calendar and the chip groups are operable from the keyboard. No control is reachable only by pointer.

UX-A11Y-002 — Accessible names · P1

Steps. With a screen reader or the accessibility inspector, walk the booking wizard, the roster editor and the admin sidebar. **Expected Result.** Every button has a name that says what it does (not "button" or an icon name). The roster's controls are labelled per day and per period (e.g. "Remove Monday period 1", "Set Monday to ..."). Photo `alt` text **describes the picture**, not the page — the blurb already says what the page is for, and a screen reader announcing the same sentence twice tells someone nothing about the image.

UX-A11Y-003 — Dialog focus · P1

Expected Result. Opening a dialog moves focus into it; Tab is trapped inside; Escape closes it; focus returns to the control that opened it.

UX-A11Y-004 — Form errors are announced and visible · P1

Expected Result. A validation failure moves focus to or announces the message, and the message is visible without scrolling on a 390-wide screen.

UX-A11Y-005 — Two tablists must not share a name · P2

Expected Result. The care-area picker is "Areas of practice"; the walkthrough is "How the process works". **Two tablists sharing a name makes both unfindable.**

UX-A11Y-006 — Reduced motion · P1

Steps. Enable "reduce motion" in the OS, then load `/`. **Expected Result.** The splash is **skipped outright** — it is decoration over content that is already rendered, so the honest answer to "don't animate" is not to show it. The walkthrough does not auto-rotate distractingly.

UX-THEME-001 — No hydration warnings · P1

Steps. With the console open, load `/`, `/book`, and each dashboard. **Expected Result. No hydration mismatch warnings.** The splash's visibility is an attribute on `<html>` read by CSS, not React state, precisely to avoid this. `suppressHydrationWarning` covers that one element only — if you see it on a descendant, raise it.

20. Error, loading and empty-state testing

The rule that applies to every case in this section: an internal database error, a column name, a row id or a stack trace **must never reach the screen**. Every route tree has an error boundary, and only the framework's opaque `digest` is shown — because an error message can carry a column name or a row id, and **patients see these screens**.

ERR-BOUNDARY-001 — Route error boundaries · P0

Steps. Force an error in each dashboard (block the Supabase host in DevTools, then navigate). **Expected Result.** A friendly error screen with a retry affordance and **only** an opaque digest. **No message, no column name, no id, no stack.** Repeat for a root-layout failure — the global error page **inlines its styles and supplies its own document shell**, because at that point nothing else has rendered.

ERR-LOAD-001 — Loading states keep the chrome · P1

Steps. Throttle the network to Slow 3G and navigate between dashboard sections. **Expected Result.** A skeleton appears. **The sidebar is not blanked** — the patient, therapist and hospital dashboards render their sidebar per page rather than in a layout, so a loading screen that forgot the sidebar would blank the chrome on every navigation.

ERR-NET-001 — Network failure mid-action · P0

Steps. Set the network to Offline, then: submit the booking; accept a suggestion; save a roster; settle a payout. **Expected Result.** Each shows a readable connection message (e.g. `Could not reach the server. Please check your connection and try again.`). **Nothing clears optimistically** — the person is left exactly where they were and can retry. When the network returns, a retry succeeds and **does not duplicate** the action.

ERR-API-001 — API failure is readable · P1

Steps. Force a 500 from a route (stop Supabase mid-request). **Expected Result.** A human message. Never the raw error text.

ERR-STALE-001 — Stale URLs · P1

Steps. Open `/admin/dashboard?section=today&tab=requests` and `?section=today&tab=sync` (tab keys that never existed). Open `/admin/dashboard/patients/<deleted id>`. Open `/book?category=<deleted id>`.

Expected Result. The admin links **fall back to that section's first screen** — so they *look* like they work. **That is exactly why links must be built with the typed helper rather than hand-written.** A deleted patient id shows a not-found state, not a crash. A deleted category id simply leaves the concern unselected.

ERR-404-001 — Not found · P2

Expected Result. `/no-such-page` renders the 404 page. `/home-visit` with the master switch off renders 404. Neither leaks internals.

ERR-EMPTY-001 — Empty states everywhere · P2

Steps. Immediately after `SETUP-RESET-001`, open every admin screen, then each role's dashboard for a brand-new account. **Expected Result.** Every screen shows a **purposeful empty state**, never a blank panel, a zero-filled table, or a spinner that never resolves. **Filter chips are hidden unless at least two of them would have rows behind them** — a filter nobody can act on is noise. **A screen that can only ever be empty is not in the sidebar at all.**

ERR-DUP-001 — Duplicate submission of every form · P1

Steps. Double-tap the primary button on: the booking wizard, the register form, the roster save, the care-plan submit, the suggestion send, the payout settle, the refund, the expense create. **Expected Result.** Exactly one record in every case. Buttons disable synchronously, not a render later.

21. Cross-role consistency tests

Why these exist. Most defects in a multi-role application are not "the feature is broken" but "two roles disagree about the same fact". Each case below drives **one business event** and then checks **every** role's view of it.

XR-B00K-001 — One booking, five views · P0

Event. Patient A books and pays for a session (`PAT-B00K-003`), and an admin assigns Therapist A.

Role	Where	Must show
Patient	<code>/patient/dashboard/sessions</code> → Upcoming	The session, <code>Confirmed</code> , with Therapist A's name and the Meet link
Therapist	<code>/therapist/dashboard/sessions</code>	The same session, same slot, patient's name, masked phone , no email
Admin	Sessions → All Sessions and Sessions → Schedule	The same session, in both, opening the same drawer
Finance	Money → Transactions	One payment row of ₹1,999 with its order and payment ids
Hospital	Your Referrals	Nothing — Patient A was not referred

Expected Result. The slot time is identical everywhere (allowing for the timezone label each screen states). The status is identical. **No screen shows a therapist the others do not.**

XR-B00K-002 — A referred patient's booking adds a sixth view · P0

Repeat with Patient C (Hospital A). **Additional expectation:** the hospital sees the referral as **Registered**, and once the session is completed, ₹249.90 appears on their Earnings and in Money → Breakdown's partner share — **the same number in both places**.

XR-PAY-001 — One payment, one figure everywhere · P0

Event. The payment from XR-B00K-001. **Expected Result.** Patient's Payments screen, Money → Transactions, Money → Summary's Gross, and the appointment's own paid state all describe **one** payment of ₹1,999. **No screen shows it twice and no screen shows a different amount.**

XR-REFUND-001 — One refund, four views · P0

Event. Patient A cancels S3 outside the window. **Expected Result.** Patient: cancelled, refund shown. Therapist: the session leaves Upcoming; **their earnings do not change** (it never earned a share). Admin Money: Gross unchanged, Refunds +₹1,999, Net −₹1,999. Hospital (if referred): partner share **reduced**. Activity Log: the cancellation, attributed.

XR-COMPLETE-001 — One completed session, five views · P0

Event. Therapist A completes S1. **Expected Result.** Therapist: Earnings +₹1,199.40, and a "Notes to write" nudge. Patient: the session moves to Past and a rating control appears. Admin Sessions: status `completed`, `completed_at` stamped. Money: the therapist share now **includes** this session (it did not while it was merely paid). Delivery: the completed count and the no-show rate update.

XR-CUTOFF-001 — The Session Completed cutoff reads the same on every surface · P0

Event. A confirmed session passes the cutoff (default 60 minutes after the slot). **Steps.** Simulate 90 minutes after the slot in **each** role's browser and look at every surface that lists that session: the patient's card and calendar, the therapist's card, the admin's All Sessions row, the admin's Schedule day panel, and the admin's session drawer. **Expected Result. Every one reads "Session Completed"** — including the admin's own. A session an hour past its start must read the same way on every screen it appears on. Changing the cutoff setting must move **all** of them together.

XR-CARE-001 — One recommendation, three views · P0

Event. THR-CARE-001. **Expected Result.** Therapist's chart, patient's Suggested Sessions **and** patient's Health Profile all render **the same rows** — one record, two readers, branching only on whose voice the copy is written in. Admin → Sessions → Recommendations lists it. **All four agree on the package, the session count and the price.**

XR-CARE-002 — One purchase, five views · P0

Event. PAT-CARE-002. **Expected Result.** Patient: Your Packages, `6 sessions · 0 used · 6 remaining`. Therapist: the programme appears in the Programmes view of My Patients with the same balance. Admin: Catalog → Purchases with the same balance and a frozen snapshot. Money: the purchase's cash appears as **Package cash collected** and is recognised session by session as **Recognised revenue** — these are **two different words for two different things and must not be conflated**. Activity Log: nothing (a patient purchase is not an admin action).

XR-CREDIT-001 — One credit consumed, four views · P0

Event. The patient books one package session. **Expected Result.** Patient widget: `1 used · 5 remaining`. Therapist: the session appears, auto-assigned and auto-confirmed with its own Meet link. Admin Purchases: the same balance. **[SQL]** the ledger holds exactly one `reserve:<appointment_id>` row. Flipping the ledger-authority switch must not change any of these four figures while the ledger and the counters agree.

XR-PAYOUT-001 — One settlement, four views · P0

Event. `FIN-PAY-002`. **Expected Result.** Therapist Earnings: paid-out increases, owed decreases by the same amount. Admin Payouts: the row settles. Money → Summary: the therapist share is unchanged (settling moves money, it does not create a new share). Activity Log: a ``payout.settle`` row with the actor, target and amount.

XR-CASH-001 — One cash collection, four views · P0

Event. `THR-SESS-007`. **Expected Result.** Therapist: the visit shows cash recorded, and their next payout figure **drops by the cash held**. Admin Cash Ledger: the collection appears as un-remitted. Money → Payouts: the Pay button shows the netted figure. After settling: the Cash Ledger stops listing it, and a second payout does **not** deduct it again.

XR-HP-001 — One health-profile update, three views · P0

Event. `THR-HP-002` then `PAT-HP-004`. **Expected Result.** Patient: their own answers, correctly attributed — **the counter must not claim the patient answered questions a clinician wrote**, and the banner must not say "Your therapist has your answers" about a record the patient never sent. Therapist: the same answers on the chart. Admin: the change request in the review queue and, after approval, in Review History. **Nobody is told they did something they did not do.**

XR-SUGG-001 — One suggestion, two views · P1

Event. `THR-SUGG-001` → `PAT-SUGG-002`. **Expected Result.** While pending: therapist sees **Waiting on the patient**, patient sees the card. After acceptance: therapist sees the booked session, patient sees it under Upcoming, and exactly one credit has moved.

XCFG-ROSTER-001 — A roster change moves nothing · P0

Purpose. This is the guard on the deliberate separation between the roster and the booking picker. It is the single most likely place for a well-intentioned "fix" to break the design.

Steps

1. Note exactly which dates and hours `/book` offers under simulated time TIME-A. Screenshot it.
2. As an admin, blank Therapist A's **entire** weekly schedule (mark every day unavailable) and save.
3. Put Therapist A on leave for the next fortnight.
4. Add a date exception that closes a day entirely.
5. Reload `/book` under the same simulated time and compare with the screenshot.
6. Check the patient's existing confirmed session with Therapist A.

Expected Result. Step 5: **the picker is byte-for-byte identical**. The roster is the clinic's planning record — who can be *offered* — and it deliberately does **not** filter the patient's picker, which applies the lead-time rule alone. Step 6: **the appointment is unchanged** — not cancelled, not moved, not flagged. **If the picker changed, do not "fix" the roster — raise it.** Connecting the two is a product decision with a deploy-sized blast radius, not a refactor.

22. Full regression journeys

Each journey is an end-to-end run through the product, executed in one sitting. Run all four before a release.

REG-J1 — The core money journey · P0

SETUP-RESET-001

- SETUP-CAT-001 (three conditions) → SETUP-PKG-001 (P1, P2, P3)
- ADM-SET-026 (scoped admins) → ADM-PEOP-006 (revenue shares)
- THR-AUTH-001 → ADM-APPR-002 → THR-AVAIL-001
- PAT-BOOK-002 → PAT-BOOK-003 (book + pay)
- ADM-SESS-002 (assign) → XR-BOOK-001 (five views agree)
- THR-SESS-005 (complete) → XR-COMplete-001
- THR-CARE-001 (recommend) → ADM-CARE-004 (clinic approves) → XR-CARE-001
- PAT-CARE-002 (accept + pay) → XR-CARE-002
- PAT-PKG-001 (spend 3 credits) → XR-CREDIT-001
- THR-SESS-005 x3 (deliver them)
- FIN-SUM-001 (identities hold) → FIN-PAY-002 (settle) → XR-PAYOUT-001
- ADM-SET-033 (every action is in the Activity Log)

Pass criterion. Both money identities hold at the end, every cross-role check agrees, and the Activity Log contains every mutating action including `payout.settle`.

REG-J2 — The home-visit and cash journey · P0

ADM-SET-013 (home visits on) → SETUP-AREA-001 → SETUP-HVPKG-001

- PAT-HV-002 (prepaid, serviceable) → PAT-HV-003 (unserviceable → waitlist)
- PAT-HV-006 (cash on visit)
- ADM-SESS-002 (assign) → THR-SESS-007 (record cash)
- THR-SESS-005 (complete) → XR-CASH-001
- FIN-PAY-003 (cash nets off the payout, and is marked remitted)
- FIN-PAY-004 (cash > owed → floors at zero, stays on the ledger)
- FIN-REF-003 (cash refund → manual pending)
- PAT-CARE-003 (a recommended home programme quotes travel per visit)
- ADM-SET-013 again (switch off → the recommendation stops being purchasable)

REG-J3 — The partner journey · P0

HOS-LEAD-001 → HOS-AUTH-002 (provision) → HOS-AUTH-001

- HOS-REF-001 (refer) → HOS-REF-004 (the whole status pipeline)
- HOS-REF-006 (patient registers with the code)
- PAT-BOOK-003 as Patient C → ADM-SESS-002 → THR-SESS-005
- HOS-MONEY-001 (commission on net) → XR-BOOK-002
- PAT-CANCEL-001 on a second referred session (refund reverses the commission)
- HOS-MONEY-002 (a therapist with no share ⇒ excluded, not guessed)
- HOS-SEC-001..004 (isolation)

REG-J4 — The clinical journey · P0

THR-HP-001 (triage, ortho) → THR-HP-002 (first fill, live)
→ PAT-HP-001 then PAT-HP-002 (locked → unlocked)
→ PAT-HP-003 (one question at a time) → PAT-HP-004 (change → review)
→ PAT-DOC-001..003 (upload, limits, delete)
→ THR-HP-005 (Pain Map on ortho; refused on neuro)
→ THR-HP-004 (re-triage to neuro MERGES, never replaces)
→ ADM-SET-020 (edit questions; disable paediatrics; ortho cannot be disabled)
→ THR-SESS-008 (session notes; invisible to the patient and to the PDF)
→ PAT-HP-005 (export as PDF, including a non-Latin name)
→ THR-LEAK-001..007 (the scanner, both tiers, and the evidence tables)
→ XR-HP-001

REG-J5 — Security sweep · P0

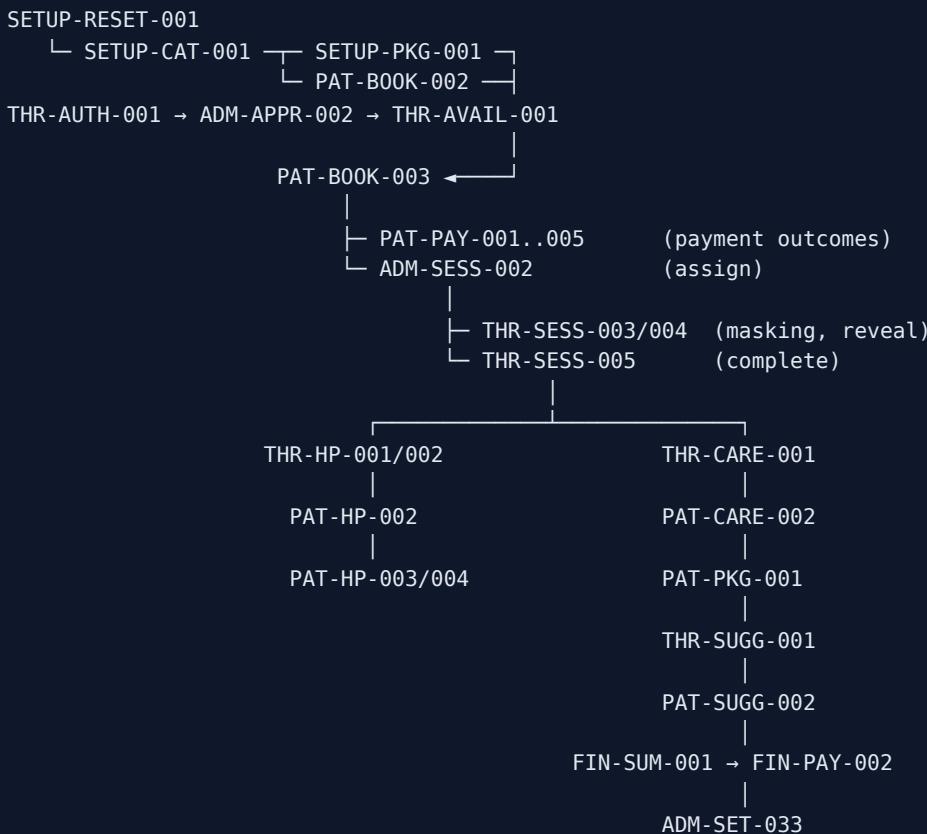
Run **all** of §18 in one pass, then `ADM-SET-027`'s full table.

REG-J6 — Payment integrity sweep · P0

Run **all** of §16.3 in one pass.

23. Test dependency map and recommended execution order

23.1 The main dependency chain



23.2 Recommended execution order

#	Phase	Tests	Notes
1	Reset	SETUP-RESET-001..003	Must be first. Confirm an admin survives.
2	Admin & catalog setup	ADM-CAT-001, ADM-CAT-005, ADM-CAT-010, SETUP-HVPKG-001, ADM-SET-026	Nothing downstream works without a catalog.
3	Create users	PAT-AUTH-002, THR-AUTH-001, HOS-LEAD-001 → HOS-AUTH-002	Patient A is created <i>inside</i> PAT-B00K-003, deliberately — that is the guest path.
4	Approve users	ADM-APPR-001..004	
5	Configure availability	THR-AVAIL-001..007, ADM-ROST-001..005	
6	Booking	PAT-B00K-001..017, PAT-HV-001..007	Time-simulation scenarios TIME-A...D.
7	Payment	PAT-PAY-001..005, \$16.3	Needs Razorpay test keys and the webhook secret.
8	Therapist session	ADM-SESS-002..004, THR-SESS-001..008	
9	Care plan	THR-CARE-001..008, ADM-CARE-001..008	
10	Purchase	PAT-CARE-001..004	
11	Credits	PAT-PKG-001..004, THR-SUGG-*, PAT-SUGG-*	
12	Clinical	THR-HP-*, PAT-HP-*, PAT-DOC-*, THR-LEAK-*	
13	Finance	\$16.1-16.2	Build the reference dataset first.
14	Configuration dependencies	\$15.4, all 46 rows	Restore every setting afterwards.
15	Security	\$18 in full	
16	UX / mobile / ally	\$19	
17	Error / loading / empty	\$20	Empty states are easiest right after a reset — consider running ERR-EMPTY-001 in phase 1.
18	Cross-role	\$21	
19	Final regression	REG-J1..J6	

23.3 Tests that must be run on a fresh database

SETUP-RESET-001, ERR-EMPTY-001, PAT-EMPTY-001, and any test that books a **fixed** future slot. A rerun on a dirty database will refuse the booking as a clash — **that refusal is correct behaviour**.

23.4 Tests that change global state — restore afterwards

Test	Restore
Every <code>ADM-SET-*</code>	Put the setting back to its documented default
<code>DBG-TIME-*</code> , and every TIME scenario	Reset to Real Time
<code>SETUP-RESET-003</code>	Re-arm <code>ALLOW_DEBUG_DATA_RESET=true</code>
<code>PAY-WH-004</code>	Restore <code>RAZORPAY_WEBHOOK_SECRET</code>
<code>ADM-PEOP-007</code> , <code>SEC-AUTH-006</code>	Re-activate the suspended accounts
<code>ADM-CAT-002</code> (deactivate)	Re-activate the category
<code>ADM-SET-020</code> (disable paediatrics)	Re-enable it

24. Coverage audit

24.1 Area x dimension matrix

Area	Patient	Therapist	Hospital	Admin	Finance	Security	Mobile
Registration / login	<code>PAT-AUTH-001..006</code>	<code>THR-AUTH-001..003</code>	<code>HOS-AUTH-001..003</code>	<code>SETUP-RESET-001</code>	—	<code>SEC-AUTH-004..007</code>	<code>UX-MOB-002</code>
Approval gates	<code>PAT-AUTH-003</code>	<code>THR-AUTH-002</code>	<code>HOS-AUTH-003</code>	<code>ADM-APPR-001..004</code>	—	<code>SEC-AUTH-006</code>	—
Booking (video)	<code>PAT-BOOK-001..017</code>	<code>THR-SESS-001</code>	—	<code>ADM-NEWB-001</code> , <code>ADM-SESS-002</code>	<code>FIN-SUM-002</code>	<code>PAT-BOOK-012</code> , <code>SEC-TAMPER-001</code>	<code>UX-MOB-001</code>
Booking (home visit)	<code>PAT-HV-001..007</code>	<code>THR-SESS-007</code>	<code>HOS-REF-002</code>	<code>ADM-CAT-010</code> , <code>ADM-SET-014</code>	<code>FIN-PAY-003</code>	<code>PAT-HV-005</code>	<code>UX-MOB-001</code>
Payment	<code>PAT-PAY-001..005</code>	—	—	<code>FIN-TXN-001</code>	\$16.3 in full	<code>PAY-AMT-001/002</code>	<code>UX-MOB-001</code>
Refunds	<code>PAT-CANCEL-001..003</code>	—	<code>HOS-MONEY-001</code>	<code>ADM-SESS-004</code>	<code>FIN-REF-001..004</code>	—	—
Sessions lifecycle	<code>PAT-SESS-001..006</code>	<code>THR-SESS-001..008</code>	—	<code>ADM-SESS-001..004</code> , <code>ADM-SCHED-001</code>	<code>FIN-SUM-002</code>	<code>THR-SEC-002</code>	<code>UX-MOB-002</code>

Area	Patient	Therapist	Hospital	Admin	Finance	Security	Mobil e
Availabil ity / roster	—	THR-AVAIL- 001..008	—	ADM-ROST- 001..005	—	THR-SEC- 001 , ADM- ROST-005	—
Health profile	PAT-HP- 001..005	THR-HP- 001..006	HOS- SEC-002	ADM-PEOP- 004 , ADM- SET-020	—	SEC-DATA- 001	UX- MOB- 004
Docume nts	PAT-D0C- 001..003	(read)	HOS- SEC-002	(read)	—	SEC-DATA- 004	UX- MOB- 003
Care plans	PAT-CARE- 001..004 , PAT- SCHED-001..003	THR-CARE- 001..008	—	ADM-CARE- 001..008	PAY- AMT-002	THR-CARE- 002	—
Suggest ed sessions	PAT-SUGG- 001..005	THR-SUGG- 001..002	—	ADM-SET-018	—	PAT-SUGG- 004	—
Session credits	PAT-PKG- 001..004	(view)	—	ADM-CAT- 014/015 , ADM-SET-019	FIN- REF-004	SEC-TAMPER- 003 , PAY- CONC-001	—
Referral s	HOS-REF-006	—	HOS- REF- 001..00 7	ADM-PEOP-008	HOS- MONEY- 001..00 3	HOS-SEC- 001	—
Earning s / payouts	—	THR-EARN- 001..004	HOS- MONEY- 001..00 3	FIN-PAY- 001..006	FIN- PAY- 001..00 6	SEC-ADMIN- 001	—
Catalog	(reads)	(reads)	—	ADM-CAT- 001..015	ADM- CAT-006	ADM-SET- 028	UX- MOB- 005
Settings	—	—	—	ADM-SET- 001..035	FIN- COST- 002	ADM-SET- 025..028	—
Contact controls	THR-LEAK-005	THR-LEAK- 001..007 , THR-SESS- 003/004	—	ADM-SET-029	—	SEC-DATA- 005	—
Risk	—	—	—	ADM-RISK- 001..003	—	ADM-RISK- 003	—
Audit log	—	—	—	ADM-SET-033	XR- PAYOUT- 001	ADM-SET- 033	—
Public site	PUB-*	—	HOS- LEAD- 001	ADM-SET- 004..008	—	SEC-ROUTE- 004	UX- MOB- 001

Area	Patient	Therapist	Hospital	Admin	Finance	Security	Mobil e
Debug bar	DBG-TIME-001	—	—	SETUP-RESET- 001..003 , DBG-NAV-001	—	SETUP- RESET- 002/003	—

24.2 Route coverage

Every route in §3 is mapped to at least one test in its own table's rightmost column. **All 9 public pages, 2 booking routes, 11 patient routes, 8 therapist routes, 6 hospital routes, 2 admin routes + 28 admin screens + 3 admin detail routes, and 4 system routes are covered.** API routes are covered by the tests that drive them plus §18's direct calls.

24.3 Admin screen coverage

All **28** screens have at least one dedicated test — see the §3.6 table. Every screen with a mutating control also has a negative and an authorization test.

24.4 Configuration coverage

All **46** configuration→dependent-feature pairs in §15.4 have a verification test. Every one names the screen where the change must be *proved*, not merely saved.

24.5 Payment coverage

Success (PAT-B00K-003), failure (PAT-PAY-001), cancellation/dismissal (PAT-PAY-002), abandonment (PAT-PAY-004), retry (PAT-PAY-001 , PAT-PAY-005), refund (FIN-REF-001..004), refund failure (FIN-REF-002), duplicate click (PAT-B00K-017), duplicate callback (PAY-WH-002), duplicate webhook (PAY-DUP-004), signature tampering (PAY-WH-001), amount tampering (PAY-AMT-001/002), concurrency (PAY-CONC-001/002), and the seven "must never happen" duplications (PAY-DUP-001..007).

24.6 Role coverage

Every protected action has both an authorized case and an unauthorized one. The unauthorized case is always tested **at the route**, not only in the UI.

24.7 State coverage

Entity	Transitions covered
Appointment	requested → confirmed → completed; → cancelled (+refund / no refund); completed → reopened; cancelled/no-show → restored; unpaid → paid
Package purchase	active → expired; active → refunded; credits reserved → consumed → released/voided
Care plan	active → accepted (purchased, closed); active → withdrawn; active → declined; superseded
Suggestion	pending → accepted; pending → declined; pending → lapsed (computed, never written)
Referral	pending_review → therapist_assigned → invite_sent → converted; → declined; → withdrawn
Payout request	pending → reviewing → completed

Entity	Transitions covered
Risk signal	open → reviewing → dismissed / actioned; reopened as a fresh signal
Account	unapproved → approved; active → suspended → active; patient auto-approved by payment attempt
Cash	collected → (corrected) → remitted; refund → manual_pending → returned

24.8 What is not testable in this environment, and why

Item	Why	What to do instead
Real email delivery	All fixtures use the reserved <code>.test</code> TLD, and the product sends no transactional email of its own (the Google Calendar invite is the only outbound notification)	Verify the request succeeded and the reset landing page renders
Google Calendar/Meet against a real calendar	Needs live Google credentials and a real calendar	Test the failure and retry path instead (<code>ADM-SESS-003</code>) — that is the behaviour that matters
Live Razorpay settlement, chargebacks, real bank refunds	Test mode does not settle	Verify the local state transitions and the gateway's own test dashboard
Server-clock-dependent gates under simulation	The simulated clock is client-side only, by design	Use real near-future slots (<code>\$7.2</code>)
<code>next start</code> ISR behaviour matching fixtures	Public pages cache for 300 s	Run against <code>next dev</code> ; if you must use <code>next start</code> , wait out or trigger revalidation
Browser-side Supabase reads in a network-isolated sandbox	Chromium needs egress for <code>?therapist=</code> resolution	Check egress first (<code>PAT-B00K-007</code> 's note) before filing a defect
Multi-region / timezone-shifted testing	The app records the browser's detected timezone	Change the OS timezone and re-run <code>PAT-B00K-002</code> ; the calendar and the stored <code>timezone</code> must agree
Load and performance	Out of scope for a manual plan	Note any screen that feels slow, especially the admin dashboard's ~40 queries

25. Questions and clarifications before execution

These are **product decisions**, not gaps in the inspection. In each case the code's current behaviour is established and stated, and the test asserts exactly that behaviour — but whether it is the *intended* behaviour is a call only the product owner can make. Confirm each before executing the affected tests; if a decision changes the answer, the named test changes with it. **Nothing here is invented behaviour.**

1. **Duplicate referral prevention (affects `HOS-REF-003`)**. Established from the schema: `patient_referrals` carries **no uniqueness constraint** on hospital + patient name, so a hospital that genuinely re-types the same referral creates a **second row**. Only the double-tap case is defended, by the form's own submit guard. **Decision needed:** is a genuine re-submission meant to be blocked at the database, de-duplicated on the admin's side, or simply left visible in the queue for a human to decline? The test currently asserts the last of these, because that is what the code does today.
1. **Therapist timezone (affects `THR-AVAIL-001`)**. Established from the code: the roster editor reads `profiles.timezone` and prints it as a label, falling back to `Asia/Kolkata`; there is **no field anywhere in the therapist's Edit Profile that sets it**, and the admin's Roster only displays it. So today it is per-therapist in the schema and un-editable in the product. **Decision needed:** should a therapist (or an admin) be able to set it, or is the fallback the intended behaviour for a clinic operating in one timezone? Until that is decided, `THR-AVAIL-001` asserts only that the header states a timezone.
1. **Tablet support (affects §19)**. The plan tests 820 × 1180 as a courtesy. **Question:** is tablet a supported breakpoint with its own expectations, or simply "desktop layout, narrower"?
1. **Paediatric fixture depth (affects §8.6)**. Patient D's paediatric data is supplied so the third specialty can be exercised, but no journey in this plan requires a paediatric patient end to end. **Question:** should paediatrics get its own full journey, or is `ADM-SET-020`'s enable/disable coverage sufficient for this release?
1. **`plan\_conversion\_low` and `post\_consultation\_dropout` (affects `ADM-RISK-001`)**. Both ship **disabled** because a threshold invented before the clinic has a baseline fires on everyone or on nobody. **Question:** should this release enable them with a provisional threshold, or leave them off? The tests currently assert they are off.
1. **Refund of a partially delivered home-visit package (affects `FIN-REF-004`)**. The session-package rule ("void what is available, never what is consumed") is explicit. The equivalent for a **cash** home-visit package, where no online payment exists to reverse, resolves to `manual_pending`. **Question:** confirm the intended split when a cash home-visit package is partly delivered — how much is expected back at the door?
1. **Reassigning a programme's therapist mid-course (affects `ADM-CAT-014`)**. Reassignment touches **future** sessions only. **Question:** should the patient be notified, and if so through which surface? The plan currently asserts only the data outcome.
1. **Expected UI copy for a handful of validation messages**. The API messages quoted throughout this document are taken verbatim from the route handlers. Where a component *re-words* a route's error before showing it, the test asserts the API string. **Question:** for any case where the on-screen wording differs from the quoted API string, confirm which is authoritative so the test can assert the right one.

26. Defect reporting template

Paste this, filled in, when reporting a failure. The **Test ID plus the actual result** is what makes a report diagnosable.

```

Test ID:
Role:
Feature:
Environment:          (local dev / staging; Supabase project ref; app URL)
Server:               (next dev / next start)
Simulated Date/Time:  (the value set in the Debug bar, or "real time")
Browser / viewport:
Step that failed:     (the numbered step)
Expected Result:      (quote it from the test case)
Actual Result:        (exactly what happened, including the exact on-screen text)
Test Data:            (the values entered)
Screenshot / video:
Console error:
Network / API error:  (route, HTTP status, response body)
Payment ID / Order ID: (if applicable)
Appointment ID:        (if applicable)
Purchase / Entitlement ID: (if applicable)
Reproducible?:        (always / intermittently / once)
First or second consecutive run on this database?
Additional notes:

```

Before filing, rule out these five known-correct behaviours:

1. A booking refused as a clash on a **second consecutive run** — leftover state, not a bug.
2. A **server-side** time gate refusing an action under a simulated clock — the simulation is client-side by design.
3. A fixture missing from a **public page under `next start`** — ISR caches for 300 seconds.
4. A `?therapist=` chip missing in a **network-isolated sandbox** — the browser needs egress.
5. `/home-visit` returning **404** while the master switch is off — that is the feature.

27. Final release checklist

Sign off each line before release.

Data and environment

- [] `ALLOW_DEBUG_DATA_RESET` is **unset** in production, and `debug_reset_all_data()` is dropped
- [] `.env.production` does not exist in the repository
- [] The hosting dashboard's own environment variables have been checked (a deleted file cannot clear those)
- [] `RAZORPAY_WEBHOOK_SECRET` is set in production
- [] Razorpay is in **live** mode with live keys; no test key remains
- [] `supabase/schema.sql` has been applied, and applies **twice** cleanly
- [] Supabase Auth → **Confirm email is OFF**
- [] The `medical-reports` bucket is **private**; `avatars` is public
- [] At least two `full`-scope admins exist

Pre-launch removals

- [] **The Debug Bar is deleted** — not merely switched off. It is a public flag, and the bar names `/admin/login` and `/admin/dashboard`

- [] The five seeded testimonials are replaced with real, consented ones, or removed
- [] Stock photography under `public/photos/` is replaced with the clinic's own, keeping the aspect ratios

Functional sign-off

- [] `REG-J1` ... `REG-J6` all pass on a fresh database
- [] Both money identities hold on the reference dataset
- [] Settings → System Health reports **no** accounting disagreements and **no** unresolved sync issues
- [] Every action in the audit vocabulary has been observed in the Activity Log, `payout.settle` included
- [] No generated password appears anywhere in the Activity Log
- [] `npm run verify` (lint + unit tests + build) passes
- [] `npm run lint` passes, including the Realtime publication coverage check
- [] The Playwright suite passes against a test project (`workers: 1` , against `next dev`)

Security sign-off

- [] Every \$18 case passes, each at the route level as well as in the UI
- [] No admin path appears in any public client bundle (once the debug bar is deleted)
- [] No stack trace, column name or row id is reachable on any error screen
- [] Append-only tables (`session_credit_ledger` , `care_plan_versions` , `communication_flags` , `contact_reveal_log` , `admin_activity_log` , `risk_reviews` , `session_note_revisions`) all refuse update and delete

UX sign-off

- [] The full booking + payment flow passes at 390 × 844 with no horizontal scrolling
- [] Keyboard-only completion of the booking wizard succeeds
- [] No hydration warnings in the console on any page
- [] Every dashboard offers **Back to Home** in all three sidebar renders

Documentation

- [] `README.md` , `AGENTS.md` and `CLAUDE.md` describe the shipped behaviour — routes, roles, environment variables, npm scripts, and every documented rule (booking lead time, refund window, payment verification, Meet sync, payout maths)